



THE COMPLETE 2026 GUIDE TO MEDICARE, SOCIAL SECURITY & RETIREMENT INCOME

RETIRE WITH CONFIDENCE

Medicare, Social Security, and the
Money Decisions That Decide Your Retirement



DARIN WEIDAUER

Gerontologist • Registered Social Security Analyst • 22-Year U.S. Air Force Veteran

Retire With Confidence — 2026 Edition

Medicare, Social Security, and the Money Decisions That Decide Your Retirement

Darin Weidauer

August 2026

RETIRE WITH CONFIDENCE

The Complete Guide to Medicare, Social Security, Income, and the Decisions That Will Define the Rest of Your Life

By Darin Weidauer

Gerontologist, U.S. Air Force Veteran, Independent Insurance Agent, and Registered Social Security Analyst “Most people spend more time researching a refrigerator than they do planning their retirement. This book changes that.”

Free Retirement Blueprint Session — ECOS Call or Text: 702-706-6564 | darinweidauer@ecos.care

TABLE OF CONTENTS

Front Matter

Dedication

About the Author

Introduction: The Retirement Gauntlet — And How to Win It

PART I: MEDICARE: YOUR FOUNDATION

Chapter 1: Medicare 101 — What It Is, What It Isn't, and Why It Matters

Chapter 2: The Four Parts of Medicare — A, B, C, and D Explained Simply

Chapter 3: When and How to Enroll — The 7-Month Window You Cannot Miss

Chapter 4: What Medicare Covers (And the Gaps That Will Shock You)

Chapter 5: Medicare Costs in 2026 — Every Premium, Deductible, and Copay

Chapter 6: Original Medicare vs. Medicare Advantage — The Real Trade-Offs

Chapter 7: Medigap / Medicare Supplement — Plans That Make Medicare Bulletproof

Chapter 8: Part D — Prescription Drugs, Formularies, and the \$2,100 Cap

Chapter 9: Dental, Vision, Hearing, and the Benefits Medicare Ignores

Chapter 10: Filling the Gaps — Indemnity, Cancer, Hospital, and Accident Plans

PART II: IRMAA, PENALTIES, AND COSTLY MISTAKES

Chapter 11: IRMAA — The Hidden Medicare Tax Nobody Warned You About

Chapter 12: Medicare Penalties — Late Enrollment Mistakes That Last a Lifetime

Chapter 13: The Home Sale Trap — How Selling Your House Can Spike Medicare Costs

Chapter 14: Roth Conversions, IRA Withdrawals, and IRMAA

PART III: SOCIAL SECURITY

Chapter 15: Social Security Basics — How Your Benefit Is Calculated

Chapter 16: When to Claim — Age 62 vs. 67 vs. 70

Chapter 17: Spousal, Survivor, and Divorced Spouse Benefits

Chapter 18: Social Security and Work — The Earnings Test

Chapter 19: Social Security and Taxes — How Much Uncle Sam Takes

Chapter 20: Registered Social Security Analysis — Why Working With an RSSA Matters

PART IV: RETIREMENT INCOME PLANNING

Chapter 21: Building Your Retirement Income Stack

Chapter 22: 401(k), IRA, and Roth IRA — The Six-Figure Tax Difference

Chapter 23: The Thrift Savings Plan (TSP) — For Federal Employees and Veterans

Chapter 24: Required Minimum Distributions (RMDs)

Chapter 25: Roth Conversions — The Tax Strategy That Can Transform Your Retirement

Chapter 26: Annuities — When They Make Sense and When They Don't

Chapter 27: Life Insurance in Retirement — Protection, Legacy, and Tax-Free Income

Chapter 28: Real Estate in Retirement — Equity, Income, and the IRMAA Trap

Chapter 29: The April Surprise — Setting Up Tax Withholding So Tax Day Never Shocks You

Chapter 30: The State Tax Map — Where You Live Changes What You Keep

PART V: PROTECTING WHAT YOU'VE BUILT

Chapter 31: Where Will You Live? — Aging in Place, Downsizing, CCRCs, and Reverse Mortgages

Chapter 32: Long-Term Care — The Risk That Can Erase a Lifetime of Savings

Chapter 33: Asset-Based Long-Term Care — Hybrid Policies That Give Your Money Back

Chapter 34: Short-Term Care Insurance — The Affordable Bridge

Chapter 35: Caring for Aging Parents — The Sandwich Years, Getting Paid, and Protecting Your Own Retirement

Chapter 36: Final Expense Planning — Protecting Your Family

Chapter 37: Indemnity Insurance — Cancer, Critical Illness, and Hospital Cash Plans

PART VI: SPECIAL SITUATIONS

Chapter 38: Veterans, TRICARE, and VA Benefits

Chapter 39: Federal Employees and FEHB

Chapter 40: Medicaid and Dual Eligibility

Chapter 41: Working Past 65 — HSAs, Creditable Coverage, and Timing Medicare

Chapter 42: The First 90 Days — A Financial Checklist for Widows and Widowers

Chapter 43: Protecting Yourself from Medicare Scams and Fraud

PART VII: FUTURE-PROOFING YOUR RETIREMENT

Chapter 44: Annual Medicare Review

Chapter 45: Helping the Grandkids — 529s, Gifting Rules, and Leaving a Legacy

Chapter 46: The Retirement Decision Timeline — Age 59½ to 75+

Chapter 47: Your Retirement Blueprint — Putting It All Together

Bonus: 10 Questions to Ask Before You Trust Anyone With Your Retirement

Bonus: The Five-Document Estate Checklist Every Retiree Needs

Conclusion: The Call That Could Change Everything

Glossary of 60+ Terms

What Changed for 2026 — and What's Coming in 2027

2026 Quick Reference Card

Sources & Data Credits / Resources and Official Links

Dedication

To every American who put in the work, served the country,
raised a family,

and now deserves a retirement that honors every sacrifice.

To the men and women of the United States Armed Forces—
you carried the weight so others could stand tall.

Now it's time to protect what you've earned.

About the Author

Darin Weidauer

Darin Weidauer is not your typical insurance agent. He's a 22-year United States Air Force veteran who began his military career as a Heavy Construction Equipment Operator, rose through the ranks to become a Cyber Warfare Officer, and finished his service as a University Professor—teaching the next generation of leaders the skills they'd carry into battle and beyond.

After his military service, Darin brought the same discipline, precision, and commitment to the world of retirement planning. He holds five Master's degrees: an MBA in Psychology from Pepperdine University, a Master's in Alternative Dispute Resolution from Pepperdine Law School, a Master's in Long-Term Care Administration from the Davis School of Gerontology at the University of Southern California (USC), an Information Technology Master's from Touro University, and a Bachelor's in Math and Business from Wayland Baptist University.

Darin is a licensed Gerontologist—a specialist in the aging process and the unique needs of older adults. He is a Registered Social Security Analyst (RSSA), one of a select group of professionals trained in the complex rules governing Social Security benefits. He has taught at Loyola Marymount University and lectured at more than 54 colleges and universities across Southern California.

As an Independent Licensed Insurance Agent, Darin holds licenses in Life, Health, Accident, Indemnity, Long-Term Care, Annuities, and Dental insurance. His expertise includes Medicare (Advantage, Supplement/Medigap, Part D), Social Security maximization, long-term care planning, asset-based LTC, short-term care insurance, life insurance, annuities, 401(k)/IRA/Roth/TSP/RMD planning, real estate and IRMAA implications, cancer/indemnity/dental/vision/hearing insurance, and comprehensive retirement income strategies.

Because Darin is independent, he doesn't work for any one insurance company. He works for you. He shops the market to find the plans that fit your needs and your budget—not a corporate sales quota.

Darin's mission is simple: help every person he meets retire with confidence, clarity, and a plan that protects everything they've worked for.

Contact Darin: 702-706-6564 | darinweidauer@ecos.care

Introduction: The Retirement Gauntlet — And How to Win It

Let me tell you about a woman I'll call Linda.

Linda worked 34 years as a school administrator. She was smart, organized, and responsible with her money. She maxed out her 403(b) every year. She paid off her house. She saved diligently. By any measure, she did everything right.

Then she turned 65, and the gauntlet began.

In the span of 18 months, Linda had to make decisions about Medicare Part A, Part B, Part D, Medigap vs. Medicare Advantage, when to file for Social Security, whether to do a Roth conversion, how to handle her Required Minimum Distributions, and what to do about long-term care. Each decision had deadlines. Each decision had penalties for getting it wrong. And almost none of it was explained in plain English.

Linda missed her Initial Enrollment Period for Part B by two months. She didn't think she needed it because she was still on her employer's plan—but her employer had fewer than 20 employees, which meant her coverage wasn't considered "creditable." The result? A 10% penalty on her Part B premium for every year she was late. For the rest of her life.

That single mistake will cost Linda more than \$12,000 over the next 20 years.

I wrote this book so that doesn't happen to you.

What You're Up Against

Here's the uncomfortable truth: the retirement system in America is not designed to be easy. Medicare has four parts, dozens of plan types, enrollment windows that open and close on strict timelines, and penalties that never expire. Social Security has more than 2,700 rules governing how benefits are calculated and paid. The tax code treats your retirement

savings like a puzzle with pieces that don't fit together unless you know the tricks.

And the people who should be helping you? Many of them are salespeople in disguise, pushing products that pay them commissions rather than plans that protect your future.

That's not what this book is. And that's not who I am.

Why I Wrote This Book

I spent 22 years in the United States Air Force. I started operating heavy equipment on runways, and I ended my career teaching cyber warfare to officers. Along the way, I earned five Master's degrees, became a licensed Gerontologist, a Registered Social Security Analyst, and an independent insurance agent licensed in every major line of coverage that affects retirees.

I've sat across the table from thousands of people approaching retirement. Some were generals. Some were teachers. Some were truck drivers. All of them had the same look in their eyes: equal parts hope and fear.

This book is my attempt to replace that fear with knowledge. Not theory. Not jargon. Real numbers, real scenarios, and real strategies you can use starting today.

What This Book Will Do for You

By the time you finish this book, you will understand exactly how Medicare works—what it covers, what it doesn't, and which plan type is right for you. You'll know when to file for Social Security and how that decision affects your spouse. You'll understand how your 401(k), IRA, Roth IRA, and other accounts work together—and how the wrong withdrawal strategy can cost you tens of thousands in unnecessary taxes. You'll know what IRMAA is and why selling your house at the wrong time could spike your Medicare premiums. And you'll have a clear, year-by-year roadmap from age 59½ to 75 and beyond.

This isn't a book you read once and put on a shelf. It's a reference guide you'll come back to every year. Keep it close.

How to Use This Book

You can read this book cover to cover, or you can jump straight to the chapter that addresses your most pressing concern. Every chapter is designed to stand on its own while also building on the chapters before it.

Throughout the book, you'll find tables with real 2026 numbers, real-world examples with dollar amounts, checklists you can use immediately, and FAQ sections that answer the questions I hear most often. At the end of every chapter, you'll find my contact information. That's not a sales pitch—it's an open invitation. If anything in this book raises a question, pick up the phone or send a text. I'm here to help.

Let's get started.

Disclaimer

This book is for educational purposes only and does not constitute legal, tax, or financial advice. While every effort has been made to ensure accuracy using official 2026 figures from the Centers for Medicare & Medicaid Services (CMS), the Social Security Administration (SSA), and the Internal Revenue Service (IRS), rules and figures can change. Always verify current information with official sources and consult with a qualified professional before making financial decisions. Individual results vary based on personal circumstances. The examples in this book use composite characters and hypothetical scenarios for illustration purposes.

Important Disclosures

ECOS Insurance Solutions and Darin Weidauer are not affiliated with, endorsed by, or approved by the Social Security Administration, the Centers for Medicare & Medicaid Services, Medicare, or any other government agency. This book is an independent educational publication. We do not offer every plan available in your area. Any information we provide is limited to those plans we do offer in your area. Please contact

Medicare.gov or 1-800-MEDICARE to get information on all of your options. ECOS is contracted with different insurance carriers in different parts of the country, so the plans we can discuss with you depend on where you live. Insurance services and product availability vary by state.

2026 Edition — v2026.1 (August 2026). Figures reflect the official 2026 program releases from CMS, SSA, and the IRS. Numbers change every year. For the current figures and any corrections to this edition, visit: MyECOS360.com/book-updates

What Changed for 2026 — And What’s Coming in 2027

Numbers move every year. Miss an update, and your budget — or your strategy — can be off by thousands. Here are the changes that matter most, verified against the official releases from the Centers for Medicare & Medicaid Services (CMS), the Social Security Administration (SSA), and the Internal Revenue Service (IRS).

The Big 2026 Changes at a Glance

Item	2025	2026	Change
Part B standard premium	\$185.00/mo	\$202.90/mo	+\$17.90 (almost 10%)
Part B deductible	\$257	\$283	+\$26
Part A hospital deductible	\$1,676	\$1,736	+\$60
Part D out-of-pocket cap	\$2,000	\$2,100	+\$100 (indexed annually)
Part D maximum deductible	\$590	\$615	+\$25
IRMAA starts at (single / married)	\$106,000 / \$212,000	\$109,000 / \$218,000	Thresholds indexed up
Social Security COLA	2.5%	2.8%	Avg. retiree check: \$2,071/mo
Max earnings taxed by Social Security	\$176,100	\$184,500	+\$8,400
Earnings test (under FRA)	\$23,400	\$24,480	+\$1,080
401(k) employee limit	\$23,500	\$24,500	+\$1,000
IRA limit	\$7,000	\$7,500	+\$500

Sources: CMS, 2026 Medicare Parts A & B Premiums and Deductibles (Nov. 14, 2025); CMS, Final CY 2026 Part D Redesign Program Instructions; SSA, 2026 COLA Fact Sheet (Oct. 24, 2025); IRS Notice 2025-67.

Historic Firsts Taking Effect in 2026

Negotiated drug prices are here. For the first time in Medicare’s history, the federal government negotiated prices directly with drug manufacturers. On January 1, 2026, new lower “Maximum Fair Prices” took effect for ten high-cost

drugs — Eliquis, Jardiance, Xarelto, Januvia, Farxiga, Entresto, Enbrel, Imbruvica, Stelara, and NovoLog/Fiasp — with discounts of

38% to 79% off previous list prices. CMS estimates roughly 9 million enrollees will save about \$1.5 billion in out-of-pocket costs in 2026 alone. (Source: CMS, Medicare Drug Price Negotiation Program.)

A new \$6,000 tax deduction for seniors. Under the One Big Beautiful Bill Act, taxpayers age 65 and older can claim an extra \$6,000 deduction per person (\$12,000 for a qualifying married couple) for tax years 2025 through 2028. It phases out above \$75,000 MAGI (single) or \$150,000 (joint). Most middle-income retirees qualify in full. (Source: IRS, “Enhanced Deduction for Seniors.”)

Insulin stays capped at \$35 per month, and vaccines recommended for seniors remain free under Part D.

★COMING IN 2027 — NOT YET IN EFFECT 15 more drugs get negotiated prices on January 1, 2027, including Ozempic, Wegovy, Rybelsus, Xtandi, Ibrance, Trelegy Ellipta, Otezla, and Linzess. CMS projects \$8.5–\$12 billion per year in savings for Part D enrollees once these take effect. A Medicare pilot program covering anti-obesity medications also begins in 2027; in the meantime, a federal agreement makes qualifying GLP-1 weight-loss drugs available for about \$50/month from mid-2026 through the end of the year. The 2027 premiums, deductibles, and COLA will be announced in October–November 2026 — and your 2027 IRMAA will be based on your 2025 tax return. If any of these affect your medications or your income plan, this is exactly the kind of thing we watch for you.

 FREE RETIREMENT BLUEPRINT SESSION — ECOS One conversation could save you thousands. Call or text Darin Weidauer at 702-706- 6564 or email darinweidauer@ecos.care. No pressure. No sales pitch. Just answers.

The Part D subsidy is ending. For 2025 and 2026, a federal program called the Part D Premium Stabilization

Demonstration held down premiums on stand-alone drug plans while the Inflation Reduction Act's redesign took effect. CMS announced on July 28, 2026 that it will not continue past this plan year — stand-alone Part D returns to what CMS calls "traditional market conditions" in 2027. The demonstration cost roughly \$9.8 billion across two years, and the Government Accountability Office found that without it, premiums for people who stayed in their 2024 plan would have close to doubled. Early estimates suggest a large share of stand-alone drug plan enrollees could see monthly increases somewhere in the \$11–\$20 range.

The 2027 national base beneficiary premium is set at **\$41.33**, up from \$38.99 — exactly the 6% ceiling the Inflation Reduction Act allows. Note that this is a benchmark used in federal calculations, not what anyone actually pays; your plan's own premium depends on its bid, its benefits and your region.

What to do about it: final 2027 plan premiums are published in mid-to-late September 2026, and Annual Notice of Change letters land in the fall. Open enrollment opens October 15. If you are in a stand-alone Part D plan, this is the year not to auto-renew. Call me in October and we'll compare what's actually in front of you.

Medicare's **GLP-1 Bridge** demonstration launched July 1, 2026, capping monthly copays for qualifying weight-loss medications at about \$50 for eligible beneficiaries.

PART ONE

PART I — MEDICARE: YOUR FOUNDATION

Every retirement plan starts with healthcare. Medicare is the foundation everything else is built upon.

Chapter 1: Medicare 101

What It Is, What It Isn't, and Why It Matters More Than You Think

Imagine this: you've just blown out the candles on your 65th birthday cake. Your family is smiling. You're thinking about golf, grandkids, maybe a trip to Europe. But somewhere in that pile of birthday cards is a letter from Medicare that will affect every year of your life from this moment forward.

Ignore that letter, and you could face penalties that last the rest of your life. Misunderstand it, and you could end up with thousands of dollars in unexpected medical bills. Get it right, and you'll have the peace of mind to actually enjoy the retirement you earned.

Medicare is the single most important decision you'll make in retirement. And yet most people spend more time researching their next car than understanding their healthcare coverage.

Let's fix that right now.

What Medicare Actually Is

Medicare is a federal health insurance program run by the Centers for Medicare & Medicaid Services (CMS). It was signed into law in 1965 by President Lyndon B. Johnson as part of the Social Security Act. Its original purpose was straightforward: give Americans aged 65 and older access to affordable healthcare.

Today, Medicare covers more than 67 million Americans. That includes people 65 and older, people under 65 with certain disabilities, and people of any age with End-Stage Renal Disease (permanent kidney failure requiring dialysis or a transplant) or Amyotrophic Lateral Sclerosis (ALS, also known as Lou Gehrig's disease).

Think of Medicare as the base layer of your retirement healthcare. It's not optional. It's not a luxury. It's the

foundation that everything else—Medigap plans, drug coverage, dental, vision, long-term care—is built upon.

What Medicare Is NOT

Here’s where people get into trouble. Medicare is not free healthcare. It’s not unlimited healthcare. And it is absolutely not the same thing as Medicaid.

Let me be blunt: Medicare has premiums, deductibles, copays, and coinsurance. It does not cover everything. It does not cover long-term care (nursing homes for extended stays). It does not cover most dental, vision, or hearing care. It does not cover care received outside the United States in most cases. And it comes with enrollment rules so strict that missing a deadline by even one day can result in permanent financial penalties.

I’ve seen people assume Medicare will “take care of everything.” That assumption has bankrupted families. Don’t let it bankrupt yours.

Medicare vs. Medicaid: Clear Up the Confusion Now

This is one of the most common mix-ups I see. Medicare and Medicaid are two completely different programs.

Feature	Medicare	Medicaid
Who It’s For	People 65+ or with qualifying disabilities	Low-income individuals and families
Funded By	Federal payroll taxes (you’ve been paying in)	Federal and state governments jointly
Administered By	Federal government (CMS)	Each state individually
Based On	Age or disability	Income and assets
Cost to You	Premiums, deductibles, copays	Little or no cost (varies by state)
Covers Long-Term Care?	Very limited (100 days max)	Yes, for those who qualify

Some people qualify for both Medicare and Medicaid at the same time. They’re called “dual eligibles,” and they have a unique set of rules we’ll cover later in Chapter 40.

Why Medicare Matters More Than Any Other Retirement

Decision

Here's a number that should get your attention: a 65-year-old retiring today should plan on roughly \$172,500 in healthcare costs over retirement — about \$345,000 for a couple — according to Fidelity Investments' most recent Retiree Health Care Cost Estimate. That's not including long-term care. That's not including dental implants or hearing aids. That's just premiums, copays, deductibles, and prescription drugs over a typical retirement.

Healthcare is likely the single largest expense you'll face in retirement—bigger than housing, bigger than food, and far bigger than most people budget for. The decisions you make about Medicare in your first year of eligibility will determine how much of that \$345,000 actually comes out of your pocket versus being covered by your plan.

Get the wrong plan, and you could pay tens of thousands of dollars more than you need to. Get the right plan, and you'll have predictable costs, broad coverage, and the freedom to focus on living rather than worrying.

The Three Groups of People Who Get Medicare

You're automatically eligible for Medicare if you meet one of these criteria:

Group 1: You're turning 65. This is the most common path. Whether you're still working or fully retired, age 65 is when Medicare eligibility begins for most Americans.

Group 2: You're under 65 with a qualifying disability. If you've been receiving Social Security Disability Insurance (SSDI) for 24 months, you're automatically enrolled in Medicare. This includes conditions like advanced heart failure, severe arthritis, and many others.

Group 3: You have End-Stage Renal Disease or ALS. These conditions grant Medicare eligibility regardless of age, with no 24-month waiting period for ALS.

REAL-WORLD EXAMPLE: The Cost of Waiting

Tom, age 66, retired at 65 but assumed his COBRA coverage was “good enough.” He didn’t sign up for Medicare Part B during his Initial Enrollment Period. When his COBRA ran out 18 months later, he enrolled in Part B during the General Enrollment Period. But because he was 12 months late, he now pays a 20% penalty on his Part B premium—permanently. In 2026, the standard Part B premium is \$202.90 per month. Tom pays \$243.48 per month. Over 20 years, that’s an extra \$9,700+ he didn’t need to spend—all because of a timing mistake.

The Bottom Line

Medicare is not a set-it-and-forget-it decision. It’s a system with moving parts, enrollment windows, penalties, and costs that change every year. But here’s the good news: it’s learnable. It’s manageable. And with the right guidance, you can make decisions that save you thousands of dollars and protect your health for decades.

The chapters that follow will walk you through every piece of Medicare—the four parts, the costs, the enrollment rules, and the plan choices. By the end of Part I, you’ll know more about Medicare than 95% of the people enrolled in it.

FREQUENTLY ASKED QUESTIONS

Q: Is Medicare really free?

A: Part A (hospital insurance) is premium-free for most people—if you or your spouse paid Medicare payroll taxes for at least 10 years (40 quarters). Part B, Part D, and any supplemental plans all have premiums. And even with Part A, you’ll still pay deductibles and copays.

Q: Do I have to sign up for Medicare at 65?

A: Not necessarily. If you’re still working and have creditable employer coverage (from an employer with 20+ employees), you can delay Part B without penalty. But the rules are strict—get this wrong and you’ll pay a penalty for life.

Q: What if I'm a veteran? Do I still need Medicare?

A: VA benefits are excellent, but they don't count as creditable coverage for Medicare purposes. Most veterans benefit from enrolling in Medicare alongside their VA benefits. We cover this in detail in Chapter 38.

Q: Can my spouse get Medicare through me?

A: Your spouse can qualify for premium-free Part A based on your work record if you have 40+ quarters of Medicare-taxed employment. They still enroll separately and must meet age or disability requirements.

Q: What happens if I move to another state?

A: Medicare is a federal program—it follows you everywhere in the U.S. However, Medicare Advantage and Medigap plan availability and pricing can vary by state and county. If you move, review your plan options in your new area.

CHAPTER 1 CHECKLIST

☐

☐ I know the difference between Medicare and Medicaid ☐ I understand the three groups who qualify for Medicare ☐ I know that late enrollment can result in permanent penalties ☐ I've noted my 65th birthday and the 7-month window around it ☐ If I'm still working at 65, I've confirmed whether my employer coverage is "creditable" > 📞 READY TO TAKE THE NEXT STEP? You don't have to figure this out alone. I've helped thousands of people navigate these exact decisions, and I'd be honored to help you, too. Call or text me for a free, no-obligation Retirement Blueprint Session . No sales pitch. Just straight answers from someone who's been there. Darin Weidauer | ECOS | 702-706-6564 | darinweidauer@ecos.care

Chapter 2: The Four Parts of Medicare

A, B, C, and D Explained Simply

If Medicare were a car, Part A would be the engine, Part B would be the steering wheel, Part C would be the premium upgrade package, and Part D would be the fuel. You need to understand all four to stay on the road.

Too many people sign up for Medicare without understanding what each part does. They end up with coverage they don't need and gaps they don't know about. This chapter will make sure that doesn't happen to you.

Part A: Hospital Insurance

Medicare Part A covers inpatient care. Think of it as your “inside the hospital” coverage. Here's what Part A covers:

Inpatient hospital stays (semi-private room, meals, nursing care, drugs administered in the hospital, and other hospital services). Skilled nursing facility care (up to 100 days per benefit period, after a qualifying 3-day hospital stay). Home health care (part-time skilled nursing, physical therapy, occupational therapy, and other services). Hospice care (comfort care for terminal illness, including medications for pain management).

Here's the key detail most people miss: Part A is premium-free for most Americans. If you or your spouse worked and paid Medicare payroll taxes for at least 10 years (40 quarters), you pay \$0 per month for Part A. If you don't have 40 quarters, you can still buy Part A—but the 2026 premium is up to \$565 per month.

Part A Costs in 2026

Cost Category	2026 Amount	What It Means
Monthly Premium	\$0 (with 40+ quarters)	Free for most people
Monthly Premium (30-39 quarters)	\$311 per month	Reduced premium
Monthly Premium (<30 quarters)	\$565 per month	Full premium

Cost Category	2026 Amount	What It Means
Inpatient Deductible	\$1,736 per benefit period	You pay this before Part A

Source: Centers for Medicare & Medicaid Services (CMS), 2026 Medicare Parts A & B Premiums and Deductibles.

Beyond 150 days | All costs | You pay everything | Skilled Nursing: Days 1-20 | \$0 coinsurance | Fully covered | Skilled Nursing: Days 21-100 | \$217.00 per day | Daily coinsurance | Skilled Nursing: Beyond 100 days | All costs | You pay everything |

Notice that Part A coverage has a hard limit. After 60 days in the hospital, you start paying \$434 per day. After 90 days, it jumps to \$868 per day using your lifetime reserve days—and you only get 60 of those in your entire life. After 150 days total, Medicare stops paying entirely. This is one of the biggest gaps in Medicare, and it’s exactly why supplemental insurance matters.

Part B: Medical Insurance

If Part A is your “inside the hospital” coverage, Part B is your “outside the hospital” coverage. It handles doctor visits, outpatient services, preventive care, and medical equipment.

Part B covers: doctor and specialist visits, outpatient surgery and procedures, diagnostic tests (lab work, X-rays, MRIs), mental health services (outpatient), durable medical equipment (wheelchairs, walkers, oxygen), preventive services (annual wellness visits, screenings, flu shots), and ambulance services when medically necessary.

Part B Costs in 2026

Unlike Part A, Part B is not free. Everyone who enrolls in Part B pays a monthly premium. The standard premium for 2026 is \$202.90 per month. But here’s the catch: if your income is above a certain threshold, you’ll pay more. A lot more. This is called IRMAA, and it’s so important that we’re dedicating an entire chapter to it later (Chapter 11).

Part B Cost	2026 Amount
Standard Monthly Premium	\$202.90
Annual Deductible	\$283
Coinsurance (after deductible)	20% of Medicare-approved amount
Excess Charges	Up to 15% above Medicare rate (Original

Medicare only)

Source: CMS, 2026 Medicare Parts A & B Premiums and Deductibles (announced November 14, 2025).

That 20% coinsurance is unlimited. There is no out-of-pocket maximum on Original Medicare Part B. If you have a \$200,000 surgery, you owe \$40,000. That’s not a typo. This is one of the most dangerous gaps in Medicare, and it’s the primary reason Medigap insurance exists.

Part C: Medicare Advantage (The All-in-One Option)

Medicare Part C—commonly called Medicare Advantage—is not a separate benefit from the government. It’s a way of receiving your Part A and Part B benefits through a private insurance company instead of through the government directly.

Think of it this way: Original Medicare (Parts A and B) is like buying groceries at the farmers’ market—you pick each item individually and pay as you go. Medicare Advantage is like a meal kit subscription—everything is bundled, and the company manages the details.

Medicare Advantage plans often include extra benefits not found in Original Medicare: dental, vision, hearing, gym memberships, transportation to medical appointments, and over-the-

counter drug allowances. Most plans include Part D drug coverage built in. And many have \$0 monthly premiums (you still pay your Part B premium).

But there are trade-offs. Medicare Advantage plans use networks (HMO, PPO, or PFFS). You may need referrals to see specialists. You’re limited to doctors and hospitals within the plan’s network (or face higher costs). And if you travel

frequently or split time between states, a network-based plan may not cover you well outside your service area.

We'll do a full, side-by-side comparison of Original Medicare vs. Medicare Advantage in Chapter 6.

Part D: Prescription Drug Coverage

Medicare Part D covers outpatient prescription drugs. It's offered through private insurance companies approved by Medicare. Every plan has a formulary—a list of drugs it covers — organized into tiers that determine your cost.

A landmark change took effect in 2025: a hard annual cap on out-of-pocket prescription drug spending for Part D enrollees. The cap started at \$2,000 in 2025 and is indexed each year — for 2026, it is \$2,100. Before this change, some seniors were paying \$10,000 or more per year for medications. The cap is a game-changer, and Medicare also offers a monthly payment option (the Medicare Prescription Payment Plan) so you can spread that \$2,100 across the year instead of paying it all at once.

We'll cover Part D in full detail in Chapter 8, including how formularies work, how to compare plans, and how the \$2,100 cap changes your financial planning.

□ COMING IN 2027: A second round of 15 negotiated drug prices — including Ozempic, Wegovy, and Rybelsus — takes effect January 1, 2027. If you take any of these medications, your costs may drop significantly next year. (Source: CMS, Medicare Drug Price Negotiation Program.)

How the Four Parts Work Together

Part	What It Covers	Premium (2026)	Key Feature
Part A	Hospital / Inpatient	\$0 (most people)	Premium-free with maximum
Part C	A + B + often D via private plan	(standard) \$0-\$200+/mo	Bundled with extras
Part D	Prescription Drugs	Varies by plan	\$2,100 annual OOP

cap (2026)

REAL-WORLD EXAMPLE: Margaret's Part B Surprise

Margaret, age 67, had Original Medicare with Parts A and B and no supplemental coverage. She needed knee replacement surgery. The Medicare-approved cost was \$50,000. Part A covered

her hospital stay after her \$1,736 deductible. But the surgeon's outpatient follow-up care was billed through Part B. After her \$283 deductible, she owed 20% of all Part B charges—with no maximum. Her out-of-pocket costs totaled over \$11,000. If Margaret had a Medigap Plan G, her total out-of-pocket would have been \$283 (the Part B deductible). That's it.

FREQUENTLY ASKED QUESTIONS

Q: Do I need all four parts?

A: At minimum, you need Parts A and B. From there, you choose: either Original Medicare with a Medigap plan and standalone Part D, or a Medicare Advantage plan that bundles everything. We'll help you decide which path is right in Chapter 6.

Q: Can I have Medicare Advantage AND a Medigap plan?

A: No. Medigap only works with Original Medicare (Parts A and B). If you enroll in a Medicare Advantage plan, you cannot use a Medigap plan. It's one path or the other.

Q: Is Medicare Advantage really free?

A: Many plans have \$0 premiums, but you still pay your Part B premium (\$202.90/month in 2026). And you'll still have copays, deductibles, and coinsurance within the Advantage plan.

Q: What's the difference between coinsurance and copay?

A: A copay is a fixed dollar amount you pay (e.g., \$30 for a doctor visit). Coinsurance is a percentage of the total cost (e.g., 20% of a \$10,000 procedure = \$2,000). Both are your share of the cost after Medicare pays its portion.

CHAPTER 2 CHECKLIST

☐ I understand what Parts A, B, C, and D each cover ☐ I know Part A is premium-free for most people with 40+ work quarters
☐ I understand that Part B has no out-of-pocket maximum ☐ I know the difference between Original Medicare and Medicare Advantage ☐ I'm aware of the \$2,100 Part D out-of-pocket cap
> 📞 READY TO TAKE THE NEXT STEP? You don't have to figure this out alone. I've helped thousands of people navigate these exact decisions, and I'd be honored to help you, too. Call or text me for a free, no-obligation Retirement Blueprint Session . No sales pitch. Just straight answers from someone who's been there. Darin Weidauer | ECOS | 702-706-6564 | darinweidauer@ecos.care

Chapter 3: When and How to Enroll

The 7-Month Window You Cannot Miss

If Chapter 1 explained what Medicare is and Chapter 2 explained how it works, this chapter is about the when—and in Medicare, timing is everything.

Miss your enrollment window by a single day, and you could face penalties that follow you for the rest of your life. I’m not being dramatic. I’m being honest.

Let’s walk through every enrollment period, every deadline, and every exception—so you never get caught off guard.

The Initial Enrollment Period (IEP): Your 7-Month Golden Window

Your Initial Enrollment Period is a 7-month window centered around your 65th birthday. It starts 3 months before your birthday month, includes your birthday month, and extends 3 months after your birthday month.

For example, if your birthday is October 15, your IEP runs from July 1 through January 31 of the following year.

During this window, you can sign up for Part A, Part B, or both. This is your cleanest, easiest entry point into Medicare. If you enroll during the three months before your birthday month, your coverage starts on the first day of your birthday month.

When You Enroll	Coverage Starts
3 months before birthday month	1st day of birthday month
During birthday month	1st day of the following month
1 month after birthday month	1st day of 2nd month after enrollment
2 months after birthday month	1st day of 3rd month after enrollment
3 months after birthday month	1st day of 3rd month after enrollment

The message is clear: enroll early. The earlier you enroll within your IEP, the sooner your coverage starts and the fewer gaps you'll have.

The General Enrollment Period (GEP): The Backup Plan with

Penalties

If you miss your IEP and don't qualify for a Special Enrollment Period, your next chance is the General Enrollment Period, which runs from January 1 through March 31 each year. Coverage doesn't start until July 1.

That means you could be without Medicare coverage for months. And here's the penalty: for every 12-month period you could have had Part B but didn't sign up, your Part B premium goes up by 10%. That penalty is permanent.

If you were eligible at 65 but didn't enroll until 67, that's two years late—a 20% penalty. On the 2026 standard premium of \$202.90, that's an extra \$40.58 per month, or about \$487 per year, for life.

Special Enrollment Periods (SEPs): The Safety Nets

There are several situations that give you a Special Enrollment Period, allowing you to enroll outside of the standard windows without penalty.

The most common SEP is for people who are still working at age 65 and have employer-sponsored health coverage from an employer with 20 or more employees. As long as you have this "creditable coverage," you can delay Part B without penalty. When you leave your job or lose that coverage, you get an 8-month SEP to enroll in Part B.

Other SEPs include: moving out of your plan's service area, losing your current coverage through no fault of your own, qualifying for Medicaid or Extra Help, being released from incarceration, or specific circumstances related to disasters or government errors.

The Creditable Coverage Trap

This is where I've seen the most expensive mistakes.

"Creditable coverage" doesn't just mean you have health insurance. It means your current coverage is at least as good as Medicare Part B, AND it comes from an employer with 20 or more employees who is actively employing you.

COBRA does not count as creditable coverage. Retiree insurance may or may not count. Coverage from a spouse's employer with fewer than 20 employees may not count. VA coverage does not count for Medicare purposes. Marketplace (ACA/Obamacare) plans do not count.

If your coverage is not creditable and you delay Part B enrollment, you will owe a permanent penalty. I cannot emphasize this enough: verify your creditable coverage status before making any enrollment decisions.

Medicare Advantage and Part D Enrollment Periods

Medicare Advantage and Part D have their own enrollment seasons:

Annual Enrollment Period (AEP): October 15 through December 7 each year. This is when you can join, switch, or drop a Medicare Advantage plan or Part D plan. Changes take effect January 1.

Medicare Advantage Open Enrollment Period (MA OEP): January 1 through March 31. If you're already in a Medicare Advantage plan, you can switch to a different Advantage plan or return to Original Medicare and pick up a Part D plan. You can make one change during this period.

REAL-WORLD EXAMPLE: The COBRA Mistake

Steve retired at 65 and elected COBRA continuation from his former employer. His HR department told him COBRA would "cover him" for 18 months. What they didn't tell him was that COBRA is not creditable coverage for Medicare purposes. Steve assumed he had 18 months to figure out Medicare. He was wrong. When his COBRA ended at age 66½, he enrolled in Part B during the General Enrollment Period—but because he

was 18 months late, he faces a permanent 10% Part B penalty. At \$202.90/month standard, his penalty adds \$20.29/month, or about \$243/year, for the rest of his life. Total estimated cost over 20 years: roughly \$4,870.

FREQUENTLY ASKED QUESTIONS

Q: I'm turning 65 but still working. Do I have to sign up?

A: If your employer has 20+ employees and you have their group health plan, you can delay Part B without penalty. You should still sign up for Part A (it's free and doesn't interfere with employer coverage). When you retire or lose that employer coverage, you'll have an 8-month SEP to enroll in Part B.

Q: My spouse has employer coverage. Does that protect me?

A: It depends. If your spouse's employer has 20+ employees and you're covered under their group plan as a dependent, you can delay Part B. If the employer has fewer than 20 employees, Medicare becomes your primary insurance at 65, and you need to enroll.

Q: Can I enroll in Medicare online?

A: Yes. You can enroll at ssa.gov, by calling Social Security at 1-800-772-1213, or by visiting your local Social Security office. Your initial enrollment in Parts A and B goes through Social Security, not Medicare directly.

CHAPTER 3 CHECKLIST

☐

☐ I've verified whether my current employer coverage is "creditable" ☐ I understand that COBRA is NOT creditable coverage for Medicare ☐ I know the Part B late enrollment penalty is 10% per year, permanently ☐ I know the Annual Enrollment Period runs October 15 - December 7 ☐ I've created a calendar reminder 3 months before my 65th birthday

> 📞 **READY TO TAKE THE NEXT STEP?** You don't have to figure this out alone. I've helped thousands of people navigate these exact decisions, and I'd be honored to help you, too. Call or text me for a free, no-obligation Retirement Blueprint

Session . No sales pitch. Just straight answers from someone who's been there. Darin Weidauer | ECOS | 702-706-6564 | darinweidauer@ecos.care

Chapter 4: What Medicare Covers (And the Gaps That Will Shock You)

Here's a question I hear all the time: "Darin, I have Medicare. I'm covered, right?"

And my answer is always the same: "You're partially covered. And the uncovered parts can destroy your retirement savings if you're not prepared."

Medicare is excellent at what it does. But what it doesn't do is where families get blindsided. Let's walk through both sides.

What Medicare DOES Cover

Medicare covers a broad range of medically necessary services. Here are the highlights:

Under Part A (Hospital/Inpatient): inpatient hospital care, skilled nursing facility care (up to 100 days), home health services, hospice care, and inpatient psychiatric care (up to 190 lifetime days in a psychiatric hospital).

Under Part B (Medical/Outpatient): doctor visits and specialist consultations, outpatient surgery, preventive services (mammograms, colonoscopies, cardiovascular screening, diabetes screening), annual wellness visits, clinical laboratory services, mental health services (outpatient), durable medical equipment, ambulance services, and some home health services.

Medicare also covers many preventive services at no cost to you—no copay, no deductible—as long as you see a doctor who accepts Medicare assignment. These include your Annual Wellness Visit ("Wellness Visit" is different from a physical exam), flu shots, hepatitis B shots, pneumonia vaccines, COVID-19 vaccines, cardiovascular disease screenings, diabetes screenings, and several cancer screenings.

What Medicare Does NOT Cover

Here’s where the conversation gets uncomfortable. Medicare has significant gaps, and these gaps are where financial disaster lives.

Not Covered by Original		
Medicare	Why It Matters	Estimated Annual Cost
Most dental care	Cleanings, fillings, dentures, extractions	\$2,000-\$10,000+
Routine eye exams	Glasses and contact lenses	\$300-\$800
Hearing aids	Devices and fitting exams	\$2,000-\$7,000 per pair
Long-term custodial care	Nursing home stays, assisted living	\$60,000-\$120,000+/year
Care outside the U.S.	Emergency care abroad (with rare exceptions)	Varies widely
Cosmetic surgery	Elective procedures	\$5,000-\$20,000+
Acupuncture (most)	Limited to chronic low back pain only	\$75-\$150 per session
Concierge or boutique medicine	Direct primary care memberships	\$1,500-\$5,000/year

That long-term care line should stop you in your tracks. A semi-private room in a nursing home has a national median cost of \$9,277 per month—more than \$111,000 per year (CareScout Cost of Care Survey, the most recent national data). Medicare covers skilled nursing facility care for up to 100 days only, and only after a qualifying 3-day hospital stay. For the millions of Americans who will need long-term care (and roughly 70% of people over 65 will), Medicare provides almost no help.

The 20% Problem: Medicare’s Unlimited Coinsurance

I call this the “20% Problem” because it’s the single biggest financial risk in Original Medicare, and most people don’t discover it until they get a bill.

After you meet your Part B deductible (\$283 in 2026), you pay 20% of the Medicare-approved amount for most outpatient services. That sounds manageable for a \$200 doctor visit (you’d owe \$40). But what about a \$300,000 cancer treatment? That’s

\$60,000 out of your pocket. And there is no annual out-of-pocket maximum on Original Medicare.

Let me repeat that: Original Medicare has no cap on what you can owe. Medicare Advantage plans, by contrast, are required to have an out-of-pocket maximum (the limit in 2026 is \$9,250 for in-network services). This is one of the most important distinctions between Original Medicare and Medicare Advantage.

REAL-WORLD EXAMPLE: The \$47,000 Surprise

Patricia, age 72, was diagnosed with non-Hodgkin's lymphoma. Her treatment included chemotherapy, radiation, multiple specialist visits, and a two-week hospital stay. Total Medicare-approved charges: \$287,000. Under Original Medicare with no supplemental coverage, Patricia's share was the Part A deductible (\$1,736) plus 20% of Part B outpatient charges, totaling approximately \$47,000. If Patricia had a Medigap Plan G, her total cost would have been \$283 (the Part B annual deductible). The rest would have been covered.

FREQUENTLY ASKED QUESTIONS

Q: Does Medicare cover physical therapy?

A: Yes, Part B covers outpatient physical therapy, occupational therapy, and speech-language pathology services when medically necessary. There is no longer a hard annual cap, but Medicare may review claims above a certain threshold.

Q: Does Medicare cover mental health counseling?

A: Yes. Part B covers outpatient mental health services, including visits with psychiatrists, psychologists, and clinical social workers. You pay 20% of the Medicare-approved amount after your deductible.

Q: Does Medicare cover weight-loss drugs like Ozempic or Wegovy?

A: Part D now covers certain anti-obesity medications for qualifying conditions as of 2026. Coverage varies by plan and formulary. Check your specific Part D plan's formulary for coverage details and prior authorization requirements.

CHAPTER 4 CHECKLIST

☐ I understand what Medicare covers and what it doesn't ☐ I know that Original Medicare has NO out-of-pocket maximum ☐ I'm aware that dental, vision, and hearing are not covered by Original Medicare ☐ I understand that Medicare covers only 100 days of skilled nursing—not long-term care ☐ I've started thinking about supplemental coverage to fill Medicare's gaps >

 **READY TO TAKE THE NEXT STEP?** You don't have to figure this out alone. I've helped thousands of people navigate these exact decisions, and I'd be honored to help you, too. Call or text me for a free, no-obligation Retirement Blueprint Session . No sales pitch. Just straight answers from someone who's been there. Darin Weidauer | ECOS | 702-706-6564 | darinweidauer@ecos.care

Chapter 5: Medicare Costs in 2026

Every Premium, Deductible, and Copay

Numbers don’t lie. And when it comes to Medicare, knowing the exact numbers is the difference between a budget that works and a retirement that falls apart.

This chapter is your 2026 Medicare cost reference. Dog-ear this page. Bookmark it. Come back to it every time you need to crunch a number. These are the official figures from CMS for the 2026 plan year.

Part A Costs (Hospital Insurance) — 2026

Part A Cost Item	2026 Amount
Monthly Premium (40+ quarters)	\$0
Monthly Premium (30-39 quarters)	\$311
Monthly Premium (<30 quarters)	\$565
Inpatient Hospital Deductible	\$1,736 per benefit period
Hospital Coinsurance (Days 61-90)	\$434 per day
Lifetime Reserve Days (Days 91-150)	\$868 per day
Skilled Nursing Facility (Days 1-20)	\$0
Skilled Nursing Facility (Days 21-100)	\$217.00 per day
Skilled Nursing Facility (Beyond 100 days)	100% — you pay all costs
Blood (first 3 pints)	You pay full cost

Part B Costs (Medical Insurance) — 2026

Part B Cost Item	2026 Amount
Standard Monthly Premium	\$202.90
Annual Deductible	\$283
Coinsurance	20% of Medicare-approved amount
Out-of-Pocket Maximum	NONE (unlimited)
Excess Charges (non-participating providers)	Up to 15% above Medicare rate

Part B IRMAA Surcharges — 2026

If your Modified Adjusted Gross Income (MAGI) from two years ago exceeds certain thresholds, you pay more for Part B. This

surcharge is called IRMAA—Income-Related Monthly Adjustment Amount. The 2026 IRMAA brackets are based on your 2024 tax return.

Individual MAGI	Married Filing Jointly	Part B Monthly Premium
\$109,000 or less	\$218,000 or less	\$202.90
\$109,001-\$137,000	\$218,001-\$274,000	\$284.10
\$137,001-\$171,000	\$274,001-\$342,000	\$405.80
\$171,001-\$205,000	\$342,001-\$410,000	\$527.50
\$205,001-\$500,000	\$410,001-\$750,000	\$649.20

Individual MAGI	Married Filing Jointly	Part B Monthly Premium
Above \$500,000	Above \$750,000	\$689.90

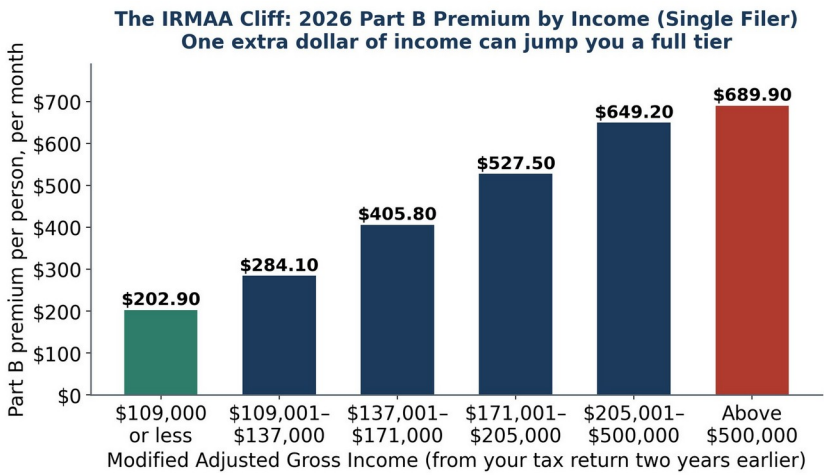
Notice the jump: going from \$109,000 to \$109,001 in individual income costs you an extra \$81.20 per month—about \$974 per year. And that’s per person. A married couple both on Medicare could pay an extra \$1,949 per year just for crossing that threshold by a single dollar. This is why income planning around IRMAA is critical, and we’ll cover strategies in detail in Chapter 11.

Part D Costs (Prescription Drug Coverage) — 2026

Part D Cost Item	2026 Amount
Average Monthly Premium (stand-alone plan)	\$34.50 (CMS projection; varies by plan)
Maximum Annual Deductible	\$615
Annual Out-of-Pocket Maximum	\$2,100
Monthly Payment Option	Available (spread \$2,100 over the year)
Late Enrollment Penalty	1% of national base premium per uncovered

Part D IRMAA Surcharges — 2026

Part D has its own IRMAA surcharge, separate from Part B. The income brackets are the same, but the amounts are different.



Source: Centers for Medicare & Medicaid Services (CMS), 2026 Medicare Parts A & B Premiums and Deductibles.

The IRMAA cliff: 2026 Part B premiums by income tier.

Individual MAGI	Married Filing Jointly	Part D Monthly Surcharge
\$109,000 or less	\$218,000 or less	\$0.00
\$109,001–\$137,000	\$218,001–\$274,000	\$14.50
\$137,001–\$171,000	\$274,001–\$342,000	\$37.50
\$171,001–\$205,000	\$342,001–\$410,000	\$60.40
\$205,001–\$500,000	\$410,001–\$750,000	\$83.30
Above \$500,000	Above \$750,000	\$91.00

What Does Medicare Actually Cost Per Year?

Let’s put it all together for a typical retiree with income below the IRMAA thresholds:

Cost Component	Monthly	Annual
Part A Premium	\$0	\$0
Part B Premium	\$202.90	\$2,435
Part D Premium (average, stand-alone plan)	\$34.50	\$414
Part B Deductible	—	\$283
Part D Deductible (max)	—	\$615
Medigap Plan G Premium (est.)	\$150–\$250	\$1,800–\$3,000
Estimated Total (without major claims)	\$387–\$487	\$5,500–\$6,750

That's the baseline. The moment you have a hospital stay, a surgery, or an expensive medication, those numbers climb—unless you have the right supplemental coverage in place. This is why the plan you choose matters just as much as the premiums you pay.

REAL-WORLD EXAMPLE: The True Cost of Original Medicare Without a Supplement

Robert and Susan, both age 68, chose Original Medicare with Part D but decided to skip Medigap to save money. They saved approximately \$4,000/year in Medigap premiums (\$2,000 each). Then Robert had a heart attack. Three days in the hospital, cardiac catheterization, two stents, and six weeks of cardiac rehab later, the total bill was \$189,000. Robert's share: \$1,736 (Part A deductible) plus roughly \$28,000 (20% of Part B charges) = nearly \$30,000 out of pocket. The \$4,000 they saved skipping Medigap was wiped out seven times over in a single health event.

FREQUENTLY ASKED QUESTIONS

Q: Do Medicare costs go up every year?

A: Generally, yes. CMS adjusts premiums, deductibles, and coinsurance annually. Part B premiums have increased in most years. That's why reviewing your coverage annually during the Annual Enrollment Period (October 15 – December 7) is critical.

Q: Is the \$2,100 Part D cap per person or per couple?

A: Per person. Each individual enrolled in Part D has their own \$2,100 out-of-pocket maximum.

Q: What's a benefit period for Part A?

A: A benefit period starts the day you're admitted to a hospital or skilled nursing facility and ends when you haven't received inpatient care for 60 consecutive days. If you're readmitted after 60 days, a new benefit period begins—and you pay the deductible again.

CHAPTER 5 CHECKLIST

☐ I know the 2026 Part A deductible (\$1,736) and daily coinsurance amounts ☐ I know the 2026 Part B premium (\$202.90/month) and deductible (\$283) ☐ I've checked my income against the IRMAA brackets for both Part B and Part D ☐ I understand the \$2,100 Part D out-of-pocket cap ☐ I've estimated my total annual Medicare costs ☐

> 📞 **READY TO TAKE THE NEXT STEP?** You don't have to figure this out alone. I've helped thousands of people navigate these exact decisions, and I'd be honored to help you, too. Call or text me for a free, no-obligation Retirement Blueprint Session . No sales pitch. Just straight answers from someone who's been there. Darin Weidauer | ECOS | 702-706-6564 | darinweidauer@ecos.care

Chapter 6: Original Medicare vs. Medicare Advantage

The Real Trade-Offs

This is the chapter where most people say, “Finally—just tell me which one to pick!”

I wish it were that simple. The truth is, there is no universally “better” option. Original Medicare with a Medigap plan is the best choice for some people. Medicare Advantage is the best choice for others. And the right answer for you depends on your health, your budget, your doctors, your travel habits, and your tolerance for financial risk.

Let’s lay it all out so you can make an informed decision.

Original Medicare: The Government-Run Option

When people say “Original Medicare,” they mean Part A and Part B administered directly by the federal government through CMS. You get a red, white, and blue Medicare card. You can see any doctor or hospital in the country that accepts Medicare—and about 97% of doctors do.

The advantage of Original Medicare is freedom. No networks. No referrals. No prior authorizations for most services. If a doctor accepts Medicare, you can go there. Period.

The disadvantage is cost exposure. Original Medicare has no out-of-pocket maximum. You pay 20% of Part B charges with no cap. You pay the Part A deductible every benefit period. And you get no dental, vision, or hearing coverage.

That’s why most people on Original Medicare pair it with a Medigap (Medicare Supplement) plan and a standalone Part D drug plan. The Medigap plan covers most or all of the gaps, and the Part D plan covers prescriptions.

Medicare Advantage: The Private Plan Bundle

Medicare Advantage plans are offered by private insurance companies (UnitedHealthcare, Humana, Aetna, Blue Cross, and many others) that contract with Medicare. When you join a Medicare Advantage plan, you still have Medicare—but your benefits are delivered through the private insurer.

The advantages of Medicare Advantage include: often \$0 or low monthly premiums, extra benefits (dental, vision, hearing, fitness, OTC allowances), a required out-of-pocket maximum (\$9,250 or less for in-network services in 2026), and Part D drug coverage is usually included.

The disadvantages include: network restrictions (HMO or PPO), potential need for referrals or prior authorizations, limited coverage outside your plan’s service area, and the plan can change its benefits, network, or formulary every year.

The Side-by-Side Comparison

Feature	Original Medicare + Medigap	Medicare Advantage (Part C)
Monthly Cost	Part B + Medigap + Part D premiums	Part B + plan premium (often
Doctor Choice	Any doctor who accepts	\$0) In-network only (or higher costs out-of-network)
Referrals Needed?	Medicare (97%+) No	Often yes (HMO plans)

Feature	Original Medicare + Medigap	Medicare Advantage (Part C)
Prior Authorization?	Rarely	Common for procedures and
Drug Coverage	Separate Part D plan required	(2026) Usually included
Dental/Vision/Hearing	Not included	Often included
Travel Coverage	Nationwide (any Medicare provider)	Limited to service area
Medigap Compatible?	Yes	(except emergencies) No
Annual Plan Changes	Medigap benefits are standardized	Plans change benefits/networks yearly
Best For	People who want	People who want

Feature	Original Medicare + Medigap	Medicare Advantage (Part C)
	maximum flexibility and predictability	lower premiums and extra benefits

The Hidden Cost of Medicare Advantage

Medicare Advantage plans look great on paper. Low premiums. Extra benefits. A gym membership. But here’s what the TV commercials don’t tell you:

Prior authorization denials are a real issue. A 2023 report from the Office of Inspector General found that Medicare Advantage plans denied approximately 13% of prior authorization requests that would have been covered under Original Medicare. That means real people with real medical needs were told “no” by their insurance company—even though Medicare itself would have said “yes.”

Network changes happen every year. Your favorite doctor could be in-network this year and out-of-network next year. Your plan could change its formulary, dropping a medication you depend on or moving it to a more expensive tier.

And if you ever want to switch back to Original Medicare and add a Medigap plan, you may face medical underwriting. In most states, after your initial Medigap Open Enrollment Period (the first 6 months after you’re 65 and enrolled in Part B), insurance companies can deny you a Medigap plan or charge you more based on your health. If you’ve been on a Medicare Advantage plan for years and your health has declined, switching back could be difficult or impossible.

Who Should Choose Original Medicare + Medigap?

Original Medicare with a Medigap plan tends to work best for people who want the freedom to see any doctor or specialist without referrals, travel frequently or live in multiple states seasonally, want predictable and low out-of-pocket costs, have chronic conditions requiring frequent specialist visits, or are willing to pay higher monthly premiums for greater financial protection.

Who Should Choose Medicare Advantage?

Medicare Advantage tends to work best for people who are generally healthy and use healthcare infrequently, want the lowest possible monthly premium, value extra benefits like dental, vision, and hearing, live in one area and don't travel extensively, and are comfortable with networks and prior authorization processes.

REAL-WORLD EXAMPLE: Two Retirees, Two Paths

Carol, age 66, chose a Medigap Plan G and a standalone Part D plan. Her total monthly cost: \$202.90 (Part B) + \$190 (Medigap) + \$42 (Part D) = about \$435/month. When she was diagnosed with breast cancer, her total out-of-pocket cost for the year was \$283 (Part B deductible). That's it. Her Medigap plan covered everything else.

David, age 66, chose a Medicare Advantage PPO plan with a \$0 premium. His monthly cost: \$202.90 (Part B only). When he was diagnosed with prostate cancer, his out-of-pocket costs totaled \$7,200 over the year (copays, coinsurance, and specialist visit costs up to his plan's out-of-pocket maximum). David saved money in premiums every month—but paid more when he got sick. Neither choice was “wrong.” They reflected different priorities and risk tolerances.

FREQUENTLY ASKED QUESTIONS

Q: Can I switch between Original Medicare and Medicare Advantage?

A: Yes, during the Annual Enrollment Period (October 15 – December 7) and the MA Open Enrollment Period (January 1 – March 31). But switching from Advantage back to Original Medicare can create Medigap issues if you're past your initial enrollment period.

Q: Are Medicare Advantage plans going away?

A: No. Medicare Advantage is growing. In 2026, more than 54% of all Medicare beneficiaries are enrolled in Advantage plans. However, CMS has been tightening regulations on these plans regarding prior authorizations and network adequacy.

Q: What if I hate my Medicare Advantage plan?

A: During the MA Open Enrollment Period (January 1 – March 31), you can switch to a different Advantage plan or return to Original Medicare and add a Part D plan. You can make one change during this window.

CHAPTER 6 CHECKLIST

- ☐ I understand the key differences between Original Medicare and Medicare Advantage
- ☐ I know Original Medicare has no out-of-pocket maximum
- ☐ I know Medicare Advantage requires using network providers
- ☐ I understand the risks of prior authorization denials in Advantage plans
- ☐ underwriting

 **READY TO TAKE THE NEXT STEP?** You don't have to figure this out alone. I've helped thousands of people navigate these exact decisions, and I'd be honored to help you, too. Call or text me for a free, no-obligation Retirement Blueprint Session . No sales pitch. Just straight answers from someone who's been there. Darin Weidauer | ECOS | 702-706-6564 | darinweidauer@ecos.care

Chapter 7: Medigap / Medicare Supplement

The Plans That Make Medicare Bulletproof

If Original Medicare is a roof over your head, Medigap is the insurance policy that repairs the roof when a storm blows through. It doesn't replace Medicare. It completes it.

Medigap plans—also called Medicare Supplement plans—are sold by private insurance companies, but their benefits are standardized by the federal government. That means Plan G from Company A covers exactly the same benefits as Plan G from Company B. The only differences are price and customer service.

That standardization is a gift. It means you can compare plans on one simple variable: cost.

How Medigap Works

A Medigap policy works alongside Original Medicare (Parts A and B). When you receive a medical service, Medicare pays its share first, and then your Medigap plan pays some or all of the remaining costs—deductibles, coinsurance, copays, and even excess charges, depending on which plan you choose.

You pay your monthly Medigap premium on top of your Part B premium. In exchange, you get predictable costs and protection from catastrophic medical bills. With the right Medigap plan, you can see any doctor in the country who accepts Medicare, get treated without referrals or prior authorizations, and know that your out-of-pocket exposure is minimal.

The 2026 Medigap Plans at a Glance

There are currently 10 standardized Medigap plans available, identified by letters. Not all plans are available in every state, and Plans C and F are only available to people who were eligible for Medicare before January 1, 2020.

Benefit	Plan A	Plan B	Plan G	Plan K	Plan L	Plan N
Part A Hospital Coinsurance	100%	100%	100%	100%	100%	100%
Part A Deductible (\$1,736)	No	100%	100%	50%	75%	100%
Part B Deductible (\$283)	No	No	No	No	No	No
Part B Coinsurance (20%)	100%	100%	100%	50%	75%	100%*
Part B Excess Charges	No	No	100%	No	No	No
Blood (first 3 pints)	100%	100%	100%	50%	75%	100%

Benefit	Plan A	Plan B	Plan G	Plan K	Plan L	Plan N
SNF Coinsurance	No	No	100%	50%	75%	100%
Foreign Travel Emergency	No	No	80%	No	No	80%
Out-of-Pocket Limit	None	None	None	\$8,000	\$4,000	None

*Plan N note: Plan N covers Part B coinsurance except for a copay of up to \$20 for some office visits and up to \$50 for emergency room visits that don’t result in an inpatient admission.

The Two Most Popular Plans: G and N

Since Plans C and F are no longer available to new Medicare enrollees (those eligible after January 1, 2020), Plan G has become the gold standard of Medigap coverage. It covers everything except the Part B deductible (\$283 in 2026). For most people, Plan G offers the most comprehensive protection at a reasonable price.

Plan N is a popular alternative for people who want lower premiums. It covers most of what Plan G covers but requires small copays for certain office visits and ER visits, and it does not cover Part B excess charges. If your doctors all accept Medicare assignment (meaning they agree to charge only the Medicare-approved amount), Plan N can save you money.

The Medigap Open Enrollment Period: Use It or Lose It

This is critical. Your Medigap Open Enrollment Period lasts 6 months. It starts the month you're 65 AND enrolled in Part B (both conditions must be met). During this window, insurance companies must sell you any Medigap plan they offer, at the best available price, regardless of your health. They cannot deny you or charge you more because of pre-existing conditions.

After this 6-month window closes, insurance companies in most states can use medical underwriting to decide whether to sell you a plan and how much to charge. If you've developed health conditions since turning 65, you could be denied coverage or face significantly higher premiums.

Do not let this window close without making a decision. Even if you choose Medicare Advantage today, understand that switching to Original Medicare with a Medigap plan later may not be easy.

How Medigap Pricing Works

Medigap premiums are set by private insurance companies and vary based on three pricing methods:

Community-rated (no-age-rated): Everyone pays the same premium regardless of age. Premiums won't increase just because you get older.

Issue-age-rated: Your premium is based on your age when you first buy the policy. It won't increase because of aging, but it may increase due to inflation or other factors.

Attained-age-rated: Your premium starts low but increases as you get older. This is the most common pricing method. Plans may be cheaper initially but can become expensive over time.

When comparing plans, ask the insurance company which pricing method they use. A plan that's \$30/month cheaper today could be \$100/month more expensive in 10 years if it's attained-age-rated.

REAL-WORLD EXAMPLE: Why the Medigap Window Matters

Janet enrolled in a Medicare Advantage plan at age 65 because the \$0 premium was attractive. At age 72, after being diagnosed with rheumatoid arthritis and diabetes, she wanted to switch to Original Medicare with a Medigap plan so she could see specialists without network restrictions. Problem: her Medigap Open Enrollment Period had closed seven years ago. When she applied for Medigap Plan G, she was either denied by most companies or quoted premiums 40–60% higher than the standard rate due to her health conditions. Janet is now stuck paying more for less coverage—or staying in a plan that doesn’t meet her needs.

FREQUENTLY ASKED QUESTIONS

Q: Can I have a Medigap plan and a Medicare Advantage plan at the same time?

A: No. Medigap only works with Original Medicare. If you join a Medicare Advantage plan, your Medigap plan won’t pay for anything, and it’s illegal for a company to knowingly sell you a Medigap plan while you’re in an Advantage plan.

Q: Do Medigap premiums go up every year?

A: Premiums can increase due to inflation, increased medical costs, or age (if you have an attained-age-rated plan). However, the benefits themselves are standardized and don’t change.

Q: Which is better: Plan G or Plan N?

A: Plan G offers more complete coverage (it covers Part B excess charges and has no copays). Plan N offers lower premiums in exchange for small copays and no excess charge coverage. If cost is your priority and your doctors accept Medicare assignment, Plan N is worth considering. If comprehensive protection is your priority, Plan G is usually the better choice.

Q: Do I need a separate Part D plan with Medigap?

A: Yes. Medigap plans do not cover prescription drugs. You’ll need a standalone Part D plan for drug coverage.

CHAPTER 7 CHECKLIST

☐ I understand that Medigap plans are standardized by the government ☐
☐ I've compared Plan G and Plan N based on my health and budget ☐ I've asked about pricing methods (community, issue-age, or attained-age) ☐ I know that Medigap does NOT include drug coverage (I need separate Part D) ☐ I understand that missing my Medigap window could mean higher costs or denial later > 📞 READY TO TAKE THE NEXT STEP? You don't have to figure this out alone. I've helped thousands of people navigate these exact decisions, and I'd be honored to help you, too. Call or text me for a free, no-obligation Retirement Blueprint Session . No sales pitch. Just straight answers from someone who's been there. Darin Weidauer | ECOS | 702-706-6564 | darinweidauer@ecos.care

Chapter 8: Part D

Prescription Drugs, Formularies, and the \$2,100 Cap

Prescription drugs are one of the top three expenses in retirement, right behind housing and food. For some retirees, medications cost more than their mortgage. And until recently, there was no limit on how much you could be asked to pay out of pocket.

That changed in a big way. Starting in 2025, Medicare Part D includes a hard annual out-of-pocket spending cap — \$2,000 in its first year, indexed to \$2,100 for 2026. This is one of the most significant improvements to Medicare in decades, and it’s worth understanding exactly how it works.

How Part D Works

Medicare Part D is offered through private insurance companies approved by Medicare. You can get Part D coverage in two ways: a standalone Prescription Drug Plan (PDP) that pairs with Original Medicare and Medigap, or through a Medicare Advantage plan that includes drug coverage (called an MA-PD).

Every Part D plan has a formulary—a list of covered drugs organized into tiers. The tier determines how much you pay for each medication.

Typical Part D Drug Tiers

Tier	Drug Type	Typical Cost
Tier 1	Preferred Generic	\$0-\$15 copay
Tier 2	Generic	\$15-\$45 copay
Tier 3	Preferred Brand	\$30-\$100 copay
Tier 4	Non-Preferred Brand	25-50% coinsurance
Tier 5	Specialty	25-33% coinsurance

(up to the \$2,100 cap)

Your out-of-pocket cost depends on which tier your drug falls into, whether you’ve met your deductible, and the specific plan

you chose. Not all plans cover every drug, and the same drug can be on different tiers in different plans. This is why comparing Part D plans annually is so important.

The \$2,100 Out-of-Pocket Cap: A Game-Changer

Before 2025, Part D had a coverage structure called the “donut hole”—a gap in coverage where you paid a higher share of drug costs. For people on expensive medications, annual out-of-pocket costs could exceed \$10,000 or more.

The Inflation Reduction Act eliminated the donut hole and replaced it with a hard annual cap on out-of-pocket spending for Part D drugs — \$2,100 in 2026. Once you’ve paid \$2,100 in a calendar year, you pay \$0 for the rest of the year for all Part D covered drugs.

How Medicare Part D works in 2026: deductible, initial coverage, then \$0 after the \$2,100 cap.

Even better: Medicare offers a monthly payment option called the Medicare Prescription Payment Plan. Instead of paying your drug costs as they occur (which could mean a \$1,500 bill in January for a specialty drug), you can spread your total out-of-pocket costs evenly across the remaining months of the year. This makes budgeting much easier.

The Part D Late Enrollment Penalty

If you go 63 or more consecutive days without creditable prescription drug coverage after you’re first eligible for Part D, you’ll pay a late enrollment penalty when you do enroll. The penalty is calculated as 1% of the “national base beneficiary premium” (\$38.99 in 2026) multiplied by the number of months you went without coverage. This penalty is added to your monthly premium permanently.

Example: if you went 24 months without Part D coverage, your penalty would be $24 \times 1\% \times \$38.99 = \9.36 , rounded to \$9.40 per month, or about \$113 per year, for life.

□ 2026 FIRST / COMING IN 2027: January 1, 2026 marked the first time Medicare-negotiated prices took

effect — ten drugs (Eliquis, Jardiance, Xarelto, Januvia, Farxiga, Entresto, Enbrel, Imbruvica, Stelara, NovoLog/Fiasp) now cost 38-79% less than their old list prices. Effective January 1, 2027 (not yet in effect): 15 more drugs join the list, including Ozempic, Wegovy, Rybelsus, Xtandi, Pomalyst, Ibrance, Calquence, Trelegly Ellipta, Ofev, Breo Ellipta, Vraylar, Janumet, Otezla, Linzess, and Xifaxan. A Part D pilot covering anti-obesity medications also begins in 2027. If you take any of these drugs, your 2027 plan comparison could look very different — review it with a licensed professional during Open Enrollment (October 15 - December 7, 2026). (Source: CMS.)

How to Choose the Right Part D Plan

Choosing the right Part D plan comes down to three factors: whether your specific drugs are on the plan’s formulary, whether your preferred pharmacy is in the plan’s network, and the total cost of the plan (premiums + deductibles + copays for your specific drugs).



Source: CMS, Final CY 2026 Part D Redesign Program Instructions; Medicare.gov. Your out-of-pocket costs stop at \$2,100.

How Medicare Part D works in 2026.

The best tool for comparing plans is Medicare’s official Plan Finder at Medicare.gov. Enter your drugs, your pharmacy, and your zip code, and it will show you the total estimated annual cost for each available plan. This is far more useful than just comparing monthly premiums.

REAL-WORLD EXAMPLE: Why Formularies Matter

Frank takes three medications: a generic blood pressure drug (\$12/month), a brand-name cholesterol drug (\$180/month), and a specialty biologic for psoriasis (\$3,200/month). Under Plan A, his biologic is on Tier 5 with 33% coinsurance. Under Plan B, the same biologic is on Tier 4 with a flat \$250 copay. Both plans cap his total at \$2,100/year (the 2026 cap), but Frank hits that cap in January with Plan A (paying the full \$2,100 in one month) versus spreading smaller payments across five months with Plan B. Frank chooses Plan B and uses the Medicare Prescription Payment Plan to spread his \$2,100 across 12 months—about \$175/month—making his budget predictable and manageable.

FREQUENTLY ASKED QUESTIONS

Q: Do I have to have Part D even if I don't take any medications?

A: You're not required to enroll, but if you don't have creditable drug coverage and go without for more than 63 days, you'll face a permanent penalty when you do enroll. Many basic Part D plans cost \$10–\$20/month—cheap insurance against a future penalty.

Q: Can I change my Part D plan every year?

A: Yes. During the Annual Enrollment Period (October 15 – December 7), you can switch Part D plans. Because formularies change yearly, reviewing your plan annually is essential.

Q: Does the \$2,100 cap include my premium?

A: No. The \$2,100 cap applies only to out-of-pocket costs (deductibles, copays, and coinsurance). Your monthly premium is separate.

Q: What if my drug isn't on any plan's formulary?

A: You can request a formulary exception from your plan, appeal a denial, or work with your doctor to find a covered alternative. You can also contact a licensed agent to help you find a plan that covers your specific medications.

CHAPTER 8 CHECKLIST

☐ I've made a list of all my current medications (name, dosage, frequency) ☐ I've used Medicare.gov Plan Finder to compare Part D plans ☐ I know which tier each of my drugs falls on in my plan ☐

☐ I've considered the Medicare Prescription Payment Plan for monthly budgeting ☐ I know the Part D late enrollment penalty and how to avoid it > 📞 READY TO TAKE THE NEXT STEP?

You don't have to figure this out alone. I've helped thousands of people navigate these exact decisions, and I'd be honored to help you, too. Call or text me for a free, no-obligation Retirement Blueprint Session . No sales pitch. Just straight answers from someone who's been there. Darin Weidauer | ECOS | 702-706-6564 | darinweidauer@ecos.care

Chapter 9: Dental, Vision, Hearing, and the Benefits Medicare Ignores

Here's a fact that shocks almost everyone I talk to: Original Medicare—the program designed to cover healthcare for seniors—does not cover the three things that deteriorate most predictably as we age: our teeth, our eyes, and our hearing.

Think about that for a moment. You pay into Medicare your entire working life. You turn 65. And the program doesn't cover the dental crown you need, the glasses you can't see without, or the hearing aids that let you participate in a conversation with your grandchildren.

This isn't an oversight. It's a gap that was built into Medicare in 1965 and has never been fixed. Understanding this gap—and filling it—is one of the most important things you can do to protect your health and your wallet in retirement.

The Dental Gap

Medicare Part A covers some dental care—but only in very specific circumstances, such as dental services related to jaw reconstruction after an accident, or extractions done in a hospital setting before cardiac surgery. Routine dental care—cleanings, fillings, root canals, crowns, dentures, and implants—is excluded.

The average retiree spends between \$2,000 and \$5,000 per year on dental care. A single dental implant can cost \$3,000 to \$5,000. Full dentures run \$1,500 to \$3,000. And periodontal disease—which affects nearly half of adults over 65—can require treatments costing \$5,000 or more.

The Vision Gap

Medicare Part B covers certain eye conditions—glaucoma screening, diabetic eye exams, and macular degeneration treatment. But routine eye exams for glasses, contact lens

fittings, and the glasses or contacts themselves are not covered.

A comprehensive eye exam runs \$100 to \$300. A pair of progressive lenses with frames costs \$300 to \$800. Cataract surgery, which about half of all Americans will need by age 80, is covered by Medicare—but the “premium” lens options that eliminate the need for glasses after surgery are an out-of-pocket upgrade.

The Hearing Gap

Medicare does not cover hearing aids or the exams required to fit them. Given that approximately one-third of people between ages 65 and 74 have hearing loss, and that number rises to nearly 50% for those over 75, this is a massive gap.

A single hearing aid costs \$1,000 to \$4,000, and most people need two. That’s \$2,000 to \$8,000 every 3 to 5 years. Over a 20-year retirement, hearing aid costs alone can total \$10,000 to \$30,000.

Medicare does cover diagnostic hearing exams ordered by a doctor to determine if you have a medical condition causing hearing loss. But the hearing aids themselves? That’s on you.

How to Fill These Gaps

You have several options for covering dental, vision, and hearing:

Medicare Advantage plans: Many MA plans include dental, vision, and hearing benefits as part of their package. The level of coverage varies widely—some plans offer basic preventive dental and vision only, while others include significant allowances for crowns, implants, glasses, and hearing aids. Always read the specifics.

Standalone dental, vision, and hearing insurance: These policies are available from many private insurers. Dental plans typically cost \$25 to \$75 per month and may have annual benefit caps of \$1,000 to \$2,000. Vision plans range from \$10

to \$25 per month. Hearing-specific plans are less common but available through some carriers.

Discount programs and membership plans: Some dental offices offer in-house savings plans (not insurance) that provide discounted rates for an annual membership fee. Organizations like AARP also negotiate group rates on dental and vision services.

Over-the-counter hearing aids: Since 2022, the FDA has allowed over-the-counter hearing aids for adults with mild to moderate hearing loss. These devices cost \$200 to \$1,000 per pair—a fraction of the cost of prescription hearing aids. They’re available at pharmacies and electronics retailers.

Gap Coverage		Annual Benefit	
Option	Monthly Cost	Limit	Best For
Medicare	\$0 additional	Varies (\$500–\$3,000)	People already on MA plans
Advantage (built-in)			
Standalone Dental Insurance	\$25–\$75	\$1,000–\$2,000	Regular dental care needs
Standalone Vision Insurance	\$10–\$25	\$200–\$500 allowance	Annual exams + glasses
Dental Savings Plans	\$80–\$200/year	No limit (discounted rates)	People needing major work
OTC Hearing Aids	N/A (one-time purchase)	N/A	Mild to moderate hearing loss

REAL-WORLD EXAMPLE: The \$23,000 Dental Surprise

George, age 71, had been on Original Medicare with a Medigap plan for six years. He was well-protected against hospital and medical costs. But he’d neglected his teeth. When he finally went to the dentist, he needed three root canals, two crowns, a bridge, and treatment for advanced periodontal disease. Total estimated cost: \$23,000. Medicare covered \$0. George’s Medigap plan covered \$0 (Medigap doesn’t cover dental). George had to dip into his retirement savings to pay the bill. A \$50/month dental plan purchased years earlier could have covered \$1,500–\$2,000/year of that cost, and more importantly,

preventive coverage would have caught the problems earlier when they were cheaper to treat.

FREQUENTLY ASKED QUESTIONS

Q: Will Medicare ever cover dental and vision?

A: Congress has debated adding dental, vision, and hearing benefits to Medicare for years. As of 2026, these benefits remain excluded from Original Medicare. Medicare Advantage plans are currently the primary way to get these benefits within the Medicare system.

Q: Are standalone dental plans worth it?

A: For most retirees, yes. Even a basic plan that covers two cleanings per year and 50-80% of major procedures can save you thousands of dollars over time. The key is choosing a plan with a benefit cap high enough to be meaningful.

Q: Do I need vision insurance if I'm healthy?

A: If you wear glasses or contacts, a basic vision plan pays for itself in one visit. If you don't currently need corrective lenses, a plan may not be necessary—but remember, vision needs typically increase after age 60.

CHAPTER 9 CHECKLIST

☐

☐ I've evaluated dental, vision, and hearing coverage options ☐
I've checked whether my Medicare Advantage plan includes these benefits (if applicable) ☐

☐ I've looked into OTC hearing aids if I have mild to moderate hearing loss ☐ I've budgeted for dental, vision, and hearing costs in my retirement plan > 📞

READY TO TAKE THE NEXT STEP? You don't have to figure this out alone. I've helped thousands of people navigate these exact decisions, and I'd be honored to help you, too. Call or text me for a free, no-obligation Retirement Blueprint Session . No sales pitch. Just straight answers from someone who's been there. Darin Weidauer | ECOS | 702-706-6564 | darinweidauer@ecos.care

CHAPTER 10

Chapter 10: Filling the Gaps

Indemnity, Cancer, Hospital, and Accident Plans

Let me ask you a question: if you were diagnosed with cancer tomorrow, could your household survive on your Medicare coverage alone?

Medicare would cover the medical treatment—the surgery, the chemo, the radiation. But who would cover the mortgage while you’re recovering? The car payment? The groceries? The gas to drive 90 minutes each way to the oncology center three times a week?

These are the expenses that Medicare was never designed to cover. And they’re the expenses that push families into financial crisis during a health event. That’s where supplemental indemnity plans come in.

What Are Indemnity Plans?

Indemnity insurance—also called supplemental insurance or “cash benefit” insurance—pays a fixed dollar amount directly to you when a qualifying health event occurs. It doesn’t pay doctors or hospitals. It pays you.

The money is yours to use however you need: rent, food, utilities, travel expenses, childcare for grandchildren, home modifications, or simply replacing lost income during recovery. There are no restrictions on how you spend it.

Indemnity plans work alongside Medicare, Medigap, and Medicare Advantage. They don’t replace those plans—they supplement them by covering the non-medical costs that come with a health crisis.

Types of Indemnity Plans

Cancer Insurance

Cancer insurance pays a lump sum or scheduled benefits when you’re diagnosed with cancer. Some plans pay a one-time lump

sum (commonly \$10,000 to \$50,000) at diagnosis. Others pay specific amounts for each phase of treatment: surgery, chemotherapy, radiation, hospital stays, transportation, and experimental treatments.

Cancer is the second leading cause of death in the U.S. and one of the most expensive conditions to treat. Even with excellent Medicare coverage, the non-medical costs of cancer (time off work for a spouse-caregiver, travel, dietary changes, home care) can exceed \$20,000 in the first year alone.

Hospital Indemnity Insurance

Hospital indemnity plans pay a daily, weekly, or per-admission cash benefit when you're admitted to the hospital. A typical plan might pay \$100 to \$500 per day of hospitalization, or a lump sum of \$1,000 to \$3,000 per hospital admission.

Remember: even with Medigap, a hospital stay means time away from normal life. Bills don't stop because you're in a hospital bed. A hospital indemnity plan gives you cash in hand to cover those bills.

Accident Insurance

Accident insurance pays benefits when you're injured in an accident. This can include falls (the leading cause of injury for adults over 65), car accidents, burns, and other covered injuries. Benefits typically include lump sums for fractures, dislocations, ER visits, ambulance rides, and follow-up care.

According to the CDC, one in four Americans aged 65 and older falls each year. Falls are the leading cause of injury-related death for seniors. An accident plan can provide \$1,000 to \$10,000 or more depending on the severity of the injury.

Critical Illness Insurance

Critical illness insurance pays a lump sum when you're diagnosed with a qualifying condition. Covered conditions typically include heart attack, stroke, cancer, kidney failure, major organ transplant, and sometimes conditions like Alzheimer's, Parkinson's, or blindness.

Lump-sum payments range from \$5,000 to \$100,000. This money can be used for anything— medical expenses, household bills, a vacation with your grandchildren before starting treatment, or simply building a financial cushion.

How Much Do These Plans Cost?

Plan Type	Typical Monthly Premium (Age 65+)	Typical Benefit Amount
Cancer Insurance	\$25-\$80	\$10,000-\$50,000 lump sum
Hospital Indemnity	\$20-\$60	\$100-\$500/day or \$1,000-
Accident Insurance	\$15-\$40	\$3,000/admission \$1,000-\$10,000 per event
Critical Illness	\$30-\$100	\$5,000-\$100,000 lump sum

These premiums are modest compared to the financial protection they provide. For \$50 to \$150 per month, you can build a safety net that covers the gaps Medicare can’t touch.

Who Needs Indemnity Plans?

Not everyone needs every type of indemnity plan. But most retirees should consider at least one or two. Here’s a quick guide:

You should strongly consider cancer insurance if you have a family history of cancer or if your household depends on two incomes (even in retirement).

You should strongly consider hospital indemnity if you have chronic conditions that could require hospitalization (heart disease, diabetes, COPD) or if you want cash to cover household costs during a hospital stay.

You should strongly consider accident insurance if you’re active, live alone, or have balance or mobility concerns. Falls happen, and they’re expensive.

REAL-WORLD EXAMPLE: How a \$40/Month Plan Saved a Family

Betty, age 69, had a Medicare Advantage plan and a \$40/month hospital indemnity plan that paid \$250/day for hospital stays. When she had a stroke and spent 12 days in the hospital followed by 30 days in a skilled nursing facility, her Medicare Advantage plan covered the medical costs. But her hospital indemnity plan paid her \$3,000 (12 days x \$250) directly. That money covered her utility bills, her husband's travel to and from the hospital, and a home aide for the first two weeks after she came home. Without that plan, her husband would have had to dip into their retirement savings—savings they'd need for the years ahead.

FREQUENTLY ASKED QUESTIONS

Q: Can I buy indemnity plans if I have pre-existing conditions?

A: It depends on the plan and the insurer. Some plans have simplified underwriting with limited health questions. Others are guaranteed issue (no health questions) but may have waiting periods for pre-existing conditions. A licensed agent can help you find the right plan for your situation.

Q: Are indemnity plan payouts taxable?

A: In most cases, benefits paid from personally-owned indemnity plans are received tax-free. Consult a tax advisor for your specific situation.

Q: Do indemnity plans replace Medigap or Medicare Advantage?

A: No. Indemnity plans supplement your existing Medicare coverage. They don't pay medical bills—they pay you directly for the non-medical costs that accompany a health event.

Q: Can I buy these plans at any time?

A: In many cases, yes. Unlike Medigap, indemnity plans don't have the same strict enrollment windows. However, premiums may be higher if you're older or have health conditions, and some plans have waiting periods.

CHAPTER 10 CHECKLIST

☐ I understand what indemnity insurance is and how it differs from medical insurance ☐ I've considered whether cancer, hospital, accident, or critical illness plans are appropriate for me

☐ I've reviewed my family health history and assessed my risk factors ☐ I've evaluated how my household would handle non-medical costs during a health crisis ☐ I've spoken with a licensed agent about my supplemental insurance options > 
READY TO TAKE THE NEXT STEP? You don't have to figure this out alone. I've helped thousands of people navigate these exact decisions, and I'd be honored to help you, too. Call or text me for a free, no-obligation Retirement Blueprint Session . No sales pitch. Just straight answers from someone who's been there.

Darin Weidauer | ECOS | 702-706-6564 |
darinweidauer@ecos.care

PART TWO

PART II — IRMAA, PENALTIES, AND COSTLY MISTAKES

The mistakes in this section are permanent, expensive, and shockingly common. Every one of them is preventable.

CHAPTER 11

Chapter 11: IRMAA

The Hidden Medicare Tax Nobody Warned You About

Of all the surprises lurking in retirement, IRMAA might be the most infuriating. Not because it's complicated—once you understand it, the math is straightforward. It's infuriating because almost nobody tells you about it until it's too late.

IRMAA stands for Income-Related Monthly Adjustment Amount. In plain English, it's a surcharge—an extra amount added to your Medicare Part B and Part D premiums—based on your income. The higher your income, the more you pay. And the income Medicare looks at isn't this year's income. It's your income from two years ago.

That two-year lookback is where the trap lives.

How IRMAA Works

Every year, the Social Security Administration reviews your tax return from two years prior. In 2026, they're looking at your 2024 Modified Adjusted Gross Income (MAGI). Your MAGI is your Adjusted Gross Income (AGI) plus any tax-exempt interest income (like municipal bond interest).

If your MAGI exceeds certain thresholds, you pay more for Part B and Part D. The surcharges are applied per person—so if you're married and both on Medicare, both of you could be hit with higher premiums.

Here's what makes IRMAA especially painful: the thresholds are cliff-based, not gradual. If your income is \$109,000, you pay the standard Part B premium of \$202.90 per month. If your income is \$109,001—one dollar more—you jump to \$284.10 per month. That's an extra \$974.40 per year because of a single dollar.

□ **PLANNING AHEAD:** Your 2027 IRMAA will be based on your 2025 tax return, and your 2028 IRMAA on your 2026 return — so income decisions you make this

year echo two years forward. The first four bracket thresholds adjust with inflation each year; the top (\$500,000/\$750,000) bracket is frozen and first becomes eligible for indexing in 2028. (Source: SSA/CMS.)

2026 IRMAA Brackets: Part B

Filing Status: Single	Filing Status: Married Filing Jointly	Part B Monthly Premium (2026)
\$109,000 or less	\$218,000 or less	\$202.90 (standard)
\$109,001 – \$137,000	\$218,001 – \$274,000	\$284.10
\$137,001 – \$171,000	\$274,001 – \$342,000	\$405.80
\$171,001 – \$205,000	\$342,001 – \$410,000	\$527.50
\$205,001 – \$500,000	\$410,001 – \$750,000	\$649.20
Above \$500,000	Above \$750,000	\$689.90

2026 IRMAA Brackets: Part D

Part D has its own separate surcharge, layered on top of whatever your Part D plan charges.

Filing Status: Single	Filing Status: Married Filing Jointly	Part D Monthly Surcharge (2026)
\$109,000 or less	\$218,000 or less	\$0.00
\$109,001 – \$137,000	\$218,001 – \$274,000	\$14.50
\$137,001 – \$171,000	\$274,001 – \$342,000	\$37.50
\$171,001 – \$205,000	\$342,001 – \$410,000	\$60.40
\$205,001 – \$500,000	\$410,001 – \$750,000	\$83.30
Above \$500,000	Above \$750,000	\$91.00

The Total IRMAA Damage: A Married Couple

Let’s see what IRMAA actually costs when you add Part B and Part D together for two people.

Income Tier	Extra Per		
(Married Filing Jointly)	Person/Month (B + D)	Extra Per Couple/Month	Extra Per Couple/Year
\$218,000 or less	\$0	\$0	\$0
\$218,001 – \$274,000	\$95.70	\$191.40	\$2,296.80
\$274,001 – \$342,000	\$240.40	\$480.80	\$5,769.60

Income Tier	Extra Per		
\$342,001 - \$410,000	\$385.00	\$770.00	\$9,240.00
\$410,001 - \$750,000	\$529.60	\$1,059.20	\$12,710.40
Above \$750,000	\$578.00	\$1,156.00	\$13,872.00

At the highest tier, a married couple pays an extra \$13,872 per year in IRMAA surcharges alone —on top of their standard premiums. Over a 20-year retirement, that’s more than \$277,000 in additional Medicare costs.

And remember: IRMAA surcharges are not deductible. They’re not refundable. They’re simply extra money out of your pocket.

The Two-Year Lookback: Why Timing Is Everything

IRMAA uses a two-year lookback. In 2026, Social Security looks at your 2024 tax return. In 2027, they look at 2025. This creates a dangerous delay between the income event and the premium impact.

Here’s why that matters: a one-time income spike—selling a property, taking a large IRA distribution, doing a Roth conversion, exercising stock options, or receiving a lump-sum pension payout—can push you into a higher IRMAA bracket two years later, even if your regular income is modest.

And the surcharge applies for the entire year. It doesn’t matter that the income spike was a one-time event. You pay the higher premium for 12 months.

Common IRMAA Triggers

These are the most common income events that trigger unexpected IRMAA surcharges:

Selling a home with significant capital gains (even with the \$250K/\$500K exclusion, the gain may push you over a threshold). Taking a large Required Minimum Distribution (RMD) from a traditional IRA or 401(k). Doing a Roth conversion. Exercising stock options or receiving a large bonus in the year before retirement. Receiving a lump-sum pension payout. Selling a business or investment property. Realizing capital gains from stock sales. Receiving a large inheritance

that generates taxable income. Municipal bond interest (tax-free for income tax but counts for IRMAA).

 **** CRITICAL WARNING: MUNICIPAL BOND INTEREST COUNTS****

Many retirees invest in municipal bonds because the interest is “tax-free.” And it is—for federal income tax purposes. But municipal bond interest IS included in your MAGI for IRMAA calculations. I’ve seen retirees with modest taxable income get hit with IRMAA surcharges because their \$80,000 in muni bond interest pushed them over the threshold. “Tax-free” does not mean “IRMAA-free.”

How to Reduce or Avoid IRMAA

IRMAA is not inevitable. With proper planning, you can manage your income to stay below the thresholds—or minimize the time you spend in higher brackets. Here are the key strategies:

Strategy 1: Roth Conversions Before Medicare

Roth IRA distributions are not included in MAGI. If you convert traditional IRA funds to a Roth before age 63 (so the income hits your tax return before the IRMAA lookback period matters), you can reduce future RMDs and future MAGI—potentially keeping yourself below IRMAA thresholds for life. The conversion itself will increase your income in the year you do it, so timing and amounts matter. This strategy works best between ages 59½ and 63.

Strategy 2: Manage the Timing of Income Events

If you’re planning to sell a home, take a large distribution, or realize capital gains, consider the IRMAA impact two years later. Can you split the income across two tax years? Can you time the event so it falls in a year where your other income is lower? Can you offset the gain with deductions, losses, or charitable contributions?

Strategy 3: Qualified Charitable Distributions (QCDs)

If you’re 70½ or older, you can donate up to \$111,000 per year (2026 limit, indexed for inflation) directly from your traditional

IRA ified chari is satisfies your RMD without adding the distribution to your taxable income—and without adding it to your MAGI for IRMAA purposes. It’s one of the most powerful IRMAA-reduction tools available.

Strategy 4: Appeal a Life-Changing Event

If your income was unusually high two years ago due to a “life-changing event,” you can file Form SSA-44 to request that Social Security use a more recent year’s income instead.

Qualifying

life-changing events include: marriage, divorce, death of a spouse, work stoppage (retirement), work reduction, loss of income-producing property (due to disaster or other event beyond your control), and loss of pension income.

This is an underused tool. If your 2024 income was inflated by a one-time event but your 2025 or projected 2026 income is lower, filing SSA-44 could save you thousands in IRMAA surcharges.

Strategy 5: Income Layering

Work with a financial advisor to create a “retirement income layer cake” that draws from different sources in the right proportions each year: some from Roth (not counted in MAGI), some from taxable accounts (where you control the timing of gains), and some from Social Security and pensions (which are partially or fully included in MAGI). The goal is to keep your total MAGI just below the nearest IRMAA threshold every year.

REAL-WORLD EXAMPLE: The \$11,000 IRMAA Surprise

Richard and Maria, both 68, have a comfortable retirement. Their combined Social Security is \$62,000. Richard’s pension is \$48,000. They take \$40,000 from their traditional IRAs each year. Maria also earns \$15,000 in municipal bond interest from a portfolio she inherited. Their total MAGI: $\$62,000 + \$48,000 + \$40,000 + \$15,000 = \$165,000$. That’s well under the \$218,000 married threshold.

But then Richard’s financial advisor suggested a \$120,000 Roth conversion to “save on taxes later.” Great idea in theory. But

that conversion pushed their 2024 MAGI to \$285,000—squarely into the second IRMAA bracket. Two years later, in 2026, both Richard and Maria are paying \$405.80/month for Part B (instead of \$202.90) and \$37.50/month in Part D surcharges. Total extra cost for the year: the Part B surcharge is $\$405.80 - \$202.90 = \$202.90$ per person per month, plus \$37.50 for Part D. $(\$202.90 + \$37.50) \times 2 \text{ people} \times 12 \text{ months} = \$5,769.60$. The Roth conversion may still have been a good long-term move—but nobody warned Richard and Maria about the \$5,770 IRMAA bill. Proper planning would have spread the conversion over multiple years to stay below the threshold.

FREQUENTLY ASKED QUESTIONS

Q: Is IRMAA a permanent surcharge?

A: No. IRMAA is recalculated every year based on your tax return from two years prior. If your income drops, your IRMAA surcharge drops—or disappears entirely—the following year. But it does apply for the full year once it's determined.

Q: Does IRMAA apply to Medicare Advantage plans too?

A: Yes. IRMAA affects your Part B premium regardless of whether you're on Original Medicare or Medicare Advantage. And Part D IRMAA applies to any Part D plan, whether standalone or bundled into an Advantage plan.

Q: Can I deduct IRMAA surcharges on my taxes?

A: IRMAA surcharges are considered Medicare premiums, and Medicare premiums may be deductible as medical expenses on Schedule A—but only if your total medical expenses exceed 7.5% of your AGI. For most people, this threshold is difficult to meet.

Q: My income was high two years ago because I sold a rental property. Can I appeal?

A: Loss of income-producing property can qualify as a life-changing event for SSA-44 purposes, but a voluntary sale may not qualify. Consult with a licensed agent or financial advisor to determine if your specific situation qualifies.

Q: Does my spouse's income affect my IRMAA if we file jointly?

A: Yes. IRMAA uses your joint MAGI if you file a joint return. Even if only one spouse has high income, both spouses pay the surcharge if both are on Medicare.

CHAPTER 11 CHECKLIST

- ☐ I understand what IRMAA is and how the two-year lookback works
 - ☐ I've reviewed my 2024 tax return to estimate my 2026 IRMAA exposure
 - ☐ I know the current IRMAA thresholds for both Part B and Part D
 - ☐ I've identified any upcoming income events that could trigger IRMAA (home sale, Roth conversion, RMDs)
 - ☐ I've considered Roth conversions before age 63 to reduce future IRMAA
 - ☐ I know about Form SSA-44 and life-changing event appeals
 - ☐ I've evaluated whether Qualified Charitable Distributions can lower my MAGI
 - ☐ I've considered working with a professional to build an IRMAA-aware income strategy
- READY TO TAKE THE NEXT STEP?

IRMAA alone can cost you tens of thousands of dollars over the course of your retirement. But it doesn't have to. I help clients build income strategies that keep them below IRMAA thresholds—legally, ethically, and effectively. Let's talk about your numbers. No pressure. Just clarity.

Darin Weidauer | 702-706-6564 | darinweidauer@ecos.care

CHAPTER 12

Chapter 12: Medicare Penalties

Late Enrollment Mistakes That Last a Lifetime

I want you to imagine a financial mistake so severe that it follows you every single month for the rest of your life. Not a one-time fee. Not a fine you pay and forget. A permanent increase to a bill you'll pay every month until the day you die.

That's what Medicare late enrollment penalties are. And they're more common than you'd think.

Every year, I meet smart, responsible people who made one incorrect assumption about Medicare enrollment—and now they're paying for it. Permanently. This chapter exists to make sure you never join that group.

The Part A Late Enrollment Penalty

Most people get Part A for free because they or their spouse paid Medicare payroll taxes for 40+ quarters. But if you don't qualify for premium-free Part A and you don't sign up during your Initial Enrollment Period, you face a penalty.

The Part A penalty is 10% of the premium, and it lasts for twice the number of years you could have had Part A but didn't. For example, if you went two years without Part A, you'd pay a 10% penalty for four years.

Since the 2026 Part A premium for people with fewer than 30 quarters is \$565/month, a 10% penalty adds \$56.50/month. While this penalty eventually expires, the years of overpayment add up.

The good news: most Americans are eligible for premium-free Part A, so this penalty is relatively uncommon. The real killers are the Part B and Part D penalties.

The Part B Late Enrollment Penalty: The Big One

This is the penalty that devastates retirement budgets. Here's how it works:

For every 12-month period you were eligible for Part B but didn't sign up (and didn't have creditable coverage), your Part B premium increases by 10%. This penalty is permanent. It never goes away. It compounds with every premium increase for the rest of your life.

 ** THE MATH THAT SHOULD SCARE YOU**

2026 standard Part B premium: \$202.90/month. If you were 2 years late: 20% penalty = \$40.58 extra/month = \$487/year = \$9,739 over 20 years. If you were 3 years late: 30% penalty = \$60.87 extra/month = \$730/year = \$14,609 over 20 years. If you were 5 years late: 50% penalty = \$101.45 extra/month = \$1,217/year = \$24,348 over 20 years. And these numbers grow every year as the base premium increases.

Years Late	Penalty %	2026 Monthly Penalty	Annual Extra Cost	20-Year Cost (estimated)
1 year	10%	\$20.29	\$243	\$4,870+
2 years	20%	\$40.58	\$487	\$9,739+
3 years	30%	\$60.87	\$730	\$14,609+

Years Late	Penalty %	2026 Monthly Penalty	Annual Extra Cost	20-Year Cost (estimated)
5 years	50%	\$101.45	\$1,217	\$24,348+
7 years	70%	\$142.03	\$1,704	\$34,087+
10 years	100%	\$202.90	\$2,435	\$48,696+

Let me put that last line in perspective: if you delayed Part B by 10 years with no creditable coverage, you'd pay double the standard premium for life. At 2026 rates, that's \$405.80/month just for Part B. And the base premium will continue to rise in future years, making the dollar amount of the penalty even larger.

The longer you wait, the more it costs — for life. Cumulative cost of the Part B late-enrollment penalty at 2026 rates.

The Most Common Part B Penalty Mistakes

In my 22 years of helping people with Medicare, these are the mistakes I see most often:

Mistake #1: Assuming COBRA Is Creditable Coverage

COBRA is a continuation of your employer's group plan after you leave your job. It's valuable for bridging gaps in coverage. But COBRA is NOT creditable coverage for Medicare purposes. If you turn 65 on COBRA and don't enroll in Part B, you will face a penalty when your COBRA ends.

This is the single most common Part B penalty mistake I encounter. HR departments frequently fail to explain this distinction. If you're on COBRA at 65, enroll in Part B immediately.

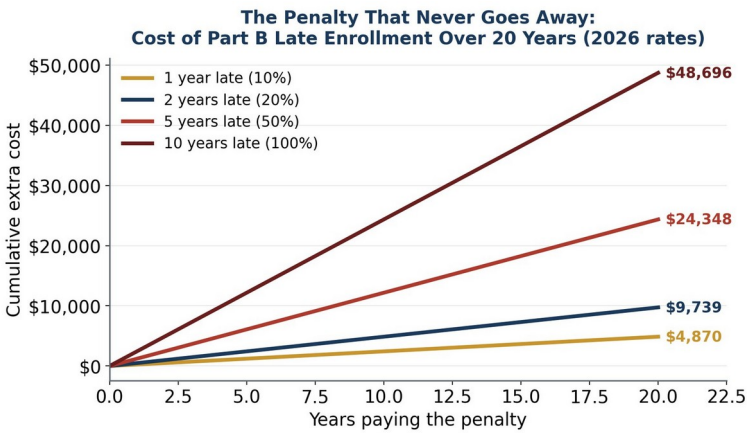


Illustration based on the 2026 standard Part B premium of \$202.90 (CMS). Penalties are 10% per full 12-month period missed, for life, and grow as the base premium rises each year.

Cumulative cost of the Part B late-enrollment penalty.

Mistake #2: Thinking Retiree Insurance Protects You

Some former employers offer retiree health insurance. In many cases, this coverage becomes secondary to Medicare once you turn 65—meaning Medicare is expected to pay first. If you don't have Part B, your retiree plan may pay nothing, or it may pay at a reduced rate, leaving you with enormous bills.

Always check with your former employer's benefits department: "Is my retiree coverage creditable for Medicare purposes?" Get the answer in writing.

Mistake #3: Working for a Small Employer

If you’re 65 and working for an employer with fewer than 20 employees, Medicare becomes your primary insurance. Your employer’s group plan becomes secondary. If you don’t enroll in Part B, you could face gaps in coverage AND a penalty.

The 20-employee threshold is critical. At 20+ employees, your employer plan is primary and Part B can be delayed without penalty. Below 20, Medicare is primary and Part B enrollment is essential.

Mistake #4: Assuming VA Coverage Counts

Veterans Affairs healthcare is excellent. But VA coverage is NOT creditable coverage for Medicare Part B purposes. If you rely solely on VA care and don’t enroll in Part B at 65, you will face a penalty when you eventually sign up. Most veterans benefit from having both VA coverage and Medicare.

Mistake #5: Not Knowing About the Special Enrollment Period

If you’re working past 65 with creditable employer coverage (20+ employees), you have an 8-month Special Enrollment Period to sign up for Part B after you or your spouse stops working, or the coverage ends—whichever comes first. Miss this 8-month window, and you’ll have to wait for the General Enrollment Period (January 1–March 31) with coverage not starting until July 1—plus a potential penalty.

The Part D Late Enrollment Penalty

Part D has its own penalty, separate from Part B. If you go 63 or more consecutive days without creditable prescription drug coverage after you’re first eligible, you’ll pay a penalty.

The calculation: 1% of the “national base beneficiary premium” (\$38.99 in 2026) multiplied by the number of full uncovered months. This penalty is added to your Part D premium permanently.

Months Without			
Creditable Drug		Monthly Penalty	
Coverage	(2026)	Annual Penalty	20-Year Cost

Months Without			
12 months	\$4.41	\$52.92	\$1,058
24 months	\$8.83	\$105.96	\$2,119

Months Without			
Creditable Drug Coverage	Monthly Penalty		
	(2026)	Annual Penalty	20-Year Cost
36 months	\$13.24	\$158.88	\$3,178
60 months (5 years)	\$22.07	\$264.84	\$5,297

The Part D penalty is smaller in dollar terms than the Part B penalty, but it still adds up over a long retirement. And like Part B, it’s permanent.

How to Avoid All Medicare Penalties

The rules are straightforward once you know them:

Enroll in Part A at 65 (it’s free for most people and there’s no reason to delay). Enroll in Part B at 65 unless you have creditable employer coverage from a company with 20+ employees—and you’re actively working there (not retired). When you stop working or lose employer coverage, enroll in Part B within 8 months. Enroll in Part D when you first become eligible, unless you have other creditable prescription drug coverage. Never assume COBRA, VA, marketplace, or retiree coverage counts as creditable without verifying. Mark every deadline on your calendar. When in doubt, call a licensed agent.

REAL-WORLD EXAMPLE: The \$31,000 Assumption

Diane, age 72, spent seven years assuming her husband’s small-employer health plan (15 employees) was keeping her safe. She thought she didn’t need Part B as long as she had “insurance.” When her husband retired at 73, Diane tried to enroll in Medicare Part B. That’s when she learned the penalty: 70% of the standard premium—permanently. At \$202.90 standard in 2026, Diane pays \$344.93 per month for Part B. Over the next 20 years, she’ll pay approximately \$34,000 more than someone who enrolled on time. One phone call to a licensed agent at age 65 would have prevented this entirely.

FREQUENTLY ASKED QUESTIONS

Q: Can Medicare penalties ever be forgiven?

A: In very rare cases, if the penalty was caused by an error by a government agency or employer, you may be able to appeal. But in the vast majority of cases, penalties are permanent. Prevention is the only reliable strategy.

Q: I have TRICARE. Do I still need Part B?

A: TRICARE for Life requires you to have both Part A and Part B. If you drop or don't enroll in Part B, TRICARE for Life will not cover you. Always maintain both parts.

Q: Does Medicare Advantage eliminate penalties?

A: No. You must have Part A and Part B to join a Medicare Advantage plan. If you have Part B penalties, those carry over to your Medicare Advantage premium structure.

Q: I turned 65 three months ago and didn't sign up. Am I in trouble?

A: Not necessarily. Your Initial Enrollment Period extends 3 months after your birthday month. Check your dates carefully—you may still be within your IEP. If not, explore whether you have any creditable coverage that would qualify you for a Special Enrollment Period.

CHAPTER 12 CHECKLIST

☐

☐ I understand the Part D late enrollment penalty (1% per month, permanent) ☐ I know that COBRA is NOT creditable coverage for Medicare ☐ I know that VA coverage is NOT creditable coverage for Part B ☐ I've verified my current coverage's creditable status in writing ☐ I know I have an 8-month SEP when employer coverage ends ☐ I've set calendar reminders for all enrollment deadlines **READY TO TAKE THE NEXT STEP?**

Penalties are permanent, but they're also preventable. If you're not sure whether you're enrolled correctly—or whether your

current coverage qualifies as “creditable”—don’t guess. One phone call can save you a lifetime of extra costs.

Darin Weidauer | 702-706-6564 | darinweidauer@ecos.care

C H A P T E R 1 3

Chapter 13: The Home Sale Trap

How Selling Your House After Age 63 Can Spike Your Medicare Costs

This is the chapter I wish every real estate agent, financial advisor, and tax preparer would read. Because the advice most retirees get about selling their home ignores one of the most expensive consequences: IRMAA.

Selling your home in retirement can be a smart financial move. You downsize. You free up equity. You simplify your life. But if you don't plan the timing carefully, the capital gain from that sale can spike your Medicare premiums for one or two years—costing you thousands of dollars in IRMAA surcharges that nobody warned you about.

Let's break down exactly how this works and, more importantly, how to avoid it.

The Capital Gains Exclusion: The Part Everyone Knows

When you sell your primary residence, the IRS allows you to exclude a portion of the capital gain from your taxable income. For a single filer, the exclusion is \$250,000. For a married couple filing jointly, it's \$500,000. To qualify, you must have owned the home and lived in it as your primary residence for at least two of the last five years.

Most people think that's the end of the story. You sell your house, take the exclusion, and move on. But here's what nobody mentions: any gain above the exclusion is taxable—and it's included in your MAGI for IRMAA purposes.

The Part Nobody Tells You: IRMAA Impact

Remember, IRMAA uses a two-year lookback. If you sell your home in 2024, the capital gain shows up on your 2024 tax return, which Medicare uses to determine your 2026 IRMAA surcharges.

Even if the gain is partially or fully excluded for income tax purposes, any portion that IS taxable gets added to your MAGI. And for many long-time homeowners—especially in high-appreciation markets like California, Nevada, Florida, Arizona, and the Northeast—the gain often exceeds the exclusion.

The Math: A Real Home Sale Scenario

Let’s walk through a realistic example.

REAL-WORLD EXAMPLE: The Henderson Home Sale

Jim and Karen, both 67, bought their home in Henderson, Nevada, in 1998 for \$185,000. Over 26 years, they made \$65,000 in capital improvements (new roof, kitchen remodel, bathroom upgrades). Their adjusted cost basis is \$250,000.

In 2024, they sell the home for \$620,000. Their capital gain is $\$620,000 - \$250,000 = \$370,000$. They apply the \$500,000 married exclusion, so the entire gain is excluded for income tax purposes. Their taxable gain from the home sale: \$0.

But now let’s change one detail. Suppose they sell for \$820,000 instead. Their capital gain is $\$820,000 - \$250,000 = \$570,000$. After the \$500,000 exclusion, they have a taxable gain of \$70,000.

Jim and Karen’s regular income (Social Security + pension + IRA withdrawals) is \$150,000. Add the \$70,000 taxable gain: their 2024 MAGI is \$220,000. That pushes them over the \$218,000 married IRMAA threshold. In 2026, both Jim and Karen pay \$284.10/month for Part B (instead of \$202.90) and \$14.50/month in Part D surcharges. Extra cost: $(\$81.20 + \$14.50) \times 2 \text{ people} \times 12 \text{ months} = \$2,296.80$ in IRMAA surcharges for that year.

The Timing Strategy: When to Sell

The IRMAA impact of a home sale depends entirely on when you sell relative to your Medicare enrollment. Here’s the timing framework:

Scenario	IRMAA Impact	Strategy
Sell before age 63	Gain hits tax return	Ideal if possible —

Scenario	IRMAA Impact	Strategy
	before IRMAA lookback period affects Medicare premiums at	sell early
Sell at age 63	65 Gain shows on tax return, affects	Risky — first year on
	IRMAA at age 65 (first year of	Medicare starts with a surcharge
Sell at age 64–65	Medicare) Gain affects IRMAA at age	Plan carefully — you’ll be on
	66–67	Medicare when the surcharge hits
Sell at age 66+	Gain affects IRMAA at age	IRMAA hit is unavoidable without other strategies
Sell and file SSA-44	68+ May reduce or eliminate IRMAA if qualifying life- changing event applies	Worth exploring — see below

Strategy 1: Sell Before Age 63

If your home sale gain will exceed the exclusion, the cleanest strategy is to sell before age 63. That way, the gain appears on your tax return two or more years before you turn 65—before you’re on Medicare and before IRMAA applies.

Of course, this isn’t always practical. Real estate decisions depend on housing markets, family needs, and life circumstances. But if you have flexibility, timing the sale before 63 eliminates the IRMAA problem entirely.

Strategy 2: Increase Your Cost Basis

Your cost basis reduces the taxable gain. Capital improvements to your home—not repairs, but improvements—increase your basis. Keep meticulous records of every improvement: new roof, HVAC system, kitchen remodel, bathroom renovation, room addition, landscaping that adds value, new driveway, new windows, and more.

Every dollar you add to your cost basis is a dollar less in capital gain. If your home has appreciated significantly, the difference

between a \$200,000 basis and a \$300,000 basis could determine whether you trigger IRMAA or not.

Strategy 3: Installment Sale

Instead of receiving the full sale price at closing, you can structure an installment sale where the buyer pays you over multiple years. This spreads the capital gain across tax years, potentially keeping your MAGI below the IRMAA threshold in any single year.

This strategy requires careful legal and tax planning, and it’s not appropriate for every situation. But for the right seller and buyer, it can save thousands in both taxes and IRMAA surcharges.

Strategy 4: Offset With Losses or Charitable Giving

Capital losses from other investments can offset capital gains from a home sale. If you have losing investments in a taxable brokerage account, harvesting those losses in the same year as the home sale can reduce your net gain.

Similarly, making large charitable contributions—especially donating appreciated stock directly to charity—can reduce your AGI, which in turn reduces your MAGI.

Strategy 5: File Form SSA-44

If the home sale qualifies as a “loss of income-producing property” (for example, if you sold a home that was also generating rental income), you may be able to file Form SSA-44 and request that Social Security use your current year’s income instead of the lookback year. This is a nuanced argument, and not every home sale qualifies—but it’s worth exploring with a licensed professional.

The Home Sale IRMAA Trap: A Complete Illustration

Year	Event	MAGI Impact	IRMAA Result
2024	Jim & Karen sell home;	MAGI rises from	No immediate effect
	\$70,000 taxable gain	\$150,000 to \$220,000	(2024 tax year)

Year	Event	MAGI Impact	IRMAA Result
2025	Normal income year	MAGI: \$150,000	2025 IRMAA based
on 2023 taxes (below threshold)			
2026 IRMAA lookback hits	SSA sees \$220,000	Both pay extra	
2024 return	MAGI	\$95.70/mo each =	
2027 IRMAA lookback hits	SSA sees \$150,000	\$2,296.80/year Back to standard premiums	
2025 return	MAGI		

One home sale. One year of elevated income. One year of IRMAA surcharges. \$2,296.80 in extra Medicare costs. And that’s for a relatively modest \$70,000 in taxable gain. For larger gains—especially in high-cost real estate markets—the IRMAA hit can be \$5,000, \$8,000, or even \$12,000.

FREQUENTLY ASKED QUESTIONS

- Q: Does the \$250K/\$500K exclusion fully protect me from IRMAA?
- A: Only if your total gain is below the exclusion amount. Any gain above the exclusion is taxable and counts toward IRMAA. Even within the exclusion, other income sources can still push you over an IRMAA threshold.
- Q: I’m selling a rental property, not my primary home. Does the exclusion apply?
- A: No. The \$250K/\$500K capital gains exclusion only applies to your primary residence. Gains from rental or investment properties are fully taxable and fully counted in your MAGI. Consider a 1031 exchange to defer the gain if you plan to reinvest in another rental property.
- Q: My real estate agent never mentioned IRMAA. Is this common?
- A: Extremely common. Most real estate agents don’t know about IRMAA. Most tax preparers don’t proactively plan for it. This is a gap in the advice most retirees receive, which is why

working with a retirement specialist who understands the full picture is so important.

Q: Can I avoid the home sale trap by putting the proceeds in a Roth IRA?

A: Not directly. The capital gain is triggered by the sale itself, not by what you do with the proceeds. However, if you use some of the proceeds to fund Roth conversions in future years (at a pace that avoids IRMAA), you can build tax-free income for the future.

CHAPTER 13 CHECKLIST

☐ I understand how a home sale can trigger IRMAA surcharges via the two-year lookback ☐ I know the capital gains exclusion amounts (\$250K single / \$500K married) ☐ I've calculated my home's approximate cost basis, including improvements ☐ I've estimated my potential taxable gain if I sell ☐ I've considered the optimal timing for a home sale relative to my Medicare enrollment ☐ I've explored strategies like installment sales, loss harvesting, and charitable giving ☐

READY TO TAKE THE NEXT STEP?

Selling your home is one of the biggest financial events of your retirement. The IRMAA implications alone can cost you thousands. Before you list that house, let's make sure the timing is right. I've helped dozens of clients time their home sales to minimize Medicare surcharges.

Darin Weidauer | 702-706-6564 | darinweidauer@ecos.care

C H A P T E R 1 4

Chapter 14: Roth Conversions, IRA Withdrawals, and IRMAA

Planning Your Income to Avoid the Surcharge

If Chapter 11 showed you what IRMAA is and Chapter 13 showed you how a home sale can trigger it, this chapter is about the income sources you control every day—and how managing them strategically can keep thousands of dollars in your pocket instead of handing them to Medicare.

Your traditional IRA, 401(k), 403(b), and TSP are retirement income powerhouses. But every dollar you withdraw from these accounts shows up on your tax return—and every dollar on your tax return feeds into your MAGI, which feeds into IRMAA.

The question isn’t whether to use these accounts. The question is how much to take, when to take it, and whether converting some of it to a Roth can change the game entirely.

Understanding the MAGI Equation

Your Modified Adjusted Gross Income (MAGI) for IRMAA includes:

Adjusted Gross Income (your total income minus specific deductions like educator expenses, student loan interest, and IRA contributions) PLUS tax-exempt interest income (including municipal bond interest).

Here’s what counts in your AGI:

Income Source	Included in MAGI?	Notes
Social Security benefits	Yes (up to 85%)	Taxable portion counts toward
Traditional IRA/401(k)/403(b) withdrawals	Yes (100%)	AGI Fully taxable as ordinary income
Roth IRA distributions	NO	Not included in AGI or MAGI
Roth 401(k) distributions	NO	Not included in AGI or MAGI
Pension income	Yes (100%)	Fully taxable

Income Source	Included in MAGI?	Notes
Wages / self-employment income	Yes (100%)	Fully taxable
Capital gains (stocks, property)	Yes (100%)	Short-term and long-term gains both count
Rental income (net)	Yes	After deductions
Municipal bond interest	Yes (for IRMAA only)	Tax-free for income tax but counts for
Dividends	Yes	MAGI Both qualified and ordinary
Annuity income (non-qualified)	Partially	Only the earnings portion is taxable
Life insurance cash value withdrawals	It depends	Loans are tax-free; withdrawals above basis are
Required Minimum Distributions	Yes (100%)	taxable income Fully taxable from traditional accounts
Health Savings Account withdrawals	No (if qualified)	Tax-free for medical expenses

The golden takeaway from this table: Roth IRA distributions do not count in your MAGI. That single fact makes Roth conversions one of the most powerful IRMAA avoidance strategies available.

The Roth Conversion Strategy for IRMAA Reduction

A Roth conversion is when you move money from a traditional IRA, 401(k), or other pre-tax retirement account into a Roth IRA. You pay income tax on the converted amount in the year you convert. But after that, the money grows tax-free, and qualified distributions from the Roth are not included in your MAGI.

Here’s the strategic framework:

Phase 1: The Conversion Window (Ages 59½ to 63)

The ideal time for Roth conversions is between ages 59½ (when you can access retirement funds without the 10% early withdrawal penalty) and age 63. Why age 63? Because IRMAA uses a two-year lookback. A conversion done at age 63 affects your IRMAA at age 65—your first year on Medicare. If you complete your conversions by age 62, the income from those

conversions may not affect your IRMAA at all (depending on when in the year you turn 65 and when the lookback lands).

The goal: convert enough traditional IRA money to a Roth to significantly reduce your future RMDs, lower your future MAGI, and keep you below IRMAA thresholds for life—all while paying tax at today’s rates, which may be lower than future rates.

Phase 2: The Careful Zone (Ages 63 to 70)

After age 63, conversions still work—but you need to be more precise. Every conversion increases your MAGI in the conversion year, which will increase your IRMAA two years later. The strategy shifts to “precision conversions”: convert just enough to fill the space between your current MAGI and the next IRMAA threshold, but not so much that you jump into a higher bracket.

Phase 3: The RMD Years (Age 73+)

Once Required Minimum Distributions begin (age 73 for most people in 2026), your traditional IRA forces money into your MAGI whether you want it there or not. If you’ve done significant Roth conversions in earlier years, your remaining traditional IRA balance is smaller, your RMDs are smaller, and your MAGI stays lower. That’s the payoff of early planning.

Roth Conversion IRMAA Impact Calculator

Let’s see how different conversion amounts affect IRMAA for a married couple with \$150,000 in base income.

Roth Conversion Amount	Resulting MAGI (Base \$150K + Conversion)	IRMAA Bracket Hit	Annual Extra Cost (Couple)
\$0 (no conversion)	\$150,000	Standard (\$202.90/mo)	\$0
\$50,000	\$200,000	Standard (\$202.90/mo)	\$0
\$68,000	\$218,000	Standard (\$202.90/mo)	\$0 (right at threshold)
\$68,001	\$218,001	\$284.10/mo bracket	\$2,296.80
\$100,000	\$250,000	\$284.10/mo	\$2,296.80

Roth Conversion Amount	Resulting MAGI (Base \$150K + Conversion)	IRMAA Bracket Hit	Annual Extra Cost (Couple)
		bracket	
\$124,000	\$274,000	\$284.10/mo bracket	\$2,296.80 (right at threshold)
\$124,001	\$274,001	\$405.80/mo bracket	\$5,769.60
\$200,000	\$350,000	\$527.50/mo bracket	\$9,240.00

Look at the difference between converting \$68,000 and \$68,001 for this couple. One extra dollar triggers \$2,296.80 in annual IRMAA surcharges. That’s not a gradual increase—it’s a cliff. This is why precision matters.

The optimal strategy for this couple: convert up to \$68,000 per year (bringing their MAGI to exactly \$218,000), filling the space just below the IRMAA threshold. Over five years, they could convert \$340,000 to a Roth without triggering any IRMAA—and dramatically reduce their future RMDs.

Traditional IRA Withdrawals: The Annual Calibration

Even if you’re not doing Roth conversions, the amount you withdraw from traditional accounts each year should be carefully calibrated against IRMAA thresholds. Here’s a simple annual planning process:

Step 1: Estimate your “fixed” MAGI components for the year (Social Security, pension, wages, rental income, etc.). Step 2: Identify the nearest IRMAA threshold above your fixed income. Step 3: The gap between your fixed income and the threshold is your “safe withdrawal zone” from traditional accounts. Step 4: If you need more income than the safe zone allows, draw the excess from Roth accounts, cash savings, or other non-MAGI sources.

The Power of Multiple Income Buckets

The most IRMAA-efficient retirees don’t rely on a single income source. They maintain multiple “buckets” of money, each with different tax treatment:

Bucket	Examples	Taxed?	Counts in MAGI?	IRMAA Impact
Pre-Tax	Traditional IRA, 401(k), 403(b),	Yes (100%)	Yes	High
Tax-Free	TSP Roth IRA, Roth	No	No	None
After-Tax	401(k) Brokerage accounts,	Gains/ dividends only	Yes (gains)	Moderate
Tax- Advantaged	HSA, cash value life insurance	No (if rules followed)	Usually no	Low/None
Social Security	SS benefits	Up to 85%	Yes (taxable	Moderate

portion)

By drawing from different buckets in the right proportions each year, you can control your MAGI with remarkable precision. This is the essence of retirement income planning—and it’s why working with a professional who understands both taxes and Medicare is worth its weight in gold.

RMDs and IRMAA: The Forced Income Problem

Starting at age 73, the IRS requires you to take minimum distributions from your traditional retirement accounts (IRA, 401(k), 403(b), TSP). These Required Minimum Distributions are based on your account balance and life expectancy. The larger your traditional account balance, the larger your RMD—and the higher your MAGI.

Here’s the problem: by the time RMDs begin, you have no choice about the income. It’s forced. If your traditional IRA has grown to \$1,000,000 by age 73, your first RMD is approximately \$37,736 (using the Uniform Lifetime Table). At \$2,000,000, it’s about \$75,472. At \$3,000,000, it’s roughly \$113,208.

If those RMDs push you over an IRMAA threshold, you’re paying surcharges on money you were forced to take. This is the “RMD-IRMAA trap,” and the only way to avoid it is to reduce your traditional account balances before RMDs begin—through Roth conversions, strategic withdrawals, or qualified charitable distributions.

Traditional IRA	Approximate First	If Combined With	IRMAA Result
Balance at Age 73	RMD	\$80K Other Income	(Single)
\$300,000	\$11,321	\$91,321	Standard
\$500,000	\$18,868	\$98,868	Standard
\$900,000	\$33,962	\$113,962	\$284.10/mo bracket
\$1,000,000	\$37,736	\$117,736	\$284.10/mo bracket
\$1,500,000	\$56,604	\$136,604	\$405.80/mo bracket
\$2,000,000	\$75,472	\$155,472	\$405.80/mo bracket

Traditional IRA	Approximate First	If Combined With	IRMAA Result
Balance at Age 73	RMD	\$80K Other Income	(Single)
\$3,000,000	\$113,208	\$193,208	\$527.50/mo bracket

A single person with \$80,000 in other income and a \$900,000 traditional IRA would pay \$284.10/month for Part B instead of \$202.90/month—an extra \$974 per year—solely because of their RMD. Over 20 years, that’s nearly \$19,500 in extra Medicare costs from one account they couldn’t avoid tapping.

The Optimal Strategy: Putting It All Together

Here’s the integrated approach I recommend to my clients:

Before age 63: Do aggressive (but tax-smart) Roth conversions, filling up the space below IRMAA thresholds each year. The goal is to move as much as possible from traditional accounts to Roth while paying tax at today’s rates.

Ages 63–72: Continue precision Roth conversions calibrated to stay just below IRMAA thresholds. Supplement income from Roth and taxable accounts as needed. Begin drawing down traditional accounts strategically.

Age 73+: RMDs kick in. If you’ve done your conversions, the remaining traditional balance is manageable and RMDs stay below IRMAA thresholds. Use Qualified Charitable

Distributions (QCDs) for any charitable giving to further reduce MAGI.

Throughout retirement: Review your income plan annually. IRMAA thresholds, tax brackets, and personal circumstances change. What worked last year may not work next year.

REAL-WORLD EXAMPLE: The Five-Year Conversion Plan

Tom and Sarah, both 60, have \$1,200,000 in traditional IRAs, \$80,000 in combined Social Security (projected at 67), and \$30,000 in pension income. Their projected MAGI at 67 without conversions: $\$80,000 + \$30,000 + \sim \$45,000 \text{ RMD (at 73)} = \$155,000$. But their IRA will grow. By age 73, it could be \$1,800,000, producing an RMD of approximately \$67,924—pushing their MAGI to \$177,924 — and climbing every year as RMD divisors shrink and COLA lifts their Social Security toward the \$218,000 married threshold.

Tom and Sarah's plan: between ages 60 and 64, they convert \$60,000 per year (\$300,000 total) to a Roth. They pay approximately \$15,000–\$18,000 per year in federal taxes on the conversions (at the 22–24% marginal rate). Total conversion tax cost: approximately \$75,000–\$90,000 over five years.

Result: By age 73, their traditional IRA balance is roughly \$1,100,000 instead of \$1,800,000. Their first RMD is approximately \$41,509 instead of \$67,924. Their MAGI stays below the \$218,000 married threshold. IRMAA avoided: rising RMDs and COLA would push them over the \$218,000 threshold in their late seventies; from that point the surcharge runs \$2,296.80/year and climbs as they enter higher brackets — on the order of \$30,000 or more across the balance of their retirement—plus the compounding tax-free growth in their Roth IRA, which could be worth hundreds of thousands more.

Total estimated benefit of the conversion plan: \$30,000+ in IRMAA savings + \$30,000–\$50,000 in reduced income taxes on RMDs + tax-free Roth growth. Total cost: \$75,000–\$90,000 in conversion taxes. Net benefit: potentially \$100,000+ over their retirement.

FREQUENTLY ASKED QUESTIONS

Q: Should I do all my Roth conversions at once?

A: Almost never. A large, one-time conversion creates a massive income spike that pushes you into higher tax brackets and higher IRMAA tiers. The strategy is to spread conversions over multiple years, staying within your tax bracket and below IRMAA thresholds each year.

Q: I'm already 70. Is it too late for Roth conversions?

A: It's never too late, but the strategy changes. At 70+, you have fewer years before RMDs begin (at 73), so the "runway" for conversions is shorter. You can still do precision conversions each year to reduce your traditional balance. Every dollar converted is a dollar less in future RMDs.

Q: Do Roth conversions affect my Social Security taxes too?

A: Yes. The converted amount increases your income, which can increase the taxable portion of your Social Security benefits (up to 85% of benefits can be taxable). This is another reason to do conversions before you start collecting Social Security if possible.

Q: My financial advisor never mentioned IRMAA. Should I be concerned?

A: Many financial advisors focus on investment performance and tax brackets but overlook Medicare implications. IRMAA is a relatively new consideration for many planners. If your advisor isn't factoring Medicare costs into your withdrawal strategy, you should bring it up—or work with someone who understands both sides.

Q: Can I 'undo' a Roth conversion if it pushes me into IRMAA?

A: No. Since 2018, Roth conversions are irrevocable. Once you convert, you cannot reverse it (this is called "recharacterization," and it was eliminated by the Tax Cuts and Jobs Act). This makes planning before converting even more critical.

CHAPTER 14 CHECKLIST

☐ I understand how traditional IRA withdrawals and Roth conversions affect my MAGI ☐
☐ I've calculated the gap between my current MAGI and the nearest IRMAA threshold ☐ I've evaluated whether Roth conversions before age 63 make sense for my situation ☐ I understand how RMDs at age 73 could push me into IRMAA territory ☐ I've considered building multiple income "buckets" (pre-tax, tax-free, after-tax) ☐ I've reviewed the QCD strategy for charitable giving after age 70½ ☐ I've discussed IRMAA with my financial advisor or retirement specialist **READY TO TAKE THE NEXT STEP?**

Roth conversions can be the most powerful tax tool in your retirement toolkit—or the most expensive mistake. The difference comes down to planning. Let me run the numbers with you, for free, so you can see exactly what a conversion would mean for your taxes, your Medicare premiums, and your long-term wealth.

Darin Weidauer | 702-706-6564 | darinweidauer@ecos.care

PART THREE

PART III — SOCIAL SECURITY

Social Security has over 2,700 rules. The filing decision you make

could mean a difference of \$100,000 or more over your lifetime.

CHAPTER 15

Chapter 15: Social Security Basics

How Your Benefit Is Calculated

Here's a number that might surprise you: the average Social Security retirement benefit in 2026 is approximately \$2,071 per month, or about \$24,852 per year. The maximum benefit for someone filing at Full Retirement Age in 2026 is \$4,152 per month—\$49,824 per year. And the maximum for someone who delays until age 70 is \$5,181 per month—\$62,172 per year. (Source: Social Security Administration, 2026 figures.)

The difference between the average and the maximum isn't just about how much you earned during your career. It's also about when you file, how you file, and whether you understand the formula that determines your benefit. Most people don't. And that lack of understanding costs the average retiree tens of thousands of dollars over their lifetime.

Let's change that right now.

How Social Security Calculates Your Benefit

Your Social Security retirement benefit is based on a formula with three key components: your earnings history, your Average Indexed Monthly Earnings (AIME), and your Primary Insurance Amount (PIA). Let me walk you through each one.

Step 1: Your Earnings History (35 Best Years)

Social Security looks at your entire earnings history—every year you worked and paid Social Security taxes (also called FICA taxes). But it only uses your 35 highest-earning years to calculate your benefit.

Each year's earnings are "indexed" to account for wage growth over time. This means your earnings from 1985 are adjusted upward to reflect what those wages would be worth in today's dollars. Once indexed, your 35 highest years are selected.

If you worked fewer than 35 years, Social Security fills the missing years with zeros. Every zero year drags down your

average. This is why someone who worked 30 years will have a lower benefit than someone with the same annual salary who worked 35+ years.

Key insight: if you’re still working and earning more than you did in your early career, every additional year replaces a lower-earning year (or a zero) in the calculation. Working a few extra years can meaningfully increase your benefit.

Step 2: Average Indexed Monthly Earnings (AIME)

After selecting your 35 highest indexed earnings years, Social Security adds them all up and divides by 420 (that’s 35 years x 12 months). The result is your AIME—the average monthly income that feeds into the benefit formula.

For example, if your 35 best years of indexed earnings total \$2,100,000, your AIME would be $\$2,100,000 \div 420 = \$5,000$.

Step 3: Primary Insurance Amount (PIA) — The Bend Point Formula

Your PIA is the monthly benefit you’d receive if you file at your Full Retirement Age (FRA). It’s calculated using a progressive formula with “bend points” that replace a higher percentage of lower earnings and a lower percentage of higher earnings.

The 2026 bend points are:

AIME Range	Replacement Rate	What This Means
First \$1,226	90%	Your first \$1,226/month in
		average earnings is replaced at 90 cents on the dollar
		replaced at 32 cents on the dollar
		\$7,391/month is replaced at only 15 cents on the dollar

Let’s apply the formula to our example (\$5,000 AIME):

90% of the first \$1,226 = \$1,103.40. Plus 32% of the amount between \$1,226 and \$5,000 (\$3,774) = \$1,207.68. Total PIA = \$1,103.40 + \$1,207.68 = \$2,311.08 per month.

That’s the benefit at Full Retirement Age. File earlier, and it’s reduced. File later, and it’s increased. We’ll cover the timing decision in detail in Chapter 16.

What’s Full Retirement Age (FRA)?

Your Full Retirement Age depends on when you were born. For anyone born in 1960 or later, FRA is 67. Here’s the complete table:

Year of Birth	Full Retirement Age
1943-1954	66
1955	66 and 2 months
1956	66 and 4 months
1957	66 and 6 months
1958	66 and 8 months
1959	66 and 10 months
1960 or later	67

If you’re reading this book at age 59½, your FRA is almost certainly 67. That’s the age at which you receive 100% of your PIA—not a penny more, not a penny less.

The Social Security Earnings Limit (Taxable Maximum)

There’s a cap on how much of your income is subject to Social Security taxes each year. For 2026, the maximum taxable earnings amount is \$184,500. Any income above that threshold is not subject to the 6.2% Social Security payroll tax and does not count toward your benefit calculation.

This cap is why even very high earners hit a benefit ceiling. No matter how much you earn above \$184,500, it doesn’t increase your future Social Security benefit.

The 2026 Cost-of-Living Adjustment (COLA)

Social Security benefits are adjusted each year for inflation through the Cost-of-Living Adjustment (COLA). The 2026 COLA is 2.8%. That means if your monthly benefit was \$2,000 in 2025, it increased to \$2,056 in January 2026.

Ten years of Social Security COLAs. The 2026 adjustment is 2.8%.

COLAs are cumulative—each year’s adjustment builds on the previous year’s benefit amount. Over a long retirement, COLAs can significantly increase your monthly check. This is one of Social Security’s most valuable features: it’s inflation-adjusted income for life.

How to Check Your Benefit Estimate

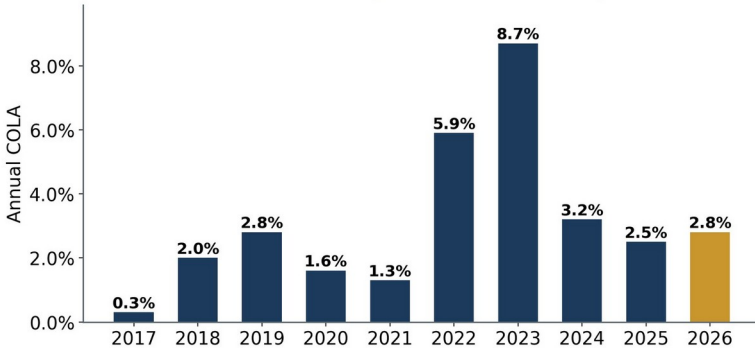
The best way to see your projected benefit is to create an account at ssa.gov and review your Social Security Statement. The statement shows your estimated benefits at age 62, Full Retirement Age, and age 70, based on your actual earnings history.

Review your statement carefully. Check that your earnings for each year are correct. If any years are missing or underreported, you could be losing benefit dollars. You have three years, three months, and 15 days from the year in question to correct an earnings record, though older corrections are sometimes possible with documentation.

REAL-WORLD EXAMPLE: The Missing Years

Frank, age 63, reviewed his Social Security statement and noticed that three years from his early career showed \$0 in earnings. He had worked those years for a small landscaping company that apparently failed to report his wages. Frank contacted SSA with old W-2 forms and tax returns. After the correction, his AIME increased by \$127 per month, and his projected PIA increased by approximately \$41 per month. Over a 25-year retirement, that correction is worth approximately \$12,300 in additional benefits. All because he checked his statement.

Social Security Cost-of-Living Adjustments, 2017-2026
The 2026 COLA is 2.8% — an average of about \$56 more per month for retirees



Source: Social Security Administration (SSA), Cost-of-Living Adjustment history and 2026 COLA Fact Sheet.

Ten years of Social Security COLAs.

FREQUENTLY ASKED QUESTIONS

Q: What if I didn't work 35 years?

A: Social Security uses zeros for missing years. Each zero year reduces your AIME and your benefit. If you're close to 35 years, working a few more years—even at a lower salary—can replace zeros and increase your benefit.

Q: Do my military years count?

A: Yes. Active-duty military service is covered by Social Security. Special credits may apply for service before 1957 and between 1957 and 2001. Check your earnings record to confirm all military years are properly credited.

Q: Does my ex-spouse's earnings record affect mine?

A: Your own benefit is calculated solely from your own earnings. However, you may be eligible for benefits based on your ex-spouse's record (see Chapter 17). Claiming on an ex-spouse's record does not reduce their benefit or their current spouse's benefit.

Q: What's the maximum Social Security benefit in 2026?

A: The maximum at FRA (67) is \$4,152/month. The maximum at age 70 is \$5,181/month. To get the maximum, you'd need to have earned at or above the taxable maximum for 35 years.

CHAPTER 15 CHECKLIST

- ☐ I've created an account at ssa.gov and reviewed my Social Security Statement
 - ☐ I've verified that all my earnings years are correctly reported
 - ☐ I understand the 35-year calculation and the impact of zero years
 - ☐ I know my Full Retirement Age (most likely 67)
 - ☐ I know the 2026 COLA is 2.8% and that benefits are adjusted for inflation
- READY TO TAKE THE NEXT STEP?

Your Social Security benefit is based on your unique earnings history and filing decision. I can run a personalized analysis that shows you exactly what you'll receive at 62, 67, and 70—and help you find the filing strategy that puts the most money in your pocket over your lifetime. It takes one phone call.

Darin Weidauer | 702-706-6564 | darinweidauer@ecos.care

CHAPTER 16

Chapter 16: When to Claim

Age 62 vs. 67 vs. 70

This is the single most consequential financial decision most Americans will ever make. Not buying a house. Not choosing a career. When you claim Social Security.

The difference between filing at 62 and waiting until 70 can be \$150,000 or more over your lifetime. And unlike most financial decisions, this one is largely irreversible. Once you start collecting, you're locked in (with very limited exceptions).

Let's walk through the math, the strategy, and the real-world implications.

The Three Key Ages

The single biggest lever you control: claiming age. 2026 maximum monthly benefits at 62, 67, and 70 (SSA).

You can start Social Security retirement benefits as early as age 62 and as late as age 70. Your Full Retirement Age (FRA) is the midpoint—67 for most readers of this book. Here's what happens at each age:

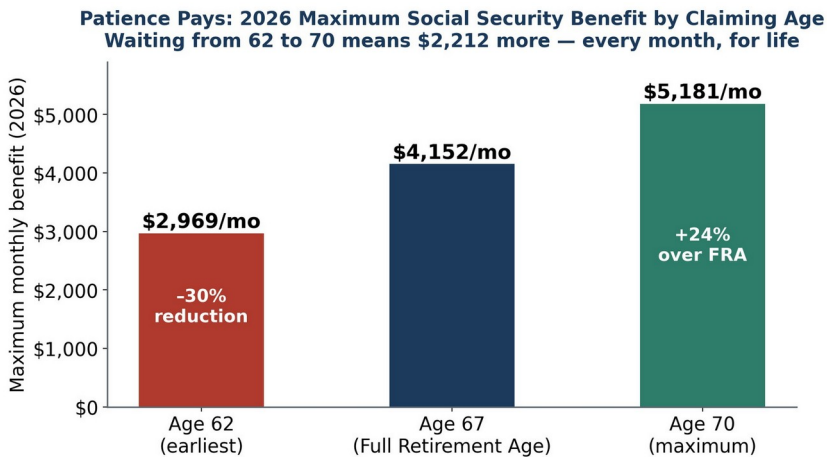
Age 62: The Early Bird Gets... a Smaller Check

If you file at 62, your benefit is permanently reduced. For someone with an FRA of 67, filing at 62 means filing 60 months early. The reduction is calculated as follows: 5/9 of 1% per month for the first 36 months early, plus 5/12 of 1% for each additional month. For a full 60 months early, the total reduction is 30%.

If your PIA (Full Retirement Age benefit) is \$2,500 per month, filing at 62 gives you \$1,750 per month. That's \$750 less—every single month, for the rest of your life.

Age 67: Full Retirement Age — 100% of Your PIA

Filing at FRA gives you exactly your Primary Insurance Amount. No reduction. No bonus. If your PIA is \$2,500, you get \$2,500.



Source: Social Security Administration (SSA), maximum benefit amounts for workers retiring in 2026.

2026 maximum monthly benefits at 62, 67 and 70.

Age 70: The Patience Premium

For every month you delay past FRA, your benefit increases by 2/3 of 1% per month—that’s 8% per year in Delayed Retirement Credits. From age 67 to 70, that’s a 24% increase.

If your PIA is \$2,500, delaying to 70 gives you \$3,100 per month. That’s \$600 more than FRA and \$1,350 more than filing at 62.

There is no benefit to waiting past age 70. Delayed Retirement Credits stop accumulating at 70.

The Filing Age Comparison Chart

Here’s how different filing ages affect your monthly benefit, assuming a PIA of \$2,500 at age 67:

Reduction/ Monthly Increase				
Filing Age	Benefit	% of PIA	Annual Benefit	vs. FRA
62	\$1,750	70%	\$21,000	-30%
63	\$1,875	75%	\$22,500	-25%

Filing Age	Benefit	% of PIA	Annual Benefit	vs. FRA
64	\$2,000	80%	\$24,000	-20%
65	\$2,167	86.7%	\$26,004	-13.3%
66	\$2,333	93.3%	\$27,996	-6.7%
67 (FRA)	\$2,500	100%	\$30,000	0%
68	\$2,700	108%	\$32,400	+8%
69	\$2,900	116%	\$34,800	+16%
70	\$3,100	124%	\$37,200	+24%

Look at the range: from \$21,000 per year at 62 to \$37,200 per year at 70. That’s a \$16,200 annual difference—and it compounds every year with COLA increases. Over 20 years of retirement, the cumulative difference between filing at 62 versus 70 can exceed \$150,000.

The Break-Even Analysis

“But Darin,” people say, “if I wait until 70, I’m giving up eight years of checks. When do I actually come out ahead?”

That’s the break-even question, and it’s a valid one. The break-even point is the age at which the total cumulative benefits from delayed filing surpass the total from early filing.

Comparison	Break-Even Age (approximate)	What This Means
Age 62 vs. Age 67	~78-79	If you live past 79, you’re

better off waiting to 70

The average life expectancy for a 65-year-old American is approximately 84 for men and 87 for women. For a healthy 65-year-old, it’s even higher. That means most people will live well past the break-even point—making delayed filing the mathematically superior choice for the majority of retirees.

But math isn’t everything. Let’s talk about when early filing makes sense.

When Filing Early (62-65) Makes Sense

Filing early is not always wrong. Here are legitimate reasons to consider it:

Health concerns: If you have a serious health condition or a family history of early mortality, collecting benefits sooner may put more total money in your pocket. The break-even analysis assumes you live long enough to recoup the delayed benefits.

You need the income now: If you've lost your job, can't find work, and have no other income sources, Social Security may be your lifeline. Survival comes first.

Your spouse will claim a survivor benefit: If you're the lower earner in a couple and your spouse plans to delay filing, your early claim provides household income while the higher earner's benefit continues to grow.

You can invest the money: Some argue that taking benefits early and investing them can produce higher returns than waiting. This is possible but risky—it depends on market performance, and Social Security's 8% annual delayed credit is a guaranteed, inflation-adjusted return. No investment offers that with zero risk.

When Delaying to 70 Makes Sense

Delaying to 70 is the strongest choice when:

You're in good health and expect to live into your mid-80s or beyond. You have other income sources (savings, pension, part-time work) to bridge the gap between retirement and age 70. You're the higher earner in a married couple (because the higher earner's benefit becomes the survivor benefit for the remaining spouse). You want the highest possible guaranteed income for life. You want to maximize the inflation-adjusted "paycheck" that Social Security provides.

The Spousal Strategy: Coordinating Your Claim

If you're married, your filing decision doesn't just affect you—it affects your spouse. When one spouse dies, the surviving spouse receives the higher of the two benefits (not both). This is the survivor benefit, and it makes the higher earner's filing decision critically important.

If the higher earner files at 62, they lock in a reduced benefit that could become the surviving spouse's only income. If the higher earner delays to 70, they lock in the maximum possible benefit—which becomes the survivor benefit, protecting the remaining spouse for life.

This is one of the most overlooked aspects of Social Security planning. We'll dive deeper into spousal and survivor benefits in Chapter 17.

REAL-WORLD EXAMPLE: The \$143,000 Decision

James and Linda are both 62. James's PIA at age 67 is \$2,800. Linda's PIA is \$1,400. If James files at 62: his monthly benefit is \$1,960. Over his lifetime (assuming he lives to 85), he collects approximately \$541,000 in total benefits. If James files at 70: his monthly benefit is \$3,472. Over the same lifetime to 85, he collects approximately \$624,000—that's \$83,000 more. But here's the bigger picture: James dies at 85. Linda, now 85 herself, receives James's benefit as a survivor. If James filed at 62, Linda's survivor benefit is \$1,960. If James filed at 70, her survivor benefit is \$3,472. Linda lives to 92. The difference in survivor benefits alone over those 7 years: $(\$3,472 - \$1,960) \times 12 \text{ months} \times 7 \text{ years} = \$126,000$. Combined with James's own higher lifetime benefits, the total advantage of delaying to 70 in this scenario: approximately \$143,000.

FREQUENTLY ASKED QUESTIONS

Q: Can I change my mind after filing?

A: You have 12 months from the date you first received benefits to withdraw your application. You'd have to repay all benefits received. After 12 months, your only option is to suspend benefits at FRA (which allows Delayed Retirement Credits to accrue from the suspension date, but not retroactively).

Q: Does the COLA apply even if I haven't started collecting?

A: Yes. Your PIA is adjusted for COLA increases every year, even before you file. So if you delay to 70, your benefit reflects all COLA increases from age 62 to 70 plus the 24% Delayed Retirement Credits.

Q: I’m self-employed. Does that change anything?

A: Self-employed individuals pay both the employee and employer portions of Social Security tax (15.3% total). Your benefits are calculated the same way as an employee’s, based on your net self-employment income.

Q: What if Social Security runs out of money?

A: The Social Security Trust Fund is projected to face shortfalls in the mid-2030s. If Congress takes no action, benefits could be reduced by approximately 17–23%. However, every major projection assumes benefits will continue—not disappear. Social Security has been modified many times in its history, and there is strong bipartisan motivation to maintain it.

CHAPTER 16 CHECKLIST

- ☐ I know my PIA and what I’d receive at ages 62, 67, and 70 ☐ I’ve considered the break-even analysis for my situation ☐ I’ve evaluated my health and life expectancy honestly ☐ If married, I’ve considered how my filing age affects my spouse’s survivor benefit ☐
- ☐ I’ve identified bridge income sources if I plan to delay filing
- READY TO TAKE THE NEXT STEP?**

The difference between the best and worst Social Security filing strategies for a married couple can exceed \$150,000. I’m a Registered Social Security Analyst, and I can run a customized analysis showing your optimal filing age—accounting for your health, your spouse’s benefit, taxes, and Medicare. This is what I do. Let’s talk.

Darin Weidauer | 702-706-6564 | darinweidauer@ecos.care

C H A P T E R 1 7

Chapter 17: Spousal, Survivor, and Divorced Spouse Benefits

The Rules Nobody Explains

Social Security isn’t just about your own work record. It’s a family benefit system—and the rules governing spousal benefits, survivor benefits, and divorced spouse benefits are among the most complex and most misunderstood in the entire program.

I’ve sat across from people who had no idea they could collect on their ex-spouse’s record. I’ve worked with widows who were leaving thousands of dollars on the table because nobody explained the survivor benefit rules to them. And I’ve seen married couples make filing decisions that cost their surviving spouse six figures.

Let’s make sure none of that happens to you.

Spousal Benefits: What Your Marriage Makes Available

If you’re married, you may be eligible for a spousal benefit based on your spouse’s work record. The spousal benefit is equal to up to 50% of your spouse’s PIA (their benefit at Full Retirement Age).

You’re eligible for a spousal benefit if you’re at least 62 years old (or any age if caring for a child under 16 or a disabled child), you’ve been married for at least one year, and your spouse has filed for their own benefits (or has suspended them at FRA or later).

Here’s the critical rule: you receive the higher of your own benefit or the spousal benefit. Social Security doesn’t add them together. You get one or the other—whichever is more.

The Spousal Benefit Calculation

Scenario	Spousal Benefit Amount
You file at your FRA	50% of spouse’s PIA

Scenario	Spousal Benefit Amount
You file before FRA	Reduced (as low as 32.5% of spouse's PIA at

on spousal benefits

Important: spousal benefits do not earn Delayed Retirement Credits. Waiting past FRA does not increase the spousal benefit beyond 50% of the worker's PIA. This is different from your own benefit, which grows 8% per year from FRA to 70.

Survivor Benefits: The Benefit That Protects the Surviving Spouse

When one spouse dies, the surviving spouse is entitled to a survivor benefit based on the deceased spouse's benefit. The survivor benefit equals up to 100% of what the deceased was receiving (or was entitled to receive) at the time of death.

If the deceased had delayed filing and was receiving a benefit enhanced by Delayed Retirement Credits, the survivor gets that enhanced amount. If the deceased filed early and was receiving a reduced benefit, the survivor gets that reduced amount (subject to a minimum floor of 82.5% of the deceased's PIA if the survivor waits until their own FRA to claim).

Survivor Benefit Eligibility

You're eligible if you're age 60 or older (50 or older if disabled), or any age if caring for the deceased's child who is under 16 or disabled. You must have been married for at least 9 months (with some exceptions for accidental death or military service). You have not remarried before age 60 (remarriage after 60 does not disqualify you).

Survivor Filing Strategy

This is an area where strategy can save—or cost—tens of thousands of dollars. A surviving spouse has several options:

Option 1: Claim the survivor benefit early and let your own benefit grow. If you're under 70 and your own benefit will eventually be higher than the survivor benefit, you can claim

the survivor benefit now and switch to your own benefit at 70 when it's maximized.

Option 2: Claim your own benefit early and switch to the survivor benefit later. If the survivor benefit is higher and you're not yet at FRA, you can collect your own (reduced) benefit now and switch to the full survivor benefit at your FRA.

Option 3: Claim whichever is higher at FRA. If you're past FRA, take the higher of the two.

The ability to “sequence” benefits—collecting one now and switching to a higher one later—is one of the most valuable strategies in Social Security. It's available to survivors, and it's frequently overlooked.

Divorced Spouse Benefits: The Rules Your Ex Never Told You

About

If you're divorced, you may be entitled to benefits based on your ex-spouse's work record. This is one of the least-known provisions in Social Security—and it can make an enormous difference for people whose own work record produces a modest benefit.

Divorced Spouse Benefit Rules

You qualify if your marriage lasted at least 10 years, you are currently unmarried, you are at least 62 years old, and your own benefit is less than what you'd receive on your ex's record.

The divorced spouse benefit works just like the spousal benefit: up to 50% of your ex-spouse's PIA at your FRA, reduced if you claim before FRA.

What Your Ex Doesn't Need to Know

Here's the part that surprises people: your ex-spouse does not need to know you're claiming on their record. They are not notified. Their benefit is not reduced. Their current spouse's benefit is not affected. You are claiming from the Social Security system, not from your ex.

Additionally, if your ex has not yet filed for benefits but is old enough to qualify (62+), you can still claim divorced spouse benefits as long as you’ve been divorced for at least two years. This is called the “independently entitled” rule.

Divorced Survivor Benefits

If your ex-spouse dies, you may be eligible for survivor benefits on their record—even if you’re divorced. The same 10-year marriage requirement applies. The benefit can be up to 100% of what they were receiving (or entitled to). You can claim as early as age 60 (or 50 if disabled). Remarriage after age 60 does not disqualify you.

Reduces Maximum Marriage Worker’s

Benefit Type	Amount	Earliest Age	Requirement	Own Benefit?
Spousal Benefit	50% of spouse’s PIA	62	1 year	No
Survivor Benefit	100% of deceased’s benefit	60 (50 if disabled)	9 months	No (deceased)
Divorced Spouse	50% of ex’s PIA	62	10 years, currently unmarried	No
Divorced Survivor	100% of deceased ex’s benefit	60 (50 if disabled)	10 years	No

REAL-WORLD EXAMPLE: The Divorced Spouse Who Found \$180,000

Patricia, age 66, was divorced after 28 years of marriage. She worked part-time throughout the marriage, earning a modest Social Security benefit of \$1,100/month on her own record. Her ex-husband’s PIA was \$3,200/month. Patricia had no idea she could claim on his record. When I showed her the numbers, she was eligible for a divorced spouse benefit of \$1,600/month (50% of \$3,200). That’s \$500/month more than her own benefit —\$6,000/year. Over a 30-year retirement, that’s \$180,000 in additional benefits she almost never claimed. And her ex-husband’s benefit was completely unaffected.

FREQUENTLY ASKED QUESTIONS

Q: Can I collect spousal benefits if my spouse hasn't filed yet?

A: Generally no. Your spouse must have filed (or suspended at FRA) for you to collect spousal benefits. The exception is divorced spouse benefits: if you've been divorced for 2+ years and your ex is 62+, you can file independently.

Q: My ex-spouse remarried. Can I still claim on their record?

A: Yes. Your ex's remarriage has no effect on your eligibility for divorced spouse benefits. And your claim does not affect their new spouse's benefits.

Q: I was married twice, both for 10+ years. Can I choose which ex to claim on?

A: Yes. Social Security will calculate your benefit based on each ex's record and give you the higher amount. You can't collect on both simultaneously.

Q: Do survivor benefits include Delayed Retirement Credits?

A: Yes. If the deceased spouse had earned Delayed Retirement Credits by filing after FRA, those credits are included in the survivor benefit. This is a major reason for the higher earner to delay filing.

CHAPTER 17 CHECKLIST

☐

☐ I understand the survivor benefit (up to 100% of deceased's benefit) ☐ I know that spousal benefits do NOT earn Delayed Retirement Credits ☐ If divorced (10+ years), I've explored divorced spouse benefits ☐ I've considered the 'sequencing' strategy for survivor benefits ☐ If married, I've factored the survivor benefit into our filing decision **READY TO TAKE THE NEXT STEP?**

Spousal, survivor, and divorced spouse benefits are where Social Security gets truly complex— and where the biggest mistakes happen. As a Registered Social Security Analyst, I've helped hundreds of people discover benefits they didn't know they had. Let me review your situation. It's free, it's confidential, and it could change your retirement.

Darin Weidauer | 702-706-6564 | darinweidauer@ecos.care

C H A P T E R 1 8

Chapter 18: Social Security and Work

What Happens If You Claim Early and Keep Working

Here’s a scenario I see regularly: someone files for Social Security at 62 because they think they should “start collecting while they can.” Then they keep working, earn a good salary, and are shocked when Social Security withholds a big chunk of their benefits.

The Social Security Earnings Test is one of the most misunderstood rules in the entire system. Let me clear it up.

The Earnings Test: How It Works

If you’re collecting Social Security before reaching your Full Retirement Age AND you’re still earning income from work, Social Security may reduce your benefit based on how much you earn. This only applies to earned income (wages and self-employment income). It does not apply to investment income, pension income, rental income, or retirement account withdrawals.

2026 Earnings Test Thresholds

Situation	Earnings Limit (2026)	Reduction Rule
Under FRA for the entire year	\$24,480	\$1 withheld for every \$2 earned above limit
Year you reach FRA (months before FRA only)	\$65,160	\$1 withheld for every \$3 earned above limit
Month you reach FRA and beyond	No limit	No reduction — earn as much as you want

Let’s say you’re 63, filed at 62, and earn \$50,000 in wages in 2026. You’re under FRA for the entire year. The amount above the \$24,480 limit is \$25,520. Social Security withholds \$1 for every \$2 above the limit: $\$25,520 \div 2 = \$12,760$ withheld from your Social Security benefits during the year.

If your monthly benefit is \$1,750, that’s \$21,000 per year. With \$12,760 withheld, you’d actually receive only \$8,240 in Social

Security that year. You filed early, took a permanent reduction, and then had most of it withheld anyway.

The Good News: You Get It Back (Sort Of)

Here's the part that surprises people: the money withheld by the Earnings Test is not lost forever. When you reach FRA, Social Security recalculates your benefit to give you credit for the months in which benefits were withheld. Your monthly benefit increases to account for those withheld months.

However—and this is important—the early filing reduction is still in effect. The recalculation only adjusts for the months withheld, not for filing early in the first place. You're still permanently locked into a reduced benefit compared to what you'd have received by waiting to FRA or 70.

Why Filing Early While Working Usually Doesn't Make Sense

If you're still working and earning above the Earnings Test limit, filing early creates three problems at once:

Problem 1: Your benefit is permanently reduced by up to 30% for filing early. Problem 2: Social Security withholds a significant portion of your benefit due to your earnings. Problem 3: The additional earnings from your job increase your taxable income, potentially pushing you into higher tax brackets and triggering IRMAA surcharges on your Medicare premiums.

In most cases, the better strategy is: keep working, don't file for Social Security yet, and let your benefit grow. Every year you wait (up to age 70), your benefit increases—and you avoid the Earnings Test entirely.

After Full Retirement Age: No Limits

Once you reach FRA, the Earnings Test disappears. You can earn as much as you want from work without any reduction to your Social Security benefit. This is a clean line: before FRA, earnings can trigger withholding; at FRA and beyond, they cannot.

REAL-WORLD EXAMPLE: The Double Penalty

Michael, age 63, filed for Social Security at 62. His PIA at 67 would have been \$2,800. By filing at 62, his benefit was reduced to \$1,960/month. But Michael continued working as a consultant, earning \$75,000/year. Earnings above the 2026 limit of \$24,480: \$51,080. Amount withheld: $\$51,080 \div 2 = \$25,540$. Michael's annual Social Security benefit: \$23,520 ($\$1,960 \times 12$). After withholding: $\$23,520 - \$25,540 = \text{negative}$. Social Security suspended his entire benefit for the year—and he still took the permanent 30% reduction. If Michael had waited until 67 to file (while continuing to work), he'd receive \$2,800/month with no withholding. If he waited to 70: \$3,472/month. The early filing decision will cost Michael over \$100,000 in lifetime benefits.

FREQUENTLY ASKED QUESTIONS

Q: Does the Earnings Test apply to my spouse's income?

A: No. The test is based on your individual earned income only. Your spouse's earnings do not affect your Social Security benefit.

Q: I'm self-employed. How does the Earnings Test apply to me?

A: Net self-employment income counts toward the Earnings Test. Additionally, in your first year of retirement, Social Security may use a monthly test instead of an annual test—if you earn less than the monthly limit in a given month, you receive your full benefit for that month regardless of annual totals.

Q: Does investment income count toward the Earnings Test?

A: No. The Earnings Test only applies to earned income (wages and self-employment income). Dividends, interest, capital gains, rental income, pension payments, and retirement account withdrawals do not count.

Q: I'm 64 and already filed. Should I stop working?

A: Not necessarily. Consider whether the withholding and early reduction are worth the trade-offs. In some cases, it may make sense to withdraw your application (within 12 months) or

suspend benefits at FRA. Talk to a professional about your specific numbers.

CHAPTER 18 CHECKLIST

☐

☐ I know that only earned income (wages/self-employment) triggers the test ☐ I've calculated how much of my benefit would be withheld if I file early and keep working ☐ I understand that withheld benefits are partially recalculated at FRA ☐ I know the Earnings Test disappears at Full Retirement Age ☐ I've weighed whether filing early while working creates a 'double penalty' **READY TO TAKE THE NEXT STEP?**

If you're working past 62 and considering filing for Social Security, stop and call me first. The Earnings Test interacts with your taxes, your Medicare premiums, and your long-term benefit in ways that aren't obvious. A quick conversation can help you avoid a decision you can't undo.

Darin Weidauer | 702-706-6564 | darinweidauer@ecos.care

C H A P T E R 1 9

Chapter 19: Social Security and Taxes

How Much of Your Benefit Uncle Sam Takes

Here’s a question that makes most retirees uncomfortable: “Is my Social Security taxable?”

The answer, for most people, is yes. And the amount that’s taxable depends on your total income from all sources. Up to 85% of your Social Security benefit can be subject to federal income tax. Let me show you exactly how it works.

The Provisional Income Formula

To determine how much of your Social Security is taxable, the IRS uses a concept called “provisional income” (also called “combined income”). Here’s the formula:

Provisional Income = Adjusted Gross Income (without Social Security) + Tax-Exempt Interest (muni bonds) + 50% of Social Security Benefits.

Notice two things: your full AGI counts (IRA distributions, pensions, wages, capital gains, etc.), and tax-exempt interest counts even though it’s not taxed. This mirrors the MAGI calculation for IRMAA—another reason to plan your income sources carefully.

The Taxation Thresholds

Filing Status	Provisional Income	% of Social Security That’s Taxable
Single	Below \$25,000	0%
Single	\$25,000 – \$34,000	Up to 50%
Single	Above \$34,000	Up to 85%
Married Filing Jointly	Below \$32,000	0%
Married Filing Jointly	\$32,000 – \$44,000	Up to 50%
Married Filing Jointly	Above \$44,000	Up to 85%

These thresholds have not been adjusted for inflation since they were set in 1983 and 1993. That means more and more retirees are pushed into the taxable range each year as incomes rise.

Today, approximately 56% of Social Security recipients pay federal taxes on their benefits.

How the Tax Is Calculated: A Walkthrough

Let's work through a realistic example.

Tom and Rita are married filing jointly. Their income: Social Security: \$48,000 combined. Pension: \$30,000. Traditional IRA withdrawal: \$20,000. Municipal bond interest: \$5,000.

Step 1: Calculate provisional income. AGI without SS: \$30,000 + \$20,000 = \$50,000. Tax-exempt interest: \$5,000. 50% of SS: \$24,000. Provisional Income: \$50,000 + \$5,000 + \$24,000 = \$79,000.

Step 2: Determine taxable percentage. \$79,000 is well above the \$44,000 married threshold, so up to 85% of their Social Security is taxable.

Step 3: Calculate taxable amount. 85% of \$48,000 = \$40,800. That \$40,800 is added to their other taxable income and taxed at their marginal rate.

If Tom and Rita are in the 22% federal bracket, the tax on their Social Security is approximately $\$40,800 \times 22\% = \$8,976$. That's nearly \$9,000 in federal taxes on their Social Security alone.

State Taxes on Social Security

The good news for many readers: most states do not tax Social Security benefits. As of 2026, only a handful of states tax Social Security, and several of those have income exemptions or are phasing out the tax.

States that fully exempt Social Security (as of 2026) include Nevada, Florida, Texas, Arizona, Tennessee, Wyoming, Washington, South Dakota, Alaska, and many others. If you live in one of these states—including Nevada, where I'm based—your Social Security is state-tax-free.

Strategies to Reduce Social Security Taxes

Strategy 1: Roth Conversions Before Filing

Roth IRA distributions are not included in provisional income. If you convert traditional IRA funds to a Roth before you start collecting Social Security (and before RMDs begin), you reduce the taxable income that makes your Social Security taxable. This is the same Roth conversion strategy that reduces IRMAA—it’s a double benefit.

Strategy 2: Tax-Efficient Withdrawal Sequencing

Draw from Roth accounts and after-tax savings in years when your Social Security taxation would otherwise spike. By mixing Roth distributions (which don’t count in provisional income) with traditional distributions (which do), you can control exactly how much of your Social Security is taxed.

Strategy 3: Qualified Charitable Distributions

QCDs from your traditional IRA reduce your AGI directly, which reduces your provisional income, which reduces the taxable portion of your Social Security. A \$10,000 QCD at the 22% bracket saves \$2,200 in income tax plus potentially reduces your Social Security taxation.

Strategy 4: Manage Capital Gains

Large capital gains (from selling stocks, property, or other investments) increase your provisional income and can push more of your Social Security into the taxable range. Consider spreading gains over multiple years or offsetting with losses.

Strategy	Impact on Provisional Income	Tax Savings Potential	Best For
Roth Conversions (pre-filing)	Reduces future AGI	High (long-term)	Ages 59½–67
Roth Withdrawals (in retirement)	Not included	Moderate (annual)	Ages 67+
Qualified Charitable	Reduces AGI directly	Moderate	Ages 70½+

Strategy Distributions	Impact on Provisional Income	Tax Savings Potential	Best For
Capital Loss Harvesting	Offsets gains	Variable	Any age
Municipal Bond Rebalancing	Careful — still counts!	Limited	Review needed

REAL-WORLD EXAMPLE: The Couple Who Cut Their SS Tax by \$4,200/Year

Dennis and Sharon, both 68, were paying taxes on 85% of their \$52,000 combined Social Security benefit. Their provisional income was \$82,000 (pension, IRA withdrawals, and muni bond interest). Working with a retirement planner, they shifted \$15,000 of their annual IRA withdrawals to Roth distributions (from a Roth they’d been building through conversions). This reduced their provisional income to \$67,000. While still above the 85% threshold, the reduced AGI dropped their marginal tax bracket from 22% to 12% on a portion of their income. Net federal tax savings: approximately \$4,200 per year. Over 20 years, that’s \$84,000 kept in their pocket.

FREQUENTLY ASKED QUESTIONS

Q: Is it possible to pay 0% tax on Social Security?

A: Yes—if your provisional income stays below \$25,000 (single) or \$32,000 (married filing jointly). This requires careful planning and typically means relying heavily on Roth distributions, after-tax savings, and keeping other income sources low.

Q: Why do muni bonds count in the SS tax formula?

A: Congress included tax-exempt interest to prevent wealthy retirees from avoiding all taxes through municipal bond portfolios. Even though muni interest isn’t taxed as income, it increases provisional income and can trigger Social Security taxation.

Q: Do I have to pay taxes on my Social Security every year?

A: You can either pay quarterly estimated taxes or have taxes withheld directly from your Social Security check by filing

Form W-4V with Social Security. Withholding options are 7%, 10%, 12%, or 22%.

Q: My Social Security is my only income. Will it be taxed?

A: For most people whose only income is Social Security, the benefit will not be taxable. The provisional income would be 50% of your benefit, which typically falls below the thresholds.

CHAPTER 19 CHECKLIST

☐ I understand the provisional income formula ☐ I know the thresholds: \$25K/\$34K (single) and \$32K/\$44K (married) ☐ I've estimated what percentage of my Social Security will be taxable ☐
☐ I've considered Roth conversions to reduce future Social Security taxes ☐ I've evaluated QCDs for charitable giving and tax reduction ☐ I've confirmed whether my state taxes Social Security
 READY TO TAKE THE NEXT STEP?

Social Security taxation is one of those topics where most people leave money on the table simply because they don't know the rules. I can help you calculate your provisional income, estimate your Social Security tax, and build a withdrawal strategy that minimizes what you owe. One conversation. Real numbers. No cost to you.

Darin Weidauer | 702-706-6564 | darinweidauer@ecos.care

CHAPTER 20

Chapter 20: Registered Social Security Analysis

Why Working With an RSSA Matters

You’ve just read five chapters on Social Security—how your benefit is calculated, when to claim, spousal and survivor rules, the Earnings Test, and taxation. If your head is spinning, you’re not alone.

Social Security has over 2,700 rules and thousands of sub-rules. The SSA’s own handbook is more than 2,500 pages. Even Social Security Administration employees sometimes give incorrect information—a 2022 Government Accountability Office report found that SSA phone representatives gave inaccurate or incomplete information in 15% of test calls.

This isn’t a system you want to navigate by guessing. And it’s certainly not something you should entrust to a generic online calculator or a well-meaning friend.

What Is a Registered Social Security Analyst?

A Registered Social Security Analyst (RSSA) is a professional who has been specifically trained and certified in the complex rules governing Social Security benefits. The RSSA designation is earned through the National Association of Registered Social Security Analysts, and it requires rigorous coursework, examination, and ongoing continuing education.

An RSSA is not a government employee. An RSSA does not work for Social Security. An RSSA is an independent professional—like a CPA for your taxes or an attorney for your legal needs—who specializes in helping you make the best possible Social Security decisions.

What an RSSA Does That an Online Calculator Can’t

Online Social Security calculators are better than nothing, but they have significant limitations. Most calculators only consider your individual benefit. An RSSA considers your complete picture.

Capability Individual benefit at	Online Calculator	RSSA Analysis
62/67/70	✓	✓
Spousal benefit optimization	Basic	Comprehensive
Survivor benefit strategy	Rarely	✓ (including sequencing)
Divorced spouse benefits	Rarely	
Earnings Test impact	Sometimes	✓ ✓ (year-by-year)
Social Security taxation analysis	No	✓ (integrated with other income)
IRMAA impact of filing decisions	No	✓
Coordination with Medicare enrollment	No	✓
Coordination with Roth	No	

Capability inflation	Online Calculator	RSSA Analysis
Windfall Elimination Provision (WEP)	Rarely	✓
Government Pension	Rarely	
Offset (GPO)		✓
Scenario comparison (multiple strategies)	Limited	Unlimited
Personalized recommendation	No	✓

An RSSA doesn’t just tell you what your benefit will be. An RSSA tells you when to file, how to coordinate with your spouse, how your filing decision affects your taxes and Medicare premiums, and how to sequence benefits to maximize your lifetime income.

The Windfall Elimination Provision (WEP) and Government Pension Offset (GPO)

These two provisions affect people who worked in government positions where they did not pay into Social Security (many state and local government employees, teachers in certain states, and some federal employees hired before 1984).

The WEP reduces your own Social Security benefit if you also receive a pension from work not covered by Social Security. Instead of the normal 90% replacement on the first bend point of your AIME, the WEP reduces that to as low as 40%,

potentially cutting your benefit by hundreds of dollars per month.

The GPO reduces spousal or survivor benefits by two-thirds of your government pension. If your government pension is \$3,000/month, your spousal benefit is reduced by \$2,000—which can completely eliminate the benefit for many people.

These provisions are complex, often misunderstood, and require specialized analysis. An RSSA can calculate the exact impact and help you plan accordingly.

What a Social Security Analysis Looks Like

When I conduct a Social Security analysis for a client, here's what we cover:

Full earnings history review to verify accuracy and identify any gaps or corrections needed. Benefit calculation at every possible filing age (62 through 70). Spousal benefit analysis (if married or previously married for 10+ years). Survivor benefit analysis and sequencing strategies. Break-even analysis adjusted for COLA. Earnings Test impact (if still working). Social Security taxation projection integrated with all other income sources. IRMAA impact analysis showing how filing decisions affect Medicare premiums. Roth conversion coordination to optimize both taxes and benefits. A written report with clear recommendations and action steps.

This analysis typically reveals \$50,000 to \$150,000 in additional lifetime benefits compared to the “default” filing strategy most people follow.

The Cost of Not Getting Help

Social Security is a one-shot decision for most people. You can't go back and undo a filing strategy that costs you \$100,000 over your lifetime. You can't recapture survivor benefits your spouse lost because you filed too early. You can't retroactively avoid IRMAA surcharges from an income spike you didn't plan for.

The cost of a professional Social Security analysis is typically zero to modest. Many RSSA professionals, myself included,

offer free initial consultations and analyses as part of our comprehensive retirement planning services. The return on that investment is measured in tens of thousands of dollars.

REAL-WORLD EXAMPLE: The Analysis That Changed Everything

Carl and Debra, both 64, planned to file for Social Security at 65. They assumed it was simple: file when you're eligible, collect your check. Carl's PIA was \$2,600. Debra's was \$1,300. During their RSSA analysis, three things became clear:

First, Carl should delay to 70. His benefit would grow from \$2,600 to \$3,224/month, and more importantly, his survivor benefit for Debra would be \$3,224 instead of the reduced \$2,340 she'd get if Carl filed at 65.

Second, Debra should file at 65 for her own reduced benefit (\$1,127/month) to provide household income while Carl delays. This is the "sequencing" strategy.

Third, Carl should do \$60,000/year in Roth conversions from ages 64 to 69 to reduce his future RMDs and avoid IRMAA when his RMDs begin at 73.

Projected additional lifetime benefits from this strategy compared to their original plan: \$172,000. That's the value of professional analysis.

FREQUENTLY ASKED QUESTIONS

Q: How is an RSSA different from a financial advisor?

A: Most financial advisors focus on investments. An RSSA specializes specifically in Social Security rules and optimization. The best outcome is working with both—an RSSA for Social Security strategy and a financial advisor for investment management—or finding a professional who does both.

Q: Does it cost anything to get an RSSA analysis?

A: It varies by professional. Many RSSAs, including myself, offer free initial analyses as part of a comprehensive retirement review. The analysis has no obligation attached.

Q: Can I trust the Social Security Administration to give me the right advice?

A: SSA employees can tell you what your benefit is and how to apply. They cannot—and are not trained to—give you strategic advice about when to file, how to coordinate with a spouse, or how your filing decision affects your taxes and Medicare. That’s what an RSSA is for.

Q: I already filed. Can an RSSA still help me?

A: Yes. If you filed within the last 12 months, you may be able to withdraw your application and refile later. If you’ve reached FRA, you can suspend your benefit to earn Delayed Retirement Credits. And regardless of timing, an RSSA can help optimize your remaining decisions— spousal strategies, taxation planning, and IRMAA management.

CHAPTER 20 CHECKLIST

☐

☐ I know the limitations of online Social Security calculators ☐ I’m aware of WEP and GPO if I have a government pension ☐ I’ve reviewed my Social Security Statement at ssa.gov ☐ I’ve considered getting a professional analysis before making my filing decision ☐ I understand that the filing decision is largely irreversible and worth getting right **READY TO TAKE THE NEXT STEP?**

You’ve read five chapters on Social Security. You now know more than most Americans. But knowing the rules and applying them to your unique situation are two different things. As a Registered Social Security Analyst, I can run a complete analysis of your benefits—for you and your spouse—and show you the filing strategy that maximizes your lifetime income. This analysis is free. It’s personal. And it could be worth more than \$100,000 over your retirement. Call or text me today. Let’s find your best strategy.

Darin Weidauer | 702-706-6564 | darinweidauer@ecos.care

PART F OUR

PART IV — RETIREMENT INCOME PLANNING

Medicare protects your health. Social Security provides a floor. This section builds the income engine that funds the retirement you deserve.

C H A P T E R 2 1

Chapter 21: Building Your Retirement Income Stack

Social Security + Medicare + Investments + Insurance

Think of your retirement income like a house. Social Security is the foundation. Medicare is the roof that keeps the weather out. Your savings and investments are the walls. And insurance products—annuities, life insurance, long-term care—are the safety systems that keep the house standing when a storm hits.

No single income source is strong enough to carry your entire retirement on its own. Social Security replaces roughly 40% of pre-retirement income for the average worker. Medicare covers healthcare but still leaves you with \$4,000 to \$7,000 per year in premiums and out-of-pocket costs. The rest—your lifestyle, your travel, your gifts to grandchildren, your ability to handle emergencies—comes from the income stack you build.

This chapter gives you the big picture. The chapters that follow go deep on each component.

The Retirement Income Stack

Your retirement income comes from five main sources. The trick is knowing how much you can safely draw from each, how they interact with taxes and IRMAA, and how to sequence withdrawals for maximum efficiency.

Income Layer	Examples	Tax Treatment	IRMAA Impact	Reliability
Layer 1: Guaranteed	Social Security, pensions, annuity payments	Partially to fully taxable	Counts in MAGI	Highest — lasts for life
Income				
Layer 2: Tax-Deferred	Traditional IRA, 401(k), 403(b),	100% taxable on withdrawal	Counts in MAGI	High — depends on balance and markets
Accounts	TSP			
Layer 3: Tax-	Roth IRA, Roth	Tax-free	Does NOT count in	High — your

Income Layer	Examples	Tax Treatment	IRMAA Impact	Reliability
Free Accounts	401(k), HSA	(qualified)	MAGI	IRMAA escape valve
Layer 4: Taxable Accounts	(medical) Brokerage, savings, CDs, bonds	Gains and dividends taxable	Counts in MAGI	Moderate — market dependent
Layer 5: Insurance & **	Cash value life insurance, reverse mortgage, rental income	Varies	Varies	Varies by product
Other**				

The Income Replacement Question

How much income do you actually need in retirement? The standard rule of thumb says 70– 80% of your pre-retirement income. But rules of thumb are dangerous because they ignore your specific reality.

A better approach: build a detailed retirement budget. Start with your essential expenses (housing, food, healthcare, utilities, insurance, transportation, taxes) and add your lifestyle expenses (travel, hobbies, dining, gifts, entertainment). Then add a buffer for emergencies and unexpected costs. That total is your income target.

For most couples I work with, the number falls between \$60,000 and \$120,000 per year, depending on their location, health needs, and lifestyle. The question then becomes: which income layers cover that target, and in what proportions?

The Tax Efficiency Framework

Here’s the framework I use with every client. It’s built around one central principle: control your taxable income each year to minimize taxes, avoid IRMAA, and protect your Social Security from unnecessary taxation.

Step 1: Start with guaranteed income (Social Security + pension). This is your floor. Step 2: Fill the gap between guaranteed income and your spending target using the most tax-efficient source available. Step 3: Use Roth distributions when you need income without increasing MAGI. Step 4: Use traditional IRA/401(k) withdrawals to “fill up” your tax bracket without crossing into the next bracket or into an IRMAA threshold. Step 5: Use taxable accounts for flexibility, managing when you realize capital gains. Step 6: Use insurance products (annuities, cash value life) for specific needs like guaranteed lifetime income or tax-free access to funds.

A Sample Income Stack in Action

Let’s see how this works for a realistic couple.

REAL-WORLD EXAMPLE: The Williams Family Income Stack

Dave and Maria Williams, both 68. Monthly spending target: \$7,500 (\$90,000/year). Income sources: Social Security (combined): \$4,200/month (\$50,400/year). Dave’s small pension: \$800/month (\$9,600/year). Traditional IRA: \$650,000. Roth IRA: \$180,000. Taxable brokerage: \$120,000. Medigap Plan G + Part D premiums: \$450/month (\$5,400/year).

Their guaranteed income covers \$5,000/month (\$60,000/year). Their gap: \$7,500 – \$5,000 = \$2,500/month (\$30,000/year). To fill the gap, they withdraw \$22,000/year from their traditional IRA (keeping total MAGI below the \$218,000 married IRMAA threshold) and \$8,000/year from their Roth IRA (tax-free, no MAGI impact). Total income: \$90,000. IRMAA status: standard (no surcharges). Federal tax bill: approximately \$6,800 (thanks to the standard deduction and careful bracket management). Social Security taxation: approximately 50–60% of benefits taxable (not the full 85%), saving them roughly \$1,500/year compared to withdrawing entirely from traditional accounts.

The Retirement Income Timeline

Age Range	Key Income Actions	Planning Priority
59½–62	Can access retirement accounts penalty-free; consider Roth conversions	Build Roth bucket; minimize future IRMAA

Age Range	Key Income Actions	Planning Priority
62-66	Earliest SS filing; bridge income from savings if delaying	Delay SS if possible; continue Roth conversions
65	SS Medicare begins; coordinate enrollment with employer coverage	Lock in Medicare; evaluate Medigap vs. Advantage
67	Full Retirement Age; no	Optimize SS filing; calibrate withdrawals
70	Earnings Test on SS Maximum SS benefit; Delayed Retirement Credits end	Lock in maximum SS; shift withdrawal strategy
73	RMDs begin from traditional accounts	Manage RMDs for taxes and
75+	Healthcare costs typically rise; LTC risk increases	IRMAA; use QCDs Review insurance coverage; update estate plans

FREQUENTLY ASKED QUESTIONS

Q: How do I know if my income stack is strong enough?

A: A good test: can your guaranteed income (Social Security + pension + annuities) cover at least your essential expenses? If yes, your investment withdrawals only need to cover lifestyle costs—which gives you much more flexibility and resilience if markets dip.

Q: Should I pay off my mortgage before retiring?

A: It depends. A paid-off mortgage reduces your essential expenses and gives you peace of mind. But if your mortgage rate is low (under 4–5%), the money might work harder invested or in a Roth. There’s no universal right answer—it’s a math-and-emotion decision.

Q: What’s a safe withdrawal rate?

A: The traditional “4% rule” suggests withdrawing 4% of your portfolio in year one and adjusting for inflation each year. More recent research suggests 3.5–4.5% depending on your allocation and retirement length. A professional can model your specific scenario.

CHAPTER 21 CHECKLIST

☐

☐ I've created a detailed retirement budget (essential + lifestyle + buffer) ☐ I know how much of my spending target is covered by guaranteed income ☐ I understand which income sources affect MAGI and which don't ☐ I've started thinking about the sequence of withdrawals across my accounts ☐ I've reviewed the retirement income timeline and noted upcoming milestones

READY TO TAKE THE NEXT STEP?

Building your income stack isn't something you do once—it's an ongoing strategy that evolves as your life changes. I can help you map your income sources, identify gaps, and build a withdrawal strategy that minimizes taxes and maximizes your money. Let's build your blueprint together.

Darin Weidauer | 702-706-6564 | darinweidauer@ecos.care

CHAPTER 22

Chapter 22: 401(k), IRA, and Roth IRA

The Differences That Could Save You Six Figures in Taxes

If you’ve been saving for retirement through an employer plan or an individual account, you probably have money in one or more of these three vehicles: a 401(k), a traditional IRA, or a Roth IRA. They might all seem like “retirement accounts,” but the tax treatment of each one is dramatically different—and those differences can mean the difference between paying \$50,000 or \$150,000 in taxes over your retirement.

Let’s break down what each account is, how it’s taxed, and why the decisions you make now about these accounts are some of the most important financial decisions of your life.

The Three Tax Buckets

Feature	Traditional 401(k) / IRA	Roth 401(k) / Roth IRA
Contributions	Pre-tax (deductible)	After-tax (not deductible)
Growth	Tax-deferred	Tax-free
Withdrawals	Taxed as ordinary income	Tax-free (if qualified)
RMDs Required?	(100%) Yes, starting at age 73	Roth IRA: No. Roth 401(k):
IRMAA Impact	Yes — increases Medicare premiums	count No — invisible to IRMAA
SS Taxation Impact	Yes — increases provisional income	No — doesn’t affect SS taxation
Estate Tax	Heirs pay income tax on inherited funds	Heirs receive tax-free (if held
Best For	Lower tax bracket now vs. retirement	5+ years) Same or higher tax bracket in retirement

The fundamental question: do you want to pay taxes now (Roth) or later (Traditional)? If you believe tax rates will be higher when you withdraw the money—either because your income will be higher or because Congress will raise rates—Roth accounts give you a permanent tax shield.

2026 Contribution Limits

Account Type	Under Age 50	Age 50+ (Catch-Up)	Age 60-63 (Super Catch-Up)
401(k) / 403(b) / TSP	\$24,500	\$32,500 (+ \$8,000)	\$35,750 (+ \$11,250)
Traditional IRA	\$7,500	\$8,600 (+ \$1,100)	\$8,600
Roth IRA	\$7,500	\$8,600 (+ \$1,100)	\$8,600
SIMPLE IRA	\$16,500	\$20,000 (+ \$3,500)	\$21,250 (+ \$4,750)

Notice the “Super Catch-Up” for ages 60–63. Starting in 2025, if you’re between 60 and 63, you can contribute an additional \$11,250 to your 401(k)—bringing your total possible contribution

to \$34,750. This is a new provision from the SECURE 2.0 Act, and it’s a powerful tool for people in their early 60s who want to maximize retirement savings.

Roth IRA Income Limits (2026)

Unlike traditional IRAs, Roth IRA contributions have income limits. If your Modified Adjusted Gross Income exceeds these thresholds, your ability to contribute directly to a Roth IRA is reduced or eliminated.

Filing Status	Full Contribution Allowed	Reduced Contribution	No Direct Contribution
Single / Head of Household	MAGI under \$150,000	\$150,000–\$165,000	Above \$165,000
Married Filing Jointly	MAGI under \$236,000	\$236,000–\$246,000	Above \$246,000

If your income exceeds the Roth IRA limits, you can still get money into a Roth through the “Backdoor Roth IRA” strategy (contribute to a non-deductible traditional IRA, then convert to Roth) or through Roth conversions from existing traditional accounts. These strategies have no income limits.

The Rollover Decision: What to Do When You Leave a Job

When you leave an employer, you typically have four options for your 401(k):

Option 1: Leave it in the former employer's plan. This is fine if the plan has good investment options and low fees. But you lose the ability to consolidate and you may have limited investment choices.

Option 2: Roll it into your new employer's plan. Good for simplicity, but only if the new plan has competitive options.

Option 3: Roll it into a traditional IRA. This gives you maximum investment flexibility and control. An IRA rollover is the most common choice and opens the door to future Roth conversions.

Option 4: Roll it into a Roth IRA (a rollover conversion). The entire amount becomes taxable income in the year of conversion, but once in the Roth, it grows and distributes tax-free forever. This is a powerful move if you're in a low-income year or early in retirement.

 **** CRITICAL WARNING: AVOID THE 60-DAY ROLLOVER TRAP****

If you receive a check from your old 401(k), you have 60 days to deposit it into another qualified account. If you miss the deadline, the entire amount is treated as a taxable distribution, and if you're under 59½, you'll owe a 10% early withdrawal penalty on top of income taxes. Always request a direct trustee-to-trustee transfer to avoid this risk entirely.

The Tax Diversification Strategy

The smartest retirees don't put all their money in one tax bucket. They build tax diversification—having money in pre-tax (traditional), tax-free (Roth), and taxable (brokerage) accounts. This gives you flexibility to draw from the right source each year based on your tax situation, IRMAA thresholds, and cash flow needs.

Think of it like a three-faucet sink. You can turn each faucet up or down depending on what you need. Need income without

increasing MAGI? Turn the Roth faucet. Need a deduction this year? Contribute to a traditional account. Need to harvest capital gains at the 0% rate? Use the taxable faucet. Flexibility is power.

REAL-WORLD EXAMPLE: The Six-Figure Tax Difference

Mark and Susan, both 62, have \$1,400,000 in traditional 401(k)s and \$0 in Roth accounts. If they withdraw \$70,000/year from their traditional accounts in retirement (on top of \$60,000 in Social Security and pension), their MAGI is \$130,000. They're in the 22% federal bracket and pay approximately \$14,000/year in federal taxes. Over 25 years, that's \$350,000 in taxes.

Alternative scenario: between ages 62 and 67, they convert \$60,000/year (\$300,000 total) to a Roth, paying approximately \$66,000 in conversion taxes. In retirement, they split withdrawals: \$40,000/year from traditional and \$30,000/year from Roth. Their MAGI drops to \$100,000. They fall into the 12% bracket on much of their income. Annual taxes drop to approximately \$8,500. Over 25 years, that's \$212,500 in taxes plus \$66,000 in conversion taxes = \$278,500 total. Tax savings: approximately \$71,500. IRMAA savings (staying below the \$106K threshold): approximately \$17,760 over 20 years on Medicare. Total benefit: roughly \$89,000.

FREQUENTLY ASKED QUESTIONS

Q: Can I contribute to both a 401(k) and an IRA?

A: Yes. The 401(k) and IRA contribution limits are separate. You can max out your 401(k) (\$24,500 in 2026) and also contribute to an IRA (\$7,500 or \$8,600 with catch-up). However, the traditional IRA deduction may be limited if you're covered by an employer plan and your income exceeds certain thresholds.

Q: Should I choose the Roth 401(k) or traditional 401(k) at work?

A: If you expect to be in a similar or higher tax bracket in retirement, choose the Roth 401(k). If you're in a high bracket now and expect to be in a lower bracket later, the traditional

401(k) saves you more today. Many people benefit from splitting contributions between both.

Q: What's the difference between a Roth IRA and a Roth 401(k)?

A: Both grow tax-free and distribute tax-free. But a Roth 401(k) has RMDs starting at age 73 (though you can roll it to a Roth IRA to avoid this), no income limits for contributions, and higher contribution limits (\$24,500 vs. \$7,500). A Roth IRA has no RMDs, income limits for direct contributions, and the 5-year rule for tax-free growth on conversions.

Q: I'm 60 and haven't saved enough. What can I do?

A: Use the Super Catch-Up provision to contribute up to \$34,750 to your 401(k) from ages 60– 63. Also contribute \$8,000 to an IRA. Consider delaying Social Security to 70 for the 24% boost. And explore whether a Roth conversion ladder makes sense now while your income may be manageable.

CHAPTER 22 CHECKLIST

☐ I know the difference between traditional and Roth accounts
☐ I've reviewed my 2026 contribution limits and catch-up provisions
☐ I've evaluated whether to contribute to traditional or Roth (or both)
☐ I understand the rollover options when leaving an employer
☐ I've started building tax diversification across pre-tax, tax-free, and taxable buckets
☐ I've checked Roth IRA income limits for direct contributions
READY TO TAKE THE NEXT STEP?

The difference between a good retirement account strategy and a mediocre one can be six figures in taxes. I can review your accounts, model different conversion and withdrawal scenarios, and show you the path that keeps the most money in your pocket. Let's get your accounts working together.

Darin Weidauer | 702-706-6564 | darinweidauer@ecos.care

C H A P T E R 2 3

Chapter 23: The Thrift Savings Plan (TSP)

A Special Chapter for Federal Employees and Veterans

If you're a federal civilian employee, a member of the uniformed services, or a veteran who served after the TSP was introduced in 1987, this chapter is written specifically for you. The Thrift Savings Plan is one of the best retirement savings vehicles in the country. But it comes with unique rules, unique advantages, and unique decisions that most financial advisors outside the federal world don't fully understand.

As a 22-year Air Force veteran, I lived with the TSP. I contributed to it. I made decisions about it. And now I help other service members and federal employees make the most of it. Let me share what I've learned.

What Makes the TSP Special

The TSP is essentially the federal government's version of a 401(k). But it has some advantages that most private-sector 401(k) plans can't match:

Ultra-low fees: TSP expense ratios are among the lowest in the industry—approximately 0.049% in 2026 (that's less than 5 cents per \$100 invested). Most private 401(k) plans charge 0.5% to 1.0% or more. Over a 30-year career, this fee advantage alone can save you tens of thousands of dollars.

Government matching (FERS employees): If you're under the Federal Employees Retirement System (FERS), the government matches your contributions dollar-for-dollar on the first 3% of pay, plus 50 cents on the dollar for the next 2%. That's a 5% match on a 5% contribution—free money you should never leave on the table.

Roth TSP option: Since 2012, you can direct contributions to a Roth TSP, which works like a Roth 401(k). Contributions are after-tax, but growth and distributions are tax-free in retirement.

TSP vs. 401(k) vs. IRA Comparison

Feature	TSP	Private 401(k)	Traditional/Roth IRA
2026 Contribution	\$24,500 (+ \$8,000 catch-up;	Same	\$7,500 (+ \$1,100 catch-up)
Limit	+ \$11,250 ages		
Expense Ratio	60-63) ~0.049%	0.5%-1.0% typical	Depends on investments chosen
Investment	5 core funds + L	Varies by employer	Virtually unlimited
Options	Funds		
Roth Option	Yes (Roth TSP)	Depends on employer	Yes (Roth IRA)
Employer Match	Yes (FERS: up to 5%)	Varies	No
Loan Option	Yes (2 loans max)	Varies	No
RMDs at 73	Yes (unless still working)	Yes (unless still working)	Traditional: Yes.
Rollover Options	Can roll to IRA or new employer plan	Can roll to IRA	Roth IRA: No Can roll between IRA types

TSP Investment Funds

Fund	What It Invests In	Risk Level	10-Year Avg Return (approx.)
G Fund	Government securities (special non-marketable)	Very Low	2.0-2.5%
F Fund	U.S. bond index (Bloomberg Barclays	Low-Moderate	1.5-3.0%
C Fund	Aggregate) S&P 500 large-cap stock index	Moderate-High	10-12%
S Fund	Small/mid-cap U.S.	High	9-11%
I Fund	stock index International stock index (developed markets)	High	5-8%
L Funds	Lifecycle target-date funds	Varies by target date	Varies

Fund	What It Invests In	Risk Level	10-Year Avg Return (approx.)
(auto-rebalancing)			

The G Fund deserves special attention. It’s unique to the TSP—no private-sector equivalent exists. The G Fund is backed by the full faith and credit of the U.S. government and has never lost money in any month, quarter, or year. It’s the ultimate safe harbor for your most conservative allocation. However, its returns barely keep pace with inflation, so holding too much in G Fund during your accumulation years can drag down your long-term growth.

The TSP Rollover Decision: Stay or Go?

When you leave federal service or the military, you face a critical decision: leave your money in the TSP or roll it to an IRA. Both options have merits.

Reasons to stay in TSP: the ultra-low fees (lower than almost any IRA), the unique G Fund, simplicity, and continued access to TSP loans if you still have one.

Reasons to roll to an IRA: much wider investment selection, the ability to do Roth conversions more easily, no RMDs on a Roth IRA (Roth TSP has RMDs unless rolled to a Roth IRA), access to individual stocks and ETFs, and the ability to work with an advisor who manages the IRA.

My recommendation for most people: keep a portion in TSP (particularly the G Fund allocation) for the low fees, and roll the rest to a traditional IRA or Roth IRA for flexibility and Roth conversion access. This is especially important if you plan to do Roth conversions in your 60s to reduce future RMDs and IRMAA.

Roth TSP vs. Traditional TSP

The Roth TSP question mirrors the Roth 401(k) question: do you want to pay taxes now or later? For service members and federal employees in lower tax brackets (which is common for

younger enlisted and GS employees), Roth TSP contributions are often the better choice because you're paying tax at a low rate now and getting tax-free growth and distributions forever.

For senior officers and high-GS employees in the 24–32% brackets, the traditional TSP may save more in current taxes. But remember: traditional TSP withdrawals will increase your MAGI, trigger IRMAA, and increase Social Security taxation. Roth TSP distributions won't.

Many federal employees benefit from splitting contributions: some to traditional TSP (reducing current taxes) and some to Roth TSP (building a tax-free bucket for retirement).

REAL-WORLD EXAMPLE: The Retired Master Sergeant's TSP Strategy

Tony, a retired Air Force Master Sergeant (E-7), left active duty at age 42 with \$220,000 in his traditional TSP. He rolled it into a traditional IRA at age 42 and continued working as a civilian DOD contractor, contributing to a new 401(k). Between ages 60 and 65, Tony converted \$40,000/year from his traditional IRA to a Roth (\$200,000 total), paying approximately \$44,000 in conversion taxes at the 22% rate.

At 73, Tony's traditional IRA balance is roughly \$300,000 instead of \$700,000 (had he not converted). His RMD is approximately \$11,321 instead of \$26,415. With roughly \$90,000 a year in pension, Social Security and other income, that difference is what keeps his MAGI below the \$109,000 single IRMAA threshold. He saves approximately \$974/year in IRMAA surcharges and has \$450,000+ in a Roth IRA generating tax-free income. Net benefit over retirement: estimated \$60,000–80,000 in combined tax and IRMAA savings.

FREQUENTLY ASKED QUESTIONS

Q: Can I roll my TSP to a Roth IRA directly?

A: You can roll your Roth TSP balance directly to a Roth IRA (tax-free). For your traditional TSP balance, you'd roll it to a traditional IRA first, then do Roth conversions over time. You cannot directly roll traditional TSP to a Roth IRA without it being treated as a taxable conversion.

Q: I’m still in the military. Should I contribute to Roth TSP?

A: For most active-duty service members—especially those in lower tax brackets or deployed in combat zones (where income is tax-free)—Roth TSP is an excellent choice. You’re paying little or no tax now and building a lifetime of tax-free growth.

Q: What happens to my TSP if I die?

A: Your TSP is distributed to your designated beneficiaries. Make sure your TSP beneficiary designation is current—a TSP designation overrides your will. If you’re divorced and remarried, verify that your ex-spouse is not still listed as beneficiary.

Q: Does my military pension affect my Social Security?

A: Military pensions are covered by Social Security (you paid FICA taxes during service), so the Windfall Elimination Provision does not apply to military pensions. Your pension income does count toward your MAGI for IRMAA and toward provisional income for Social Security taxation.

CHAPTER 23 CHECKLIST

☐

☐ I’ve reviewed the five core TSP funds and Lifecycle Funds ☐ I’ve evaluated whether to stay in TSP, roll to an IRA, or do a partial rollover ☐ I’ve considered Roth TSP contributions or Roth conversions from traditional TSP ☐ I’ve updated my TSP beneficiary designation ☐ I’ve confirmed that my military years are correctly reflected in my Social Security record **READY TO TAKE THE NEXT STEP?**

I spent 22 years in the Air Force. I know the TSP inside and out. Whether you’re still serving, recently separated, or decades into retirement, I can help you make the most of your TSP—and coordinate it with your pension, Social Security, Medicare, and everything else. Call a fellow veteran. I’ve got your six.

Darin Weidauer | 702-706-6564 | darinweidauer@ecos.care

C H A P T E R 2 4

Chapter 24: Required Minimum Distributions (RMDs)

The IRS’s Forced Withdrawal Rule

You’ve spent decades putting money into your retirement accounts. You’ve deferred taxes on contributions and growth. Now the IRS wants its share. That’s the essence of Required Minimum Distributions: the government gave you a tax break on the way in, and now it’s collecting on the way out.

Starting at age 73 (for most people in 2026), you must begin taking mandatory withdrawals from your traditional retirement accounts every year. Miss one, and the penalty is severe. Take too much without planning, and you’ll spike your taxes and IRMAA. Get it right, and RMDs become just another manageable line item in your retirement income plan.

Who Must Take RMDs and When

RMDs apply to traditional IRAs, traditional 401(k)s, 403(b)s, 457(b)s, TSP, SIMPLE IRAs, SEP IRAs, and inherited retirement accounts. They do NOT apply to Roth IRAs during the owner’s lifetime (one of the Roth IRA’s most powerful advantages).

Under the SECURE 2.0 Act, the RMD starting age depends on when you were born:

Year of Birth	RMD Starting Age	First RMD Due
1950 or earlier	72	Already started
1951-1959	73	Year you turn 73
1960 or later	75	Year you turn 75

If you’re reading this book at age 59½ and were born in 1966 or 1967, your RMDs won’t begin until you turn 75. That gives you approximately 15 years to plan—and potentially 15 years of Roth conversions to reduce the traditional balance before RMDs force your hand.

How RMDs Are Calculated

Your RMD is calculated by dividing your account balance on December 31 of the previous year by a life expectancy factor from the IRS Uniform Lifetime Table. The larger your balance, the larger your RMD.

2026 RMD Table (Selected Ages)

Distribution Period % of Account RMD on RMD on				
Age	(Divisor)	Withdrawn	\$500,000	\$1,000,000
73	26.5	3.77%	\$18,868	\$37,736
75	24.6	4.07%	\$20,325	\$40,650
78	22.0	4.55%	\$22,727	\$45,455
80	20.2	4.95%	\$24,752	\$49,505
83	17.7	5.65%	\$28,249	\$56,497
85	16.0	6.25%	\$31,250	\$62,500
88	13.6	7.35%	\$36,765	\$73,529
90	12.2	8.20%	\$40,984	\$81,967

Distribution Period % of Account RMD on RMD on				
Age	(Divisor)	Withdrawn	\$500,000	\$1,000,000
93	10.1	9.90%	\$49,505	\$99,010
95	8.6	11.63%	\$58,140	\$116,279

Notice the trend: at 73, you withdraw about 3.8%. By 85, it’s 6.25%. By 95, it’s nearly 12%. For large accounts, these forced withdrawals can generate enormous taxable income in your later years—precisely when you’re least able to manage it.

The RMD Penalty: Don’t Miss One

If you fail to take your full RMD, the IRS imposes a penalty. Under SECURE 2.0, the penalty was reduced from 50% to 25% of the shortfall (and to 10% if corrected within two years). Even at 25%, this is severe. Miss a \$40,000 RMD, and you owe \$10,000 in penalties.

The First-Year RMD Trap

You can delay your first RMD until April 1 of the year after you turn 73 (or 75). But this creates a trap: if you delay your first

RMD, you'll have to take two RMDs in the same calendar year — your delayed first RMD plus your regular second-year RMD. Two large distributions in one year can push you into a higher tax bracket and a higher IRMAA tier.

In most cases, it's better to take your first RMD in the year you turn 73 rather than delaying to April 1 of the following year.

Strategies to Manage RMDs

Strategy 1: Roth Conversions Before RMDs Begin

Every dollar you convert to a Roth before age 73 (or 75) is a dollar that doesn't generate an RMD. This is the single most effective RMD reduction strategy. We covered this in detail in Chapter 14 and Chapter 25.

Strategy 2: Qualified Charitable Distributions (QCDs)

If you're 70½ or older, you can direct up to \$111,000 per year (2026, indexed for inflation) from your traditional IRA directly to a qualified QCD counts toward your RMD but is not included in your taxable income or MAGI. This means it satisfies your RMD obligation without triggering taxes, IRMAA, or Social Security taxation impacts.

Strategy 3: Still Working? You May Delay

If you're still working past 73 and participating in your current employer's 401(k) (and you don't own more than 5% of the company), you can delay RMDs from that 401(k) until you actually retire. This exception does not apply to IRAs or previous employers' plans—only your current employer's plan.

Strategy 4: Strategic Withdrawal Timing

Take your RMD early in the year and invest it in a taxable account. This gives you more time for the proceeds to generate returns in the new account and avoids the risk of forgetting the RMD as year-end approaches.

REAL-WORLD EXAMPLE: The Double-RMD Tax Bomb

Donna turned 73 in November 2026. She decided to delay her first RMD until April 1, 2027. Her IRA balance was \$800,000,

making her 2026 RMD approximately \$30,189. Her 2027 RMD (based on her year-end 2026 balance) is approximately \$31,008. By delaying, Donna takes both RMDs in 2027: $\$30,189 + \$31,008 = \$61,197$ in forced distributions in a single year. Combined with her \$55,000 in Social Security and pension income, her 2027 MAGI is \$116,197. This pushes her into the first IRMAA surcharge bracket two years later, costing her an extra \$974/year in Medicare surcharges (at 2026 rates). If she'd taken her first RMD on time in 2026, each year's MAGI would have been roughly \$85,000—safely below the IRMAA threshold.

FREQUENTLY ASKED QUESTIONS

Q: Do I have to spend my RMD?

A: No. You must withdraw it from the retirement account, but you can reinvest it in a taxable brokerage account, put it in savings, use it for living expenses, donate it via QCD, or anything else. The key is getting it out of the tax-deferred account.

Q: Do Roth IRAs have RMDs?

A: No. Roth IRAs do not have RMDs during the owner's lifetime. This is one of the Roth's most significant advantages. Note: Roth 401(k)s and Roth TSP accounts do have RMDs, but you can roll them to a Roth IRA to avoid this.

Q: What if I have multiple IRAs?

A: Your RMD is calculated separately for each IRA, but you can take the total from any one or combination of your IRAs. You cannot satisfy an IRA RMD from a 401(k) or vice versa—those must be taken from the specific account.

Q: Can I take more than my RMD?

A: Yes. The RMD is the minimum. You can always take more. But every dollar above the RMD increases your taxable income, MAGI, and potentially your IRMAA and Social Security taxation.

CHAPTER 24 CHECKLIST

☐ I know when my RMDs will begin (age 73 or 75 depending on birth year) ☐ I've estimated my first RMD based on my current account balances ☐ I understand the penalty for missing an RMD (25% of the shortfall) ☐ I know the first-year trap of delaying to April 1 and getting two RMDs in one year ☐ I've considered Roth conversions to reduce my future traditional balance and RMDs ☐
 READY TO TAKE THE NEXT STEP?

RMDs are coming whether you're ready or not. The question is whether they'll be a manageable part of your income plan or a tax bomb that costs you thousands in extra taxes and IRMAA surcharges every year. I can model your RMDs, show you the impact, and build a strategy to minimize the damage. Let's plan before the clock runs out.

Darin Weidauer | 702-706-6564 | darinweidauer@ecos.care

C H A P T E R 2 5

Chapter 25: Roth Conversions

The Tax Strategy That Can Transform Your Retirement

If I could give every retiree one piece of financial advice, it might be this: convert at least some of your traditional retirement money to a Roth IRA—and do it strategically, starting as early as possible.

We’ve touched on Roth conversions in several chapters already because they affect everything: your taxes, your IRMAA, your Social Security taxation, your RMDs, and your estate. This chapter is where we pull it all together into a complete Roth conversion strategy.

What a Roth Conversion Is

A Roth conversion is the process of moving money from a traditional retirement account (IRA, 401(k), 403(b), TSP) into a Roth IRA. You pay income tax on the converted amount in the year of conversion. After that, the money grows tax-free and distributes tax-free for the rest of your life—and your heirs’ lives.

There are no income limits on Roth conversions. Anyone can convert, regardless of how much they earn. There are no limits on the amount you can convert in a single year, though tax consequences increase with larger conversions.

The Seven Benefits of Roth Conversions

Benefit	Why It Matters
1. Tax-free withdrawals in retirement	Every dollar in a Roth comes out tax-free
4. Reduces future RMDs from traditional accounts	Medicare surcharges Smaller traditional balance = smaller forced withdrawals
5. Doesn’t increase Social Security taxation	Roth distributions aren’t included in provisional income
6. Tax-free inheritance for heirs	Beneficiaries receive Roth distributions tax-

tax-paid and protected

The Roth Conversion Tax Impact Illustration

Every conversion triggers a tax bill. The key is managing the size of each conversion to stay within your desired tax bracket and below IRMAA thresholds. Here’s how different conversion amounts affect taxes for a married couple with \$80,000 in other income (Social Security + pension).

Conversion	Total Taxable	Marginal Tax	Approx. Tax on	IRMAA Impact
Amount	Income	Bracket	Conversion	(2 years later)
\$20,000	\$100,000	12%	\$2,400	None

Conversion	Total Taxable	Marginal Tax	Approx. Tax on	IRMAA Impact
Amount	Income	Bracket	Conversion	(2 years later)
\$40,000	\$120,000	22%	\$6,600	None (below \$5,770/yr

The sweet spot for this couple: convert up to \$138,000 per year (bringing total income to \$218,000), staying just below the married IRMAA threshold while filling the 24% bracket. Every dollar converted at 24% that would have been withdrawn later at 24% or higher—plus IRMAA surcharges and Social Security taxation—is a net win.

One caution on precision. That \$138,000 figure lands you exactly on the threshold, and IRMAA has no forgiveness in it. A corrected 1099, a December capital gains distribution from a mutual fund you forgot you owned, a little more interest income than you projected — any of these can put you a few hundred dollars over, and the penalty is a full year of surcharges for both spouses. Leave yourself \$3,000 to \$5,000 of room. And before you pull the trigger in December, call someone. This is the single most common place I see good planning undone by one unwatched number.

The Five-Year Rule

The Roth IRA has a “five-year rule” that often creates confusion. Here’s what you need to know:

For contributions: You can withdraw your contributions at any time, tax-and penalty-free. Only earnings are subject to the five-year rule.

For conversions (if under 59½): Each conversion has its own five-year clock. If you withdraw the converted amount before five years and before age 59½, you'll owe a 10% penalty (but not income tax, since you already paid that).

For conversions (age 59½ or older): No penalty applies, regardless of the five-year clock. You can access converted funds immediately. The five-year rule is essentially irrelevant for most retirees doing conversions after 59½.

For earnings: To withdraw earnings tax-free, you must be 59½ or older AND the Roth IRA must have been open for at least five years from the first contribution or conversion. If you're opening your first Roth IRA, start the clock as soon as possible—even with a small contribution.

The Roth Conversion Ladder

The Roth conversion ladder is a multi-year strategy where you convert a specific amount each year, designed to fill available tax space without triggering IRMAA. Think of it as building a ladder one rung at a time.

Year 1: Convert \$60,000 (fill 22% bracket). Year 2: Convert \$60,000 (fill 22% bracket). Year 3: Convert \$60,000 (fill 22% bracket). Year 4: Convert \$60,000 (fill 22% bracket). Year 5: Convert

\$60,000 (fill 22% bracket). Total converted: \$300,000. Total tax paid: approximately \$66,000. Traditional IRA reduced by \$300,000, plus the Roth has been growing tax-free during this period.

Five years later, you have \$300,000+ in your Roth (with growth), dramatically lower future RMDs, and a permanent tax-free income source.

REAL-WORLD EXAMPLE: The Teacher Who Saved \$94,000

Barbara, a retired teacher, age 61, had \$900,000 in a 403(b) and \$45,000 in Social Security at age 67. Between ages 61 and 66, she converted \$70,000/year (\$420,000 total) to a Roth, paying approximately \$84,000 in conversion taxes (at a blended rate of ~20%).

At age 73, her traditional 403(b) balance was roughly \$600,000 instead of \$1,200,000. Her RMD was approximately \$22,642 instead of \$45,283. Her MAGI stayed below the \$109,000 single IRMAA threshold every year. Over her retirement, she saved approximately \$94,000 in combined income taxes, IRMAA surcharges, and Social Security taxation compared to doing no conversions.

FREQUENTLY ASKED QUESTIONS

Q: Is it too late to start Roth conversions?

A: If you have traditional retirement money and you're still breathing, it's not too late. Even small conversions reduce your future tax burden. The earlier you start, the more years of tax-free growth you capture—but conversions at any age can provide benefits.

Q: Should I use retirement account money to pay the conversion tax?

A: Ideally, no. Pay the conversion tax from outside sources (savings, taxable accounts) so that 100% of the converted amount goes into the Roth. Using retirement funds to pay the tax reduces the amount being converted and can trigger additional taxes and penalties if you're under 59½.

Q: Do Roth conversions affect my health insurance premiums before Medicare?

A: Yes. Before 65, if you're getting ACA marketplace insurance, the conversion income increases your MAGI, which can reduce or eliminate premium subsidies. Factor this in when sizing conversions between 59½ and 65.

Q: Can I convert my 401(k) directly to a Roth IRA?

A: In many cases, yes—through an “in-service” conversion if your plan allows it, or by rolling to a traditional IRA first and

then converting. If you've left the employer, roll the 401(k) to a traditional IRA, then convert at your chosen pace.

CHAPTER 25 CHECKLIST

☐ I understand what a Roth conversion is and how it's taxed ☐ I know the seven benefits of Roth conversions ☐

IRMAA

☐ I understand the five-year rule (and know it's largely irrelevant after 59½) ☐ I've considered a multi-year Roth conversion ladder strategy ☐ I've started (or planned to start) my first Roth IRA to begin the five-year clock **READY TO TAKE THE NEXT STEP?**

Roth conversions are the most powerful tool in retirement tax planning—but the difference between a good conversion strategy and a bad one can be tens of thousands of dollars. I'll model your specific numbers, show you the optimal conversion amount each year, and make sure we don't trigger unnecessary IRMAA. Let's build your Roth ladder together.

Darin Weidauer | 702-706-6564 | darinweidauer@ecos.care

CHAPTER 26

Chapter 26: Annuities

When They Make Sense and When They Don't

No financial product generates more heated debate than annuities. Some advisors love them. Some despise them. TV personalities warn you to “never buy an annuity.” Insurance agents tell you they’re the answer to everything.

The truth, as usual, is somewhere in the middle. Annuities are tools—and like any tool, they’re excellent for certain jobs and terrible for others. Understanding which type of annuity serves which purpose is the key to making a smart decision.

What an Annuity Actually Is

At its core, an annuity is a contract with an insurance company. You give them money (either a lump sum or a series of payments), and in return, they promise to give you a stream of income —either for a set period or for the rest of your life.

That “for the rest of your life” part is the annuity’s unique superpower. No other financial product can guarantee income for life, regardless of how long you live or what the stock market does. Social Security is the closest comparison—and in many ways, an annuity is like buying a personal pension.

Types of Annuities

Annuity Type	How It Works	Risk Level	Best For
Single Premium Immediate (SPIA)	Lump sum in; monthly payments start immediately	Very Low	Guaranteed lifetime income starting now
Deferred Income Annuity (DIA)	Lump sum in; payments start at a	Very Low	Planning guaranteed income for later (e.g., age
Fixed Annuity	future date Guaranteed interest rate for a set period	Low	80+) Safe growth with no market risk
Fixed Indexed	(like a CD) Growth linked	Low-Moderate	Market participation

Annuity Type	How It Works	Risk Level	Best For
Annuity (FIA)	to an index (S&P 500) with a floor		without loss risk
Variable Annuity	(0% min) Invested in sub- accounts (mutual fund- like)	Moderate-High	Growth-oriented investors (watch fees)
Multi-Year Guaranteed (MYGA)	Fixed rate guaranteed for multiple years	Very Low	CD alternative with tax deferral

When Annuities Make Sense

You need guaranteed lifetime income to cover essential expenses. If Social Security and pensions don’t cover your must-pay bills (housing, food, healthcare, utilities), an annuity can close the gap with certainty.

You’re worried about outliving your money. A lifetime annuity eliminates longevity risk. No matter how long you live, the checks keep coming.

You want to reduce investment stress. If market volatility causes you anxiety that affects your quality of life, putting a portion of your portfolio into a fixed or indexed annuity removes that stress from the equation.

You want tax-deferred growth. Annuities grow tax-deferred (you don’t pay taxes on gains until withdrawal). For people who’ve maxed out their 401(k) and IRA contributions, a non-qualified annuity offers additional tax-deferred growth.

When Annuities Don’t Make Sense

You need liquidity. Most annuities have surrender charges (penalties for withdrawing too early), typically lasting 5–10 years. If you might need access to the full lump sum, an annuity is the wrong choice.

You’re being sold a variable annuity with high fees. Variable annuities are notorious for layers of fees: mortality and expense

charges (1.0–1.5%), administrative fees (0.1–0.3%), investment management fees (0.5–1.0%), and optional rider fees (0.5–1.5%). Total fees can exceed 3% per year, which dramatically erodes your returns.

You're using it inside a tax-advantaged account. Since IRAs and 401(k)s are already tax-deferred, putting an annuity inside one adds complexity and fees without additional tax benefit. This is a common sales tactic that rarely serves the client.

You're in poor health. Annuities with lifetime income features are priced based on average life expectancy. If your health suggests a shorter-than-average lifespan, you may not live long enough to recoup your investment.

REAL-WORLD EXAMPLE: The Right Annuity vs. The Wrong One

Right: Helen, age 69, has \$800,000 in savings but no pension. Her Social Security is \$2,100/month. Her essential expenses are \$4,500/month. She has a \$2,400/month gap. Helen purchases a SPIA (Single Premium Immediate Annuity) for \$350,000, generating \$2,450/month for life. Her essential expenses are now 100% covered by guaranteed income. The remaining \$450,000 stays invested for growth and discretionary spending. Helen sleeps well at night.

Wrong: Gary, age 62, was sold a variable annuity inside his IRA by an advisor who earned a 6% commission. The annuity charges 2.8% in annual fees. Over 20 years, those fees consume approximately \$280,000 of what a low-cost index fund would have produced. Gary would have been better served by a simple portfolio of index funds inside his IRA.

FREQUENTLY ASKED QUESTIONS

Q: Are annuity payouts taxable?

A: It depends. For non-qualified annuities (purchased with after-tax money), each payment is partly a return of principal (tax-free) and partly earnings (taxable). For qualified annuities (inside an IRA), payments are fully taxable as ordinary income.

Q: What happens to my annuity when I die?

A: It depends on the contract. Some annuities have a death benefit that pays remaining value to your beneficiary. A life-only SPIA pays nothing after you die (which is why they pay higher monthly amounts). Most contracts offer options for joint-life payments, period-certain guarantees, or return-of-premium riders.

Q: Can I get out of an annuity I already bought?

A: During the surrender period, you'll pay a surrender charge (typically 1-8% depending on the year). After the surrender period, you can cash out freely. Some annuities also allow "free withdrawals" of up to 10% per year without surrender charges.

Q: Should I buy an annuity from the person selling it to me?

A: Be cautious if the person recommending the annuity earns a commission from the sale. Ask how much they're being paid. A good advisor will disclose this transparently. Better yet, work with a fee-based advisor or independent agent who compares multiple products.

CHAPTER 26 CHECKLIST

☐

☐ I've identified whether I have a guaranteed income gap in retirement ☐ I know to avoid variable annuities with high fees

☐ I understand surrender charges and liquidity limitations ☐

I've evaluated whether an annuity belongs inside or outside a retirement account ☐ If considering an annuity, I've compared quotes from multiple carriers **READY TO TAKE THE NEXT STEP?**

Annuities can be your best friend or your worst enemy in retirement. The difference is matching the right product to the right need—and avoiding the sales pressure that pushes people into the wrong one. As an independent agent, I compare annuities from dozens of carriers to find the one that fits your situation. No single company. No sales quota. Just the right product for you.

Darin Weidauer | 702-706-6564 | darinweidauer@ecos.care

C H A P T E R 2 7

Chapter 27: Life Insurance in Retirement

Protection, Legacy, and Tax-Free Income

Most people think of life insurance as something you need when you’re young: protect your family if something happens to you, replace your income, cover the mortgage. But life insurance in retirement serves a completely different set of purposes—and for many retirees, it’s one of the most underutilized tools in their financial toolkit.

Why Life Insurance Still Matters After 65

In retirement, life insurance can serve four powerful purposes:

Income replacement for a surviving spouse: When one spouse dies, one Social Security check disappears. If the surviving spouse depends on that income, a life insurance death benefit can bridge the gap. Debt protection: If you still carry a mortgage, car loan, or other debts, a death benefit ensures your surviving spouse isn’t burdened with payments. Legacy and inheritance: Life insurance proceeds pass to beneficiaries income-tax-free (in most cases), making it an efficient wealth transfer vehicle. Tax-free access to cash value: Certain permanent life insurance policies build cash value that you can borrow against tax-free, providing supplemental income that doesn’t appear in your MAGI—and therefore doesn’t trigger IRMAA.

Types of Life Insurance in Retirement

Policy Type	Purpose	Cash Value?	Premium	Best For
Term Life	Temporary coverage for a	No	Lowest	Covering specific debts or income
Whole Life	Permanent coverage with guaranteed cash value	Yes (guaranteed)	Highest	Legacy planning, conservative cash value

Policy Type	Purpose	Cash Value?	Premium	Best For
Universal Life (UL)	Flexible permanent coverage	Yes (interest-based)	Moderate-High	Flexible premium needs
Indexed Universal Life (IUL)	Permanent coverage with market-linked growth	Yes (index-linked, floored at 0%)	Moderate-High	Tax-free supplemental income via loans
Final Expense	Small whole life policy (\$5K-\$25K)	Yes (small)	Low	Covering burial/funeral costs

The Tax-Free Income Strategy: Cash Value Life Insurance

Here’s where life insurance gets interesting for retirement income planning. Certain permanent policies—particularly Indexed Universal Life (IUL)—build cash value over time. You can borrow against that cash value tax-free, because policy loans are not considered income by the IRS.

That means: no income tax on the loan, no MAGI increase, no IRMAA impact, no effect on Social Security taxation. The loan is eventually repaid from the death benefit. If the policy is properly structured, this creates a stream of tax-free income that’s invisible to every government formula that penalizes retirees for having “too much income.”

This strategy is not for everyone. It requires significant premiums over many years to build adequate cash value. It works best for people who are healthy enough to qualify for coverage, have maximized their Roth contributions, and want an additional tax-free income layer.

REAL-WORLD EXAMPLE: The Life Insurance Income Bridge

Robert, age 66, purchased an IUL policy at age 50 and funded it aggressively for 15 years. By age 66, his policy’s cash value was approximately \$320,000. Starting at age 68, Robert began taking \$15,000/year in tax-free policy loans. This income doesn’t appear on his tax return. It doesn’t affect his MAGI, IRMAA, or Social Security taxation. Over 20 years, Robert

accesses \$300,000 in tax-free income. When Robert passes away, the remaining death benefit (minus outstanding loans) goes to his children, also income-tax-free.

FREQUENTLY ASKED QUESTIONS

Q: I'm 65. Is it too late to buy life insurance?

A: Not necessarily. Term life coverage may be available into your 70s, though premiums increase significantly with age. Final expense (burial) policies are available into the 80s with simplified underwriting. Cash value policies work best when started earlier, but some options exist for later purchases.

Q: Are life insurance proceeds taxable?

A: Death benefit proceeds are generally received income-tax-free by your beneficiaries. However, if the total estate exceeds federal estate tax exemption limits (\$15 million per individual in 2026, made permanent and indexed for inflation under the One Big Beautiful Bill Act of 2025), the proceeds could be subject to estate taxes unless the policy is held in an irrevocable trust.

Q: Should I cancel my life insurance in retirement?

A: Not automatically. Evaluate whether anyone depends on your income. If you have a surviving spouse who'd lose Social Security income, outstanding debts, or estate planning objectives, keeping coverage may be essential. If no one depends on you financially and you have no debts, the premiums may be better used elsewhere.

CHAPTER 27 CHECKLIST

- ☐ I've evaluated whether my surviving spouse needs income replacement
- ☐ I've reviewed my current life insurance policies and coverage amounts
- ☐ I've considered cash value life insurance as a tax-free income supplement
- ☐ I've updated my beneficiary designations on all policies

READY TO TAKE THE NEXT STEP?

Life insurance in retirement is about more than death benefits. It's about protecting your spouse, creating a tax-free income

stream, and leaving a legacy. I can review your current policies, assess your needs, and recommend the right coverage—or help you determine that you don’t need any at all. Honest advice. That’s the deal.

Darin Weidauer | 702-706-6564 | darinweidauer@ecos.care

C H A P T E R 2 8

Chapter 28: Real Estate in Retirement

Equity, Income, and the IRMAA Trap

Your home may be the largest asset you own. It's also one of the most emotionally charged financial decisions in retirement. Do you stay? Downsize? Rent? Sell and invest the proceeds? Use a reverse mortgage?

Each path has financial consequences—and as we covered in Chapter 13, the IRMAA implications of a home sale can be significant. This chapter goes beyond the home sale trap to look at how real estate fits into your complete retirement income strategy.

The Five Ways to Use Real Estate in Retirement

Option 1: Stay and Age in Place

Staying in your current home eliminates moving costs and preserves your community. The key consideration is whether the home is affordable on a fixed income (property taxes, insurance, maintenance, utilities) and whether it's physically suitable as you age (stairs, accessibility, proximity to healthcare). Many retirees underestimate maintenance costs—budget 1-2% of the home's value annually.

Option 2: Downsize and Invest the Difference

Selling a larger home and buying a smaller one frees up equity. If you sell for \$600,000, buy for \$350,000, and net \$200,000 after moving costs and capital gains, that \$200,000 can generate \$8,000–\$10,000/year in investment income for decades. Remember to plan the timing around IRMAA (see Chapter 13).

Option 3: Sell and Rent

Renting eliminates property tax, major maintenance, and the risk of declining home values. It also converts 100% of your home equity to liquid assets. The trade-off is that rent can

increase over time and you lose the potential for home price appreciation.

Option 4: Rental Property Income

If you own a second property or convert your primary home to a rental after relocating, rental income can be a steady cash flow source. Net rental income is included in your MAGI for IRMAA. You can deduct expenses like depreciation, repairs, insurance, and property management. But being a landlord at 75 isn’t for everyone—consider whether the hassle justifies the income.

Option 5: Reverse Mortgage (HECM)

A Home Equity Conversion Mortgage (HECM) allows homeowners aged 62+ to borrow against their home equity without making monthly payments. The loan is repaid when the homeowner sells, moves out, or passes away.

Key fact for IRMAA planning: reverse mortgage proceeds are not income. They’re loan advances. That means they don’t count in your MAGI, don’t trigger IRMAA, don’t affect Social Security taxation, and aren’t taxable. For some retirees, a reverse mortgage is a legitimate tax-free income supplement.

However, reverse mortgages reduce the equity available to your heirs, have upfront costs (mortgage insurance premiums, origination fees, closing costs), and carry risk if home values decline. They should be evaluated carefully—not as a last resort, but as a strategic option.

Real Estate, Capital Gains, and IRMAA: The Complete Picture

Real Estate Event	Tax Impact	IRMAA Impact	Planning Note
Sell primary home (gain under exclusion)	No capital gains tax	No IRMAA impact	Clean transaction
Sell primary home (gain ABOVE exclusion)	Capital gains tax on excess	Excess gain counts in MAGI	Time the sale; see Ch. 13
Sell rental/investment property	Full gain taxable (no exclusion)	Full gain counts in MAGI	Consider 1031 exchange
Receive rental income	Net income	Net income	Maximize

Real Estate Event	Tax Impact	IRMAA Impact	Planning Note
	taxable	counts in MAGI	deductions
Reverse mortgage proceeds	Not taxable (it's a loan)	No IRMAA impact	IRMAA-invisible income source
Sell home and do	Conversion is taxable	Conversion counts in	Stagger over multiple years
Roth conversion with proceeds		MAGI	

REAL-WORLD EXAMPLE: The Downsizer’s Smart Sequence

Ann and Bill, both 66, sold their Las Vegas home for \$750,000 (purchased in 2002 for \$190,000, with \$60,000 in improvements; basis: \$250,000). Their gain: \$500,000. The married exclusion covers the full \$500,000—no taxable gain, no IRMAA impact. They bought a smaller home for \$380,000, invested \$300,000 of the proceeds in a diversified portfolio, and used \$20,000 for moving and furnishing. The \$300,000 portfolio, generating an average 5% return, adds \$15,000/year to their retirement income indefinitely.

Now compare: if Ann and Bill’s home had sold for \$850,000, their gain would be \$600,000, with \$100,000 taxable after the exclusion. That \$100,000 gain, added to their \$160,000 regular income, pushes their MAGI to \$260,000—triggering the first IRMAA surcharge tier for both of them two years later. Extra cost: roughly \$2,297/year in surcharges (at 2026 rates). By timing the sale one year earlier (before age 63 for the lookback), or by taking a lower offer, they could have avoided the entire surcharge.

FREQUENTLY ASKED QUESTIONS

Q: Should I pay off my mortgage before retiring?

A: There’s no universal answer. Paying off the mortgage eliminates a monthly payment and reduces stress. But if your mortgage rate is low (3–4%), the money might earn more invested. Consider your cash reserves, your risk tolerance, and the psychological value of being debt-free.

Q: Does a 1031 exchange avoid IRMAA?

A: Yes, in the year of exchange. A 1031 exchange defers the capital gain entirely, meaning no taxable gain shows up on your

return that year. However, the gain is deferred, not eliminated — when you eventually sell the replacement property, the deferred gain comes due.

Q: Is a reverse mortgage safe?

A: HECMs are federally insured through FHA. You can never owe more than the home's value (the “non-recourse” feature). But you must maintain the home, pay property taxes, and keep homeowners insurance current. Failing to do so can trigger default.

Q: Can I sell my home and move into a rental to reduce IRMAA?

A: Yes. If you sell and rent, the sale proceeds are liquid and can be invested for income. The rental payment itself doesn't affect MAGI. Just plan the sale timing to manage IRMAA as described in Chapter 13.

CHAPTER 28 CHECKLIST

☐ I've evaluated whether to stay, downsize, rent, or use a reverse mortgage ☐ I know my home's approximate cost basis (purchase price + improvements) ☐
☐ I've considered the IRMAA impact of a home sale in my current or planned timing ☐ If I own rental property, I've factored net rental income into my MAGI ☐ I've explored 1031 exchanges if selling investment property ☐ I understand that reverse mortgage proceeds are not taxable income **READY TO TAKE THE NEXT STEP?**

Real estate is often the biggest asset—and the biggest wild card—in retirement. Whether you're selling, downsizing, renting, or considering a reverse mortgage, I can help you model the tax and IRMAA consequences before you make a move. One conversation now can save you thousands later.

Darin Weidauer | 702-706-6564 | darinweidauer@ecos.care

CHAPTER 29

Chapter 29: The April Surprise

Setting Up Tax Withholding So Tax Day Never Shocks You

You’ve spent the last eight chapters building your retirement income stack — Social Security, IRA withdrawals, maybe a pension, an annuity, or rental income. Here’s the part nobody warns you about: not one of those income sources withholds taxes automatically the way your paycheck did.

For forty years, your employer quietly held back federal taxes from every check. You never had to think about it. Then you retire, the paychecks stop, and suddenly you are the payroll department. Get it wrong, and the first April after you retire delivers a tax bill for thousands of dollars — plus an underpayment penalty on top. I call it the April Surprise, and I see it every single spring.

Why the First Year of Retirement Is the Danger Zone

The IRS operates on a pay-as-you-go system. You’re required to pay tax on income as you receive it during the year — through withholding, quarterly estimated payments, or both. Miss the mark by too much and the IRS charges an underpayment penalty that works like interest: 7% annually in the first quarter of 2026, 6% in the second, and back to 7% for the quarter beginning July 1, 2026 — compounded daily.

The good news: every retirement income source has a withholding switch. You just have to know which form flips it.

Your Withholding Control Panel

Income Source	The Form	Default If You Do Nothing	The Smart Move
Social Security	Form W-4V	\$0 withheld	Choose 7%, 10%, 12%, or 22% (only these four are allowed)
Pension / annuity (monthly)	Form W-4P	Withheld as if single, no adjustments	Complete W-4P so it reflects your real tax

Income Source	The Form	Default If You Do Nothing	The Smart Move
IRA / 401(k) withdrawal (one-time)	Form W-4R	10% withheld unless you opt out or change it	picture Set the percentage to match your actual bracket
Interest, dividends, capital gains	No withholding form	\$0 withheld	Cover with quarterly estimates or extra withholding elsewhere

Source: IRS Forms W-4V, W-4P, and W-4R; IRS Publication 505, Tax Withholding and Estimated Tax.

Notice the trap in row one: Social Security withholds nothing unless you file Form W-4V. Neither do most brokerage accounts. A couple drawing \$60,000 in Social Security and \$40,000 in IRA withdrawals can easily sail through the year with almost no tax paid in — and owe the entire bill in April.

Two Ways to Stay Square with the IRS

Option 1: Withholding. The simplest path. The IRS treats withheld tax as if it were paid evenly throughout the year — no matter when it actually comes out . That timing quirk creates one of the most useful tricks in retirement tax planning, which we’ll get to in a moment.

Option 2: Quarterly estimated payments (Form 1040-ES). Four payments a year, due April 15, June 15, September 15, and January 15. They work fine, but you have to remember them, the “quarters” aren’t actually equal, and a missed payment starts the penalty clock for that period even if you catch up later.

The Safe Harbor Rules — Your Penalty-Proof Numbers

You avoid the underpayment penalty entirely if your withholding and timely estimates add up to the smaller of:

90% of this year's total tax , or
100% of last year's total tax
(110% if your adjusted gross
income was over \$150,000).

• •

And if you owe less than \$1,000 after withholding, there's no penalty at all. For most new retirees, the cleanest strategy is the prior-year safe harbor: look at last year's total tax on your return, make sure at least that much (or 110%) gets paid in this year, and you're penalty-proof no matter how your income actually lands.

The December RMD Trick

Here's the professional-grade move built on that withholding timing quirk. Because withholding is treated as paid evenly across the year, a single IRA withdrawal in December with heavy withholding can retroactively cure an entire year of underpayment.

Say it's November and you realize you've paid in almost nothing. Take your required minimum distribution (or any IRA withdrawal) in December and instruct the custodian to withhold 40, 50, even 90 percent for taxes. The IRS treats that money as if it arrived in four tidy quarterly installments. A quarterly estimate made in December can't do that — it only counts for the fourth quarter. This one technique has saved my clients thousands in penalties.

Don't Forget the New \$6,000 Senior Deduction

For tax years 2025 through 2028, taxpayers 65 and older get an extra \$6,000 deduction (\$12,000 if both spouses qualify), phasing out above \$75,000 of modified adjusted gross income for single filers and \$150,000 for joint filers. If you set your withholding based on your old tax bill, you may actually be over-withholding now. Run the numbers both ways — the goal is to hit the safe harbor, not to give the IRS an interest-free loan.

REAL-WORLD EXAMPLE: Bill and Janet's First April

Bill and Janet retired in January at 66. Their income: \$58,000 in combined Social Security, a \$24,000 pension, and \$38,000 in IRA withdrawals. They never filed a W-4V, the pension withheld

a token amount, and they waived withholding on the IRA checks because “we’ll deal with it at tax time.” Tax time dealt with them: a \$9,400 federal bill plus an underpayment

penalty. The fix took twenty minutes — a W-4V electing 10% on Social Security, a corrected W-4P, and 15% withholding on IRA distributions. The next April, their bill was \$212.

FREQUENTLY ASKED QUESTIONS

Q: Can I just pay everything I owe on April 15?

A: You can — but the IRS will add the underpayment penalty, calculated quarter by quarter at rates that have run 6–7% in 2026, compounded daily. Paying as you go is almost always cheaper.

Q: How do I start withholding from Social Security?

A: Download Form W-4V from irs.gov or ssa.gov, choose 7%, 10%, 12%, or 22%, and mail or deliver it to your local Social Security office. It typically takes effect within a month or two.

Q: Do Roth IRA withdrawals need withholding?

A: Qualified Roth withdrawals are tax-free, so no. That’s one more reason the Roth conversions we covered in Chapter 25 simplify life later — but remember, the conversion itself is taxable in the year you do it, and that usually calls for withholding or an estimate.

Q: What about state taxes?

A: Excellent question — and it deserves its own chapter. Turn the page.

CHAPTER 29 CHECKLIST

- ☐ I’ve identified every income source I’ll have in retirement and how (or whether) it withholds taxes
- ☐ I’ve filed Form W-4V for Social Security withholding, or deliberately chosen not to ☐ I’ve updated Form W-4P with each pension or annuity payer ☐
- ☐ I understand the December RMD withholding technique ☐
- I’ve re-checked my withholding against the new \$6,000 senior

deduction > 📞 READY TO TAKE THE NEXT STEP? Your first year of retirement is the easiest year to get taxes wrong — and the easiest to get right with a little setup. Bring last year's tax return to a free, no-obligation Retirement Blueprint Session and we'll build your withholding plan together, in plain English. Darin Weidauer | ECOS | 702-706-6564 | darinweidauer@ecos.care

C H A P T E R 3 0

Chapter 30: The State Tax Map

Where You Live Changes What You Keep

Setting up your federal withholding is half the tax picture. The other half is drawn on a map. Two couples with identical incomes — same Social Security, same pension, same IRA withdrawals — can owe thousands of dollars a year apart in taxes based on nothing but their zip code.

I’m not telling you to move. Most people shouldn’t uproot their lives for a tax break. But if you’re already weighing a move — closer to grandkids, warmer winters, a smaller house — the state tax picture belongs on your decision checklist. And if you’re staying put, you should still know exactly what your state takes, because it changes your withholding math from Chapter 29.

The Nine States with No Income Tax at All

Alaska, Florida, Nevada, New Hampshire, South Dakota, Tennessee, Texas, Washington, and Wyoming levy no state income tax on wages or retirement income. Your Social Security, pension, IRA withdrawals, and annuity payments all arrive state-tax-free. (Here in Nevada, this is one reason so many of my clients’ friends keep showing up.)

One nuance: Washington applies a capital gains tax to very large gains, and “no income tax” never means “no taxes” — these states fund themselves through sales and property taxes instead. More on that below.

Only 8 States Still Tax Social Security in 2026

This list has been shrinking for years. Missouri, Nebraska, and Kansas dropped off in 2024. West Virginia completed its three-year phase-out on January 1, 2026 — Social Security is now fully exempt there. That leaves just eight states taxing benefits in 2026, and every one offers income-based relief:

State	Who Actually Pays in 2026
Colorado	65+ fully deduct taxed benefits; ages 55–64
	fully deduct only if AGI is \$75,000 or less (\$95,000 joint)
	\$25,000 (\$32,000 joint) Exempt under \$100,000 AGI (\$150,000 joint)
New Mexico	
Rhode Island	Exempt at full retirement age with income

AGI (\$90,000 joint) — thresholds raised for 2026 joint); partial exemption slightly above

Sources: State departments of revenue; West Virginia Tax Division (HB 4880 phase-out schedule); Money and Kiplinger 2026 state tax surveys. Thresholds change frequently — verify with your state before acting.

Notice something? In most of these eight states, a typical retiree under the income limits pays nothing on Social Security anyway. The people actually affected are higher-income retirees — the same folks managing IRMAA in Chapter 11.

The Pension and IRA Question — Where the Real Money Is

For many of my clients, Social Security is the smaller tax issue. The bigger one: how your state treats pensions, 401(k), and IRA withdrawals.

The standouts: Illinois, Iowa, Mississippi, and Pennsylvania exempt most retirement income — pensions and retirement-account withdrawals included — even though they have a state income tax. Michigan joins them in 2026 , completing its phase-out: most retirement income is now exempt up to \$67,610 for single filers and \$135,220 for joint filers.

Military retirees: more than 20 states now fully exempt military retirement pay, and the list keeps growing. If you wore the uniform, check your state’s specific military exemption — it’s often more generous than the general pension rule (we cover the full veteran picture in Chapter 38).

Don't Fall for the Income-Tax-Only Trap

A state with no income tax can still cost you more overall. Before any move, compare all four:

1.	Income tax — on Social Security, pensions, and withdrawals, as above.
2.	Property tax — and, critically, whether the state offers a senior freeze, homestead exemption, or circuit-breaker credit. A “cheap” state with high property taxes on the home you plan to keep for
3.	25 years is not cheap. Sales tax — combined state and local rates run from zero to over 9%.
4.	Estate or inheritance tax — the federal exemption is \$15 million in 2026, but a dozen-plus states impose their own estate or inheritance taxes at far lower thresholds.

REAL-WORLD EXAMPLE: The \$2,900 Zip Code

Dave and Ellen, both 67, split the year between Minnesota and a paid-off place near Tampa. Income: \$52,000 Social Security, \$48,000 in IRA withdrawals. In Minnesota, their AGI kept them just over the exemption thresholds — costing roughly \$2,900 a year in state tax between partially taxed benefits and fully taxed IRA withdrawals. By properly establishing Florida domicile (183+ days, driver’s license, voter registration, homestead filing), that line item went to zero. Over a 25-year retirement, that’s more than \$70,000 — enough to fund the long-term care premiums we’ll discuss in Part V.

FREQUENTLY ASKED QUESTIONS

Q: Is it enough to just spend winters in a no-tax state?

A: No. Domicile is a legal status, not a vibe. States like Minnesota, California, and New York audit aggressively. You generally need 183+ days, plus consistent evidence — license, voting, homestead, doctors, even where your dog goes to the vet.

Q: My state taxes Social Security. Should I panic?

A: Probably not. Check the thresholds in the table above — most retirees fall under them. If you're above them, the Roth conversion and income-planning strategies from Chapters 14 and 25 help at the state level too.

Q: Do states tax Roth withdrawals?

A: Qualified Roth withdrawals are tax-free in every state — one more argument for the Roth strategies in Chapter 25.

CHAPTER 30 CHECKLIST

☐ I know whether my state taxes Social Security — and whether my income is over or under the exemption threshold

☐ I know how my state treats pension and IRA withdrawals ☐

☐ If considering a move, I've compared income, property, sales, and estate taxes — not just one ☐ If splitting time between states, I understand the 183-day domicile rules > 📞 **READY TO TAKE THE NEXT STEP?** Thinking about where to spend your retirement years — or wondering what your current state really costs you? Bring your questions to a free Retirement Blueprint Session and we'll look at the whole picture: taxes, healthcare networks, and the plans available where you'd land. Darin Weidauer | ECOS | 702-706-6564 | darinweidauer@ecos.care

PART FIVE

PART V — PROTECTING WHAT YOU'VE BUILT

You spent a lifetime building your retirement. A single unplanned

event can erase it in months. These chapters are your shield.

CHAPTER 31

Chapter 31: Where Will You Live?

Aging in Place, Downsizing, CCRCs, and the Truth About Reverse

Mortgages

We’ve protected your income. Now we protect everything it pays for — starting with the roof over your head. Because before you can answer this part’s big question (“who will take care of you, and who pays for it?”), you have to answer an even more basic one: where will you be living when you need that care?

According to AARP, about three-quarters of adults over 50 say they want to stay in their current home as they age. It’s a beautiful goal. It’s also a plan that fails more often than it should — not because staying home is wrong, but because nobody engineered the house, the money, or the backup plan to make it work. Let’s engineer yours.

Option 1: Age in Place — The Home You Love, Upgraded

Staying put is usually the cheapest option if the house cooperates. Walk through your home with fresh eyes: stairs, bathtubs, narrow doorways, that basement laundry. Then price the fixes:

Modification	Typical Cost
Grab bars and lever handles	\$100-\$500 installed
Comfort-height toilet, hand-held shower	\$300-\$1,000
Walk-in shower conversion	\$3,000-\$15,000
Stair lift	\$3,000-\$8,000
Widened doorways (per door)	\$700-\$2,500
First-floor bedroom/bath addition	\$30,000-\$100,000+

Source: Typical contractor ranges; costs vary widely by region. Veterans: ask about VA home modification grants (Chapter 38).

A few thousand dollars of modifications, done before the fall instead of after, is some of the best money in retirement. The

bigger aging-in-place costs are the invisible ones — lawn care, snow removal, and eventually the in-home care we'll price in Chapter 32 (\$6,483 a month median for a home health aide). Staying home is not automatically the cheap option once care enters the picture.

Option 2: Downsize — Unlock the Equity, Mind the Traps

Selling the four-bedroom and buying the two-bedroom can free up hundreds of thousands of dollars and cut your carrying costs. Two traps to respect:

The tax trap. The home-sale exclusion shelters \$250,000 of gain (\$500,000 married) — but after decades of appreciation, many long-time owners blow past it. Gain above the exclusion is taxable and lands in your MAGI, which can trigger the IRMAA surcharges we covered in Chapter 13. Time the sale with your Medicare premiums in mind.

The math trap. A smaller house isn't automatically cheaper — condo fees, a new market, and moving costs eat surprising chunks. Run the full before/after monthly budget, not just the sale price.

Option 3: The Care Continuum — Independent Living,

Assisted Living, and CCRCs

Independent living / 55+ communities bundle housing, maintenance, and social life — you're buying convenience and community, not care.

Assisted living adds help with daily activities at a national median of about \$5,900 a month (we'll break down all care costs in the next chapter).

Continuing Care Retirement Communities (CCRCs) — also called Life Plan Communities — put the whole continuum on one campus: independent living, assisted living, and skilled nursing. You buy in once and move through levels of care as needed. The price of that certainty:

...	Entry fees commonly run from around \$100,000 to over \$1 million depending on market, unit, and contract (a substantial portion is often refundable to your estate — read the contract). Monthly fees typically \$3,000-\$7,000+.
	Contract types matter enormously: Type A (“life care”) charges the highest entry fee but locks your monthly rate roughly flat even if you need full nursing care. Type B (modified) discounts future care. Type C (fee-for-service) is cheapest to enter — and you pay full market rates when care begins.

Before writing a six-figure entry check, have an attorney review the contract and ask for the community’s audited financials. A CCRC’s promise is only as good as its balance sheet.

Option 4: The Reverse Mortgage — Powerful Tool, Sharp Edges

A reverse mortgage lets homeowners 62 and older convert home equity into cash — a lump sum, monthly payments, or (most usefully) a standby line of credit — with no required monthly mortgage payment. The loan comes due when you sell, move out for good, or pass away.

The version worth discussing is the FHA-insured Home Equity Conversion Mortgage (HECM) . For 2026, FHA counts home value up to \$1,249,125 (up from \$1,209,750 in 2025) when calculating your proceeds. Key protections: it’s non-recourse — you or your heirs never owe more than the home is worth — an eligible non-borrowing spouse can remain in the home, and HUD requires independent counseling before you can sign.

Now the sharp edges, because this is where the TV commercials go quiet:

• • •

It's expensive money. Upfront FHA mortgage insurance of 2% of the home's value, plus 0.5% annually on the balance, plus origination fees (capped at \$6,000) and closing costs. The balance grows every year you hold it. You must keep up property taxes, insurance, and maintenance — fall behind and the loan can be called due. This is how people actually lose homes with reverse mortgages.

It shrinks the inheritance. Every dollar drawn, plus compounding interest, comes out of what the house leaves behind.

When it shines: a house-rich, cash-modest retiree committed to staying put, using a line of credit as a standby buffer — instead of selling investments in a down market or as a last-resort care fund. When it doesn't: funding a lifestyle the income can't support, or any situation where you may move within a few years.

REAL-WORLD EXAMPLE: Two Sisters, Two Right Answers

Carol, 74, loves her paid-off ranch house. She spent \$11,000 on a walk-in shower, grab bars, and a stair lift to the garage, and opened a HECM line of credit she hasn't touched — her "care faucet," ready if home health aides ever become necessary. Her sister Dianne, 77, a widow with a big colonial and a bad hip, sold instead (\$480,000 gain — under the exclusion, no IRMAA spike), and used part of the proceeds for a Type A CCRC entry fee. Three years later Dianne moved to the assisted-living wing on campus; her monthly fee barely moved. Two different answers. Both engineered, not improvised.

FREQUENTLY ASKED QUESTIONS

Q: Will Medicare pay for home modifications?

A: Original Medicare generally doesn't pay to modify your home. Some Medicare Advantage plans offer limited safety-modification or in-home support benefits — check your plan's Evidence of Coverage. Veterans may qualify for VA grants (Chapter 38).

Q: Can I lose my home with a reverse mortgage?

A: You can't lose it for missing mortgage payments — there are none. You can lose it for not paying property taxes or homeowner's insurance, or for moving out permanently. Those obligations are non-negotiable.

Q: Is a CCRC entry fee tax-deductible?

A: Often a portion is — communities issue a letter stating the percentage attributable to prepaid medical care, which may count as a medical expense deduction. Ask before you buy, and confirm with your tax professional.

CHAPTER 31 CHECKLIST

☐ I've honestly assessed whether my current home can support me at 80 and 90 — stairs, bathrooms, maintenance, distance from family

☐ I've priced the modifications that would make aging in place safe ☐ If downsizing, I've checked my gain against the \$250,000/\$500,000 exclusion and the IRMAA calendar

☐ If considering a CCRC, I know the contract type (A, B, or C) and have seen the community's financials

☐ I understand what a reverse mortgage costs, what it protects, and what it puts at risk ☐

> 📞 **READY TO TAKE THE NEXT STEP?** Your home is probably your biggest asset and your most emotional decision. Before you modify, sell, buy in, or borrow against it, let's put the numbers on one page — housing, care costs, taxes, and Medicare — in a free Retirement Blueprint Session . Darin Weidauer | ECOS | 702-706-6564 | darinweidauer@ecos.care

C H A P T E R 3 2

Chapter 32: Long-Term Care

The Risk That Can Erase a Lifetime of Savings

I need to tell you something uncomfortable. It’s something most financial advisors dance around, most families avoid discussing, and most retirees refuse to think about until it’s too late.

Long-term care is the single greatest financial risk in retirement. It is the one expense that can take a carefully built retirement nest egg—\$500,000, \$1,000,000, even more—and drain it to zero in just a few years. And the odds of needing it are far higher than most people realize.

As a gerontologist—a specialist in aging—I’ve studied this risk extensively. I’ve seen families devastated by it. And I’ve helped hundreds of clients build plans that protect them from it. This chapter will give you the facts, the numbers, and the strategies you need.

The Numbers That Should Keep You Up at Night

According to the Department of Health and Human Services, approximately 70% of Americans who reach age 65 will need some form of long-term care during their remaining years. Not 10%. Not 30%. Seventy percent.

The average length of long-term care need is approximately 3 years for women and 2.2 years for men. But averages hide the extremes: roughly 20% of people who need long-term care will need it for more than 5 years. Alzheimer’s patients, in particular, may need care for 8 to 12 years.

2026 Long-Term Care Costs by Type

Type of Care	National Median Monthly Cost	National Median Annual Cost	5-Year Cost
Nursing Home (Semi-Private Room)	\$9,277	\$111,325	\$556,625
Nursing Home (Private Room)	\$10,646	\$127,750	\$638,750

Type of Care	National Median Monthly Cost	National Median Annual Cost	5-Year Cost
Assisted Living Facility	\$5,900	\$70,800	\$354,000
Home Health Aide (full-time)	\$6,483	\$77,796	\$388,980
Homemaker Services (full-time)	\$6,292	\$75,504	\$377,520
Adult Day Care	\$2,200	\$26,400	\$132,000

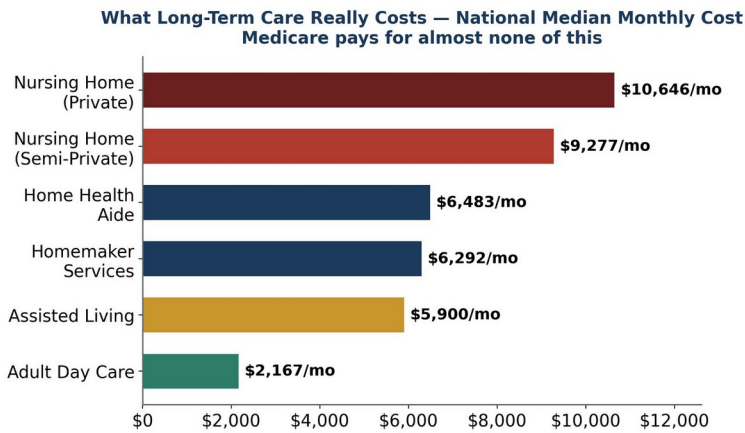
Source: CareScout Cost of Care Survey 2024 (Genworth) — the most recent published national medians. Expect 3–5% annual increases.

National median monthly cost of long-term care by setting. Medicare covers almost none of this.

Long-Term Care Costs by State (Sample)

Long-term care costs vary enormously by geography. Here are some examples for a semi-private nursing home room:

State	Monthly Cost (Semi-Private)	Annual Cost	Notes
Nevada	\$9,628	\$115,536	Above national average; Las Vegas area higher growing senior population
Ohio	\$7,756	\$93,072	continental U.S. Midwest average
Colorado	\$9,125	\$109,500	Mountain West



Source: CareScout Cost of Care Survey 2024 (Genworth) — the most recent published national medians. Costs typically rise 3-5% per year.

National median monthly cost of long-term care by setting.

premium

What Medicare Does NOT Cover

This is where the misconception lives. Most people believe that Medicare will cover them if they need long-term care. It won't.

Medicare covers skilled nursing facility care for up to 100 days per benefit period—but only after a qualifying 3-day inpatient hospital stay, only for skilled care (not custodial care), and with daily coinsurance of \$217.00 per day starting on day 21. After 100 days, Medicare coverage ends entirely.

Custodial care—the kind of care most people actually need (help with bathing, dressing, eating, toileting, transferring, and continence—the Activities of Daily Living, or ADLs)—is not covered by Medicare. Not for one day. Not for one dollar.

Original Medicare doesn't cover it. Medicare Advantage doesn't cover it. Medigap doesn't cover it. This is a gap so large that it can swallow a lifetime of savings.

The Six Activities of Daily Living (ADLs)

Long-term care insurance policies typically trigger benefits when you can't perform two or more of the six Activities of Daily Living without assistance, or when you have a severe

cognitive impairment (such as Alzheimer’s or dementia). The six ADLs are:

ADL	What It Means
Bathing	Ability to wash yourself in a tub, shower, or

(or manage through appliances)

What Happens If You Don’t Plan

Without long-term care coverage, you have three options—none of them good:

Option 1: Self-insure. Pay out of pocket from your savings and investments. At \$9,000+/month for a nursing home, a \$500,000 retirement account lasts roughly 4–5 years. And that’s if it’s your only expense.

Option 2: Rely on family. Your spouse, children, or other family members provide care. This is the most common “plan” in America—and it’s devastating. Family caregivers suffer higher rates of depression, anxiety, and physical health problems. They often reduce their own work hours or leave jobs entirely, sacrificing their own financial security.

Option 3: Spend down to Medicaid. If you exhaust nearly all of your assets, you may qualify for Medicaid, which does cover long-term care. But Medicaid eligibility requires near-poverty: in most states, countable assets must be below \$2,000 for an individual. You lose the freedom to choose your facility, your caregiver, or the level of care you receive.

⚠️ ** THE SPOUSE IMPOVERISHMENT CRISIS**

When one spouse needs nursing home care and the couple “spends down” to qualify for Medicaid, the “community spouse” (the one still living at home) faces strict limits on how much they can keep. In 2026, the community spouse can retain a maximum of approximately \$157,920 in countable assets (Community Spouse Resource Allowance). Everything above that must be spent on care before Medicaid kicks in. The community spouse also faces minimum monthly income allowances, which can leave them with barely enough to

survive. I’ve seen it happen: one spouse enters a nursing home, and the other is left financially gutted.

Types of Long-Term Care Insurance

There are three main approaches to insuring against long-term care costs. Each has different strengths.

1. Traditional Long-Term Care Insurance

Traditional LTC insurance is a standalone policy that pays daily or monthly benefits when you need long-term care (triggered by inability to perform 2+ ADLs or cognitive impairment). You choose a daily benefit amount, a benefit period, an elimination period (waiting period before benefits begin), and optional inflation protection.

Pros: Specifically designed for LTC. Can be customized to your exact needs. Tax-qualified premiums may be deductible. Provides substantial daily benefits (\$150-\$400+ per day).

Cons: Premiums can increase over time (insurers have raised rates significantly in recent years). If you never need care, you don’t get your premiums back (use-it-or-lose-it). Underwriting is health-based—if you wait too long, you may not qualify.

2. Asset-Based (Hybrid) Long-Term Care Insurance

This is the product I recommend most often, and it’s covered in full detail in Chapter 33. Asset-based LTC combines life insurance or an annuity with long-term care benefits. If you need care, it pays for care. If you don’t, your beneficiaries receive a death benefit or you get your money back. You never lose your premium.

3. Short-Term Care Insurance

Short-term care insurance covers care for periods of up to 12 months (sometimes up to 2 years). It’s more affordable than traditional LTC and easier to qualify for. Covered in Chapter 34.

Feature	Traditional LTC	Asset-Based (Hybrid)	Short-Term Care
Benefit Period	2-5+ years	2-6+ years	Up to 360 days

Feature	Traditional LTC	Asset-Based (Hybrid)	Short-Term Care
			typically
Monthly Premium	\$150-\$500+ (can increase)	Single premium or fixed annual	\$50-\$150 (level)
Premium Increases?	Yes (common in recent years)	No (locked in)	No (typically level)
Money Back If Not Used?	No (use-it-or-lose-it)	Yes (death benefit or cash back)	No
Health Underwriting	Full medical underwriting	Full medical underwriting	Simplified underwriting
Maximum Issue Age	Typically 75-80	Varies (up to 80-85)	Up to 89 in some cases
Tax-Qualified?	Yes (premiums may be deductible)	Yes (1035 exchange possible)	Varies
Best For	Comprehensive multi-year coverage	People who want guaranteed return of	Bridge coverage or budget-conscious

premium

When to Buy Long-Term Care Insurance

Timing matters enormously. Here’s the general framework:

Ages 50–60: The sweet spot. You’re likely healthy enough to qualify, premiums are lower, and you have time to lock in rates before they rise. If you’re buying asset-based coverage, earlier purchases mean more years of growth on the underlying policy.

Ages 60–65: Still viable but premiums are higher and health conditions become more common. Don’t wait past this range if you can help it.

Ages 65–75: Traditional LTC becomes very expensive and underwriting is stricter. Asset-based policies are still available. Short-term care insurance (Chapter 34) becomes an important fallback option.

Age 75+: Most traditional LTC policies are no longer available. Short-term care may still be an option. Asset-based policies may be available with simplified underwriting.

REAL-WORLD EXAMPLE: The \$387,000 Lesson

Ed and Martha, both 74, never bought long-term care insurance. “We’ll cross that bridge when we come to it,” Ed always said. At 78, Ed was diagnosed with Alzheimer’s. Martha cared for him

at home for two years, but as the disease progressed, she couldn’t manage alone. Ed entered a memory care facility at \$12,500/month. He lived there for three years before passing away.

Total cost of Ed’s care: \$450,000 (2 years of home modification and in-home help: \$63,000 + 3 years of memory care: \$387,000). Their retirement savings of \$620,000 was reduced to \$170,000. Martha, now 84, has roughly \$170,000 to last the rest of her life plus her Social Security of \$1,800/month. She is one health event away from Medicaid.

A long-term care policy purchased at age 60 for approximately \$3,500/year (\$70,000 total over 20 years) would have covered \$300,000–\$500,000 of Ed’s care costs. The insurance would have preserved Martha’s financial independence.

FREQUENTLY ASKED QUESTIONS

Q: Will Medicaid pay for a nursing home?

A: Yes, but only after you’ve “spent down” nearly all your assets. Medicaid is a safety net, not a plan. You lose choice in facilities, care quality, and financial freedom. Planning ahead lets you stay in control.

Q: Can I buy LTC insurance if I already have health problems?

A: It depends on the condition. Diabetes, heart disease, and many other conditions don’t automatically disqualify you, but they may limit your options or increase premiums. Conditions like Alzheimer’s, Parkinson’s, or recent stroke typically result in denial. The earlier you apply, the more options you have.

Q: My family has longevity genes. Isn’t that a good thing?

A: Longevity is a blessing—but it also increases the probability of needing long-term care. The longer you live, the more likely you’ll experience cognitive decline, frailty, or chronic

conditions that require assistance. Long-lived families have the most to gain from LTC coverage.

Q: Is long-term care insurance tax-deductible?

A: For tax-qualified policies, premiums count as medical expenses. In 2026, the deductible amount depends on your age: up to \$5,880 for those aged 61–70, and up to \$7,350 for those aged 71+. These count toward the 7.5% AGI threshold for medical expense deductions on Schedule A.

Q: What about the government paying for long-term care?

A: There is no federal program that covers long-term custodial care for the general population. Medicare covers limited skilled care only. Medicaid covers LTC but requires asset depletion. VA benefits cover some LTC for eligible veterans (see Chapter 38). Private insurance remains the primary planning tool.

CHAPTER 32 CHECKLIST

☐ I understand that 70% of people over 65 will need some form of long-term care ☐ I know the current costs of nursing homes, assisted living, and home care in my state ☐
☐ I know the six Activities of Daily Living that trigger LTC insurance benefits ☐ I've evaluated the three types of LTC insurance: traditional, asset-based, and short-term ☐ I've considered the impact of a long-term care event on my spouse's financial security ☐ I've discussed long-term care planning with my family **READY TO TAKE THE NEXT STEP?**

Long-term care is the conversation nobody wants to have—but the families who have it are the ones who survive a crisis intact. As a gerontologist and licensed long-term care specialist, I've helped hundreds of families build protection plans that preserve their savings and their dignity. Let me help you build yours. This is what I was trained to do.

Darin Weidauer | 702-706-6564 | darinweidauer@ecos.care

CHAPTER 33

Chapter 33: Asset-Based Long-Term Care

Hybrid Policies That Give You Your Money Back

The biggest objection I hear about traditional long-term care insurance is this: “What if I pay premiums for 20 years and never need care? I’ve wasted all that money.”

It’s a fair concern. Traditional LTC insurance is a use-it-or-lose-it product. If you stay healthy—which is the outcome you’re hoping for—you don’t get your premiums back.

Asset-based long-term care insurance solves this problem entirely. And it’s become the most popular form of long-term care coverage in America for good reason.

How Asset-Based LTC Works

An asset-based LTC policy combines two financial products into one: a life insurance policy (or annuity) and long-term care coverage. You fund the policy with a single lump-sum premium or a series of scheduled payments. In return, you get three guarantees:

Guarantee 1: If you need long-term care, the policy pays a monthly benefit to cover care costs—typically two to six times your initial premium.

Guarantee 2: If you never need long-term care, your beneficiaries receive a death benefit when you pass away. Your money isn’t lost—it’s paid out as a life insurance benefit, income-tax-free.

Guarantee 3: If you change your mind, many policies offer a return of premium feature. You can get most or all of your money back (depending on the policy and timing).

In short: you use it for care, you leave it to your heirs, or you get it back. Your money is never “wasted.”

How the Leverage Works

The magic of asset-based LTC is leverage. A single premium creates a pool of benefits significantly larger than the amount you deposit.

Single	Death Benefit			
Premium	(no LTC needed)	Total LTC	Monthly LTC	Benefit
Deposit		Benefit Pool	Benefit	Duration
\$75,000	\$120,000	\$240,000	\$4,000/ month	5 years
\$100,000	\$165,000	\$360,000	\$5,000/ month	6 years
\$150,000	\$240,000	\$540,000	\$7,500/ month	6 years
\$200,000	\$320,000	\$720,000	\$10,000/ month	6 years

Look at that \$100,000 example: you deposit \$100,000 and get access to \$360,000 in long-term care benefits—a 3.6x leverage ratio. If you never use the LTC benefits, your heirs receive \$165,000 tax-free. If you want your money back, you can surrender the policy and recover most or all of your \$100,000. This is why asset-based LTC has exploded in popularity.

Funding Options

Asset-based policies offer flexibility in how you fund them:

Single premium: One lump-sum payment, often from a CD, savings account, or other conservative investment that’s earning low returns. This is the most common approach.

10-pay or limited-pay: Spread the premium over 5, 7, or 10 annual payments. This makes the cost more manageable for people who don’t want to part with a large lump sum all at once.

1035 exchange: If you have an existing life insurance policy or annuity that you no longer need in its current form, you can exchange it tax-free into an asset-based LTC policy under IRC Section 1035. This is one of the most powerful repositioning

strategies in retirement planning— turning an underperforming asset into one that provides both LTC protection and a death benefit.

Tax Advantages

Asset-based LTC policies enjoy favorable tax treatment:

LTC benefit payments are generally received tax-free (up to IRS per diem limits, approximately \$420/day in 2026 for per diem policies). Death benefit proceeds are received income-tax-free by beneficiaries. A 1035 exchange from an existing life insurance policy or annuity avoids triggering a taxable event. LTC premiums embedded in the policy may qualify for the medical expense deduction.

Asset-Based LTC vs. Traditional LTC vs. Self-Insuring

Factor	Traditional LTC	Asset-Based LTC	Self-Insuring
Premium Risk	Can increase significantly	Locked in at purchase	N/A
Use-It-or-Lose-It?	Yes	No (death benefit or return)	N/A
LTC Benefit Pool	Defined daily/monthly	Leveraged pool (2-6x deposit)	Limited to your savings
Death Benefit	None	Yes (income-tax-free)	Whatever's left in your estate
Return of Premium	No	Yes (most policies)	N/A
Premium Flexibility	Annual premiums	Single, 5-pay, or 10-pay	N/A
Ideal Candidate	Healthy, ages 50-65	Healthy, ages 50-75,	Very wealthy (can
with savings to absorb \$500K+ in reposition care costs)			
Risk to Spouse Managed by insurance	Managed by insurance	High—care costs may deplete shared	

savings

Who Should Consider Asset-Based LTC?

Asset-based LTC is an excellent fit for people who have \$75,000–\$300,000 or more sitting in conservative investments (CDs, money market, savings accounts, low-yield bonds) earning minimal returns. That money isn’t working hard for you. An asset-based LTC policy puts it to work in three ways: LTC protection, life insurance, and guaranteed return of premium.

It’s also ideal for people who are philosophically opposed to “wasting” money on traditional LTC premiums, want guaranteed premiums that never increase, have an old life insurance policy or annuity they’d like to reposition, or want to protect their spouse from the financial devastation of a long-term care event.

REAL-WORLD EXAMPLE: The CD That Became a Safety Net

Gloria, age 63, had \$150,000 in a CD earning 2.1% APY. After taxes, her annual return was roughly \$2,500—barely keeping pace with inflation. She repositioned the \$150,000 into an asset-based LTC policy.

Her policy provides: a \$540,000 long-term care benefit pool (\$7,500/month for 6 years). A \$240,000 death benefit if she never needs LTC. And the ability to recover approximately \$145,000 if she surrenders the policy within the first 10 years.

Gloria’s \$150,000 is no longer sitting idle. It’s providing \$540,000 in LTC protection, a \$240,000 legacy for her children, and she can still get her money back if she changes her mind. The CD was earning \$2,500/year. The asset-based policy is providing \$540,000 in protection and \$240,000 in legacy value—with the same \$150,000.

FREQUENTLY ASKED QUESTIONS

Q: What if I need the money for something else after buying the policy?

A: Most asset-based policies have a return of premium feature. You can surrender the policy and get your money back (the exact amount depends on the policy year and surrender

schedule). You lose the LTC and death benefits, but your principal is returned.

Q: Can a married couple share an asset-based LTC policy?

A: Some carriers offer shared benefit riders that allow couples to access each other's LTC benefit pools. If one spouse uses their benefits, the other spouse can draw from the remaining shared pool. This is a powerful way to maximize protection for both partners.

Q: I'm 70. Am I too old for asset-based LTC?

A: Not necessarily. Many carriers issue policies up to ages 80–85, depending on health. The leverage ratio (how much LTC benefit you get per premium dollar) decreases with age, but coverage is still available and can be substantial.

Q: How does a 1035 exchange work?

A: If you own a life insurance policy or annuity you no longer need, you can exchange it directly into an asset-based LTC policy without triggering taxes. The cash value of your existing policy

becomes the premium for the new policy. This must be a direct transfer between insurers—you never touch the money.

CHAPTER 33 CHECKLIST

☐ I understand how asset-based LTC provides three guarantees: care, death benefit, or return of premium

☐
based policy

☐ I've reviewed the leverage ratios: how much LTC benefit I'd receive per premium dollar ☐ I've explored whether a 1035 exchange from an existing policy makes sense ☐ I've considered shared benefit riders if I'm married ☐ I've talked to a licensed agent about specific quotes from multiple carriers
READY TO TAKE THE NEXT STEP?

Asset-based LTC is the solution I recommend most often because it eliminates the “what if I never use it” concern. Your

money is never wasted—it's either protecting you, leaving a legacy, or coming back to you. I work with multiple carriers to find the best leverage for your dollar. Let me show you what your money could do.

Darin Weidauer | 702-706-6564 | darinweidauer@ecos.care

C H A P T E R 3 4

Chapter 34: Short-Term Care Insurance

The Affordable Bridge Most People Overlook

Here's a statistic that surprises most people: the average long-term care event lasts less than one year. While we plan for worst-case scenarios—and we should—the reality is that many people need care for weeks or months, not years.

That's where short-term care insurance comes in. It's more affordable than traditional LTC, easier to qualify for, and perfectly designed for the most common care scenarios. And yet almost nobody knows it exists.

What Short-Term Care Insurance Covers

Short-term care insurance covers the same types of care as traditional LTC: nursing home care, assisted living, home health aides, homemaker services, and adult day care. The difference is the duration: short-term care policies typically cover care for up to 360 days (some up to 2 years, depending on the policy).

Benefits are triggered the same way: inability to perform two or more ADLs, or cognitive impairment. Most policies have an elimination period (waiting period) of 0 to 30 days before benefits begin.

Why Short-Term Care Insurance Is a Game-Changer

Affordability: Short-term care premiums are a fraction of traditional LTC premiums. A 65-year-old might pay \$50–\$120/month for short-term care versus \$200–\$500+/month for traditional LTC.

Simplified underwriting: Many short-term care policies have simplified health questions instead of full medical underwriting. This means people with health conditions that would disqualify them from traditional LTC may still qualify for short-term care.

Higher issue ages: Some carriers offer short-term care insurance to applicants as old as 89. Traditional LTC is typically unavailable past age 75–80.

Level premiums: Most short-term care premiums are guaranteed level—they won’t increase over time. This is a significant advantage over traditional LTC, where rate increases have been common and sometimes dramatic.

Short-Term Care vs. Traditional LTC

Feature	Short-Term Care	Traditional LTC
Benefit Duration	Up to 360 days (some up to 2 years)	2-5+ years
Monthly Premium (age 65)	\$50-\$150	\$200-\$500+
Premium Increases?	Typically no (level premiums)	Yes (common)
Underwriting	Simplified (3-8 health questions)	Full medical underwriting
Maximum Issue Age	Up to 89	Typically 75-80
Elimination Period	0-30 days	30-180 days

Feature	Short-Term Care	Traditional LTC
Covers Home Care?	Yes	Yes
Covers Nursing Home?	Yes	Yes
Best For	Most common care needs; bridge to family plan or recovery	Extended multi-year care needs

The Bridge Strategy

Here’s how smart planners use short-term care insurance: as a bridge. If you need care after a surgery, a fall, a stroke, or the early stages of a cognitive decline, short-term care covers the first 6-12 months while your family develops a longer-term plan. During that time, your family can assess whether you’ll recover fully, need ongoing home care, or need a transition to a facility. The insurance buys time—and time, in a caregiving crisis, is the most valuable thing in the world.

Short-term care can also serve as a complement to other coverage: pair it with an asset-based LTC policy for extended

protection, or use it as standalone coverage for people who can't afford or can't qualify for traditional LTC.

Who Should Consider Short-Term Care Insurance?

People aged 65–89 who were denied traditional LTC insurance due to health conditions. People on a tight budget who need some coverage rather than none. People who already have partial LTC coverage and want to fill a gap. Families who want “buy-time” coverage while developing a care plan. Veterans (short-term care can complement VA benefits; see Chapter 38). People who believe they're unlikely to need multi-year care but want protection for the most common scenarios.

REAL-WORLD EXAMPLE: The Fall That Changed Everything

Dorothy, age 77, had no long-term care insurance. She'd been denied traditional LTC coverage at age 72 due to diabetes and a previous hip replacement. At a friend's suggestion, she purchased a short-term care policy at age 74 for \$95/month—\$1,140/year.

At 77, Dorothy fell in her kitchen and fractured her pelvis. After 5 days in the hospital, she needed 8 weeks of skilled nursing followed by 4 months of home health aide services. Total care costs: approximately \$47,000.

Dorothy's short-term care policy paid \$4,500/month for 6 months: \$27,000 total. Her out-of-pocket cost: approximately \$20,000 instead of \$47,000. Without the policy, Dorothy would have depleted a significant portion of her savings. The \$3,420 in total premiums she'd paid over three years returned nearly 8x in benefits.

FREQUENTLY ASKED QUESTIONS

Q: Is short-term care insurance the same as short-term disability?

A: No. Short-term disability replaces income when you can't work due to illness or injury. Short-term care insurance pays for actual care services (nursing, home health, assisted living) when you need help with Activities of Daily Living. They're completely different products.

Q: Can I buy short-term care insurance for my parents?

A: Yes, and this is very common. Adult children often purchase short-term care policies for aging parents who are past the age or health threshold for traditional LTC. The simplified underwriting makes it accessible for many older adults.

Q: Does short-term care insurance cover Alzheimer's or dementia?

A: Yes. Most policies cover cognitive impairment as a benefit trigger. However, because Alzheimer's typically requires years of care, the 360-day benefit period may not cover the full duration. For dementia-specific risk, consider pairing short-term care with an asset-based LTC policy.

CHAPTER 34 CHECKLIST

☐ I understand what short-term care insurance covers and how it differs from traditional LTC ☐
☐ I've considered the 'bridge strategy' for buying time during a care event ☐ I know that short-term care has simplified underwriting and higher issue ages ☐ I've explored pairing short-term care with other coverage (asset-based LTC, VA benefits) **READY TO TAKE THE NEXT STEP?**

Short-term care insurance is the most underutilized tool in retirement planning. Most people don't know it exists—and those who do often wish they'd bought it sooner. If traditional LTC isn't in the budget or you've been declined for coverage, let me show you what short-term care can do. It's affordable, accessible, and it might just save your retirement.

Darin Weidauer | 702-706-6564 | darinweidauer@ecos.care

CHAPTER 35

Chapter 35: Caring for Aging Parents

The Sandwich Years, Getting Paid, and Protecting Your Own Retirement

The last three chapters covered insurance that pays professionals to provide care. This chapter is about the care that usually comes first — the care you provide. Because for millions of readers of this book, “long-term care planning” isn’t theoretical. It’s Tuesday. It’s driving Mom to dialysis, managing Dad’s twelve medications, and doing it all while trying to save for your own retirement.

You are not alone — you are the majority. According to the AARP and National Alliance for Caregiving report Caregiving in the US 2025 , 63 million Americans — nearly 1 in 4 adults — are family caregivers , up 45% in a decade. They average 27 hours a week of care. Nearly a quarter provide 40+ hours — a full-time unpaid job. About 29% are “sandwich generation” caregivers , supporting children and parents at the same time. And roughly half report at least one negative financial impact, with average out-of-pocket costs around \$7,200 a year .

Let’s make sure love for your parents doesn’t quietly dismantle your own retirement.

The Career Trap: Why Quitting Is the Most Expensive Option

The instinct is noble: “I’ll just leave work early and take care of Mom myself.” Before you do, add up the true cost — it’s far more than the lost paychecks:

...	Lost salary and benefits for every year out of the workforce Lost 401(k) contributions and matches — and their decades of compounding
	A smaller Social Security check for life. Your benefit is built on your 35 highest earning years (Chapter 15). Zeros dropped

...

Lost salary and benefits for every year out of the workforce
Lost 401(k) contributions and matches — and their decades of compounding

into that formula in your 50s and 60s — your peak earning years — permanently shrink the benefit you'll live on at 85.

For many families, the smarter math is a blend : keep working, use FMLA when needed (up to 12 weeks of job-protected unpaid leave at covered employers), hire paid help for the hours you can't cover, and tap the programs below that pay family caregivers.

Yes, Family Caregivers Can Get Paid — Here's Where to Look

Medicaid consumer-directed care. If your parent qualifies for Medicaid (Chapter 40), most states run “self-directed” or “structured family caregiving” programs that let the care recipient hire and pay their own caregiver — including, in many states, an adult child. Names and rules vary by state; start with your Area Agency on Aging or state Medicaid office.

Veterans programs. If Mom or Dad served, the VA is often the richest source of caregiving dollars: the Aid and Attendance pension supplement pays qualifying wartime veterans who need help with daily activities up to \$2,431 a month for a single veteran in 2026 (\$2,874 with a dependent spouse) — money that can pay any caregiver, including family. The Program of Comprehensive Assistance for Family Caregivers (PCAFC) pays a monthly stipend directly to caregivers of eligible veterans. Full details in Chapter 38.

Long-term care insurance. Some policies — especially the hybrid policies from Chapter 33 — pay cash benefits the family can direct to any caregiver. Check whether your parent's policy pays informal caregivers.

Medicare's GUIDE dementia program. Launched by CMS in July 2024, the GUIDE Model gives families of Medicare patients with dementia a care navigator, 24/7 support line, caregiver

training, and a respite-care benefit of up to \$2,500 per year at participating providers. If dementia is part of your family's story, ask the neurologist whether their practice participates.

A written family agreement. If parents will pay you directly, put it in writing — a personal care agreement with market-rate pay, signed before services begin. Done informally, those payments can later be treated as disqualifying gifts under Medicaid's five-year look-back (Chapter 40). Done correctly, they're legitimate compensation. This one document prevents both Medicaid problems and sibling wars.

The Tax Side: Small Breaks That Add Up

Claiming a parent as a dependent. If you provide more than half of your parent's support and their gross income (not counting most Social Security) is under the IRS limit — roughly \$5,250, indexed annually; confirm the current figure in IRS Publication 501 — you may claim them as a qualifying relative and take the \$500 Credit for Other Dependents. They do not have to live with you. Medical expense deduction. If you can claim your parent (or meet the support test), the medical bills you pay for them — including a share of assisted living attributable to care

...

— count toward your own deduction for expenses above 7.5% of AGI. Dependent care credit. If you pay for adult day care so you can work, the Child and Dependent Care Credit can apply to a dependent parent who can't care for themselves.

Protect the Caregiver, Too

Get the legal house in order while your parent can still sign : durable financial power of attorney, healthcare power of attorney, HIPAA releases, and an advance directive (the same five documents from this book's estate checklist — now for Mom and Dad). Hold the family meeting early: who does what,

who pays what, and how decisions get made. And schedule your own respite before burnout schedules it for you — the AARP report found 1 in 5 caregivers rate their own health as fair or poor. You cannot pour from an empty cup, and your parent's plan collapses if you do.

REAL-WORLD EXAMPLE: Linda's Blend

Linda, 58, nearly quit her \$72,000 job when her mother's Parkinson's advanced. We ran the numbers: retiring seven years early would cost roughly \$500,000 in lost wages, matches, and growth — and cut her own Social Security by hundreds a month for life. Instead: her mother (a Korean War veteran's widow) qualified for VA survivor pension with Aid and Attendance, funding 25 hours a week of home care; Linda shifted to four ten-hour days; a signed care agreement paid her for weekend care without endangering future Medicaid eligibility; and a GUIDE respite benefit covered two recharge weekends a quarter. Her mother stayed home. Linda's retirement stayed intact.

FREQUENTLY ASKED QUESTIONS

Q: Will Medicare pay me to care for my parent?

A: Traditional Medicare does not pay family caregivers for custodial care. The money flows through the routes above — Medicaid self-direction, VA benefits, LTC insurance, GUIDE respite — not through a Medicare paycheck.

Q: My siblings do nothing but criticize. Any advice?

A: Put everything in writing — the care agreement, a shared expense log, and meeting notes. Money and resentment are the two things that split families; documentation defuses both. A geriatric care manager can serve as a neutral referee.

Q: Does helping my parents affect my own IRMAA or taxes?

A: Supporting a parent doesn't raise your MAGI — but how you raise the money can. Pulling extra IRA dollars to fund care counts as income (Chapters 11 and 14). Plan withdrawals with the same care you'd use for any other expense.

CHAPTER 35 CHECKLIST

☐ I've calculated the true lifetime cost before reducing or leaving work ☐

Attendance/PCAFC, LTC policy benefits, GUIDE respite

☐ Any payments from my parent to me are documented in a written personal care agreement ☐ My parent's five essential legal documents are signed — while they can still sign ☐ I've claimed the tax breaks I'm entitled to as a caregiver ☐ I have a respite plan, because my parent's plan depends on me staying healthy > 📞 READY TO TAKE THE NEXT STEP? If you're caring for a parent right now, you don't need a lecture — you need money, hours, and a plan. In a free Retirement Blueprint Session we'll find every benefit your parent qualifies for and make sure your own retirement survives the sandwich years. Darin Weidauer | ECOS | 702-706-6564 | darinweidauer@ecos.care

C H A P T E R 3 6

Chapter 36: Final Expense Planning

Protecting Your Family from a \$15,000+ Surprise

Nobody wants to talk about their own funeral. I get it. But here’s the reality: if you don’t plan for this expense, your family will—often at the worst possible time, with the least emotional bandwidth, and with money they may not have readily available.

The average cost of a funeral in the United States in 2026 is between \$8,000 and \$15,000 for a traditional burial, and \$5,000 to \$8,000 for a cremation with services. Add in cemetery costs, headstones, flowers, obituaries, death certificates, and the post-funeral gathering, and the total easily reaches \$12,000 to \$20,000 or more.

That’s a bill that comes due within days of your passing. Not weeks. Not months. Days.

The Real Costs of Dying

Expense	Typical Cost Range (2026)
Funeral service (viewing, ceremony, hearse)	\$3,500-\$7,500
Casket	\$2,000-\$10,000
Cemetery plot	\$1,000-\$5,000+
Grave opening and closing	\$1,000-\$2,500
Headstone or marker	\$1,000-\$5,000
Cremation (with services)	\$3,000-\$8,000
Embalming	\$500-\$1,500
Death certificates (multiple copies)	\$50-\$200
Obituary notices	\$200-\$1,500
Flowers, programs, reception	\$500-\$3,000
Total (traditional burial)	\$10,000-\$25,000+

What Final Expense Insurance Is

Final expense insurance—also called burial insurance or funeral insurance—is a small whole life insurance policy specifically designed to cover the costs of your funeral, burial, and related end-of-life expenses. Coverage amounts typically

range from \$5,000 to \$25,000, though some policies go up to \$50,000.

Unlike traditional life insurance, final expense policies are designed for simplicity: coverage amounts are modest, premiums are affordable (typically \$30–\$100/month depending on age and amount), underwriting is simplified or guaranteed issue (no medical exam required), premiums are level (they never increase), and coverage lasts for life.

Types of Final Expense Policies

Simplified Issue

Simplified issue policies ask a limited number of health questions (typically 5–15) but don’t require a medical exam. If you answer the questions favorably, you get full coverage immediately. Premiums are lower than guaranteed issue because the insurer has some health screening. This is the better option if you can qualify.

Guaranteed Issue

Guaranteed issue policies accept everyone, regardless of health. No health questions. No exam. No possibility of being declined. The trade-off: guaranteed issue policies typically have a graded death benefit—if you die within the first 2–3 years (from natural causes), the policy pays only a return of premiums paid plus interest, not the full death benefit. After the graded period, the full benefit applies. Premiums are also higher than simplified issue.

Feature	Simplified Issue	Guaranteed Issue
Health Questions?	Yes (5–15 questions)	No
Medical Exam?	No	No
Immediate Full Coverage?	Yes (if approved)	Graded (2–3 year waiting period)
Premiums	Lower	Higher
Best For	People in decent health	People with significant health conditions
Typical Coverage	\$5,000–\$50,000	\$5,000–\$25,000
Age Range	45–85	45–90

Why Not Just Save the Money?

Some people argue they can “just save” for funeral costs instead of buying insurance. Theoretically, yes. But practically, this approach has problems:

Savings can be depleted by other emergencies. A market downturn, medical bills, or long-term care costs can drain the savings you earmarked for burial. Life insurance, by contrast, is protected from creditors in most states and can’t be depleted by other expenses.

The death benefit is guaranteed. No matter when you die, the benefit pays the full amount. With savings, you need to accumulate enough before you die—and nobody knows when that will be.

The benefit is immediately available. Life insurance proceeds are typically paid within 7–14 days. Savings in bank accounts or brokerage accounts can be frozen upon death pending probate. Your family may not be able to access your savings quickly enough to pay for the funeral.

REAL-WORLD EXAMPLE: The Family That Was Prepared

Rosa purchased a \$15,000 final expense policy at age 72 for \$78/month. She paid premiums for 6 years before passing away at age 78. Total premiums paid: \$5,616. Death benefit received by her family: \$15,000. Her family used the money to cover the funeral (\$11,200), outstanding medical bills (\$2,400), and travel expenses for family members (\$1,400). Without the policy,

her four adult children would have each needed to contribute \$3,750—a significant burden for two of them who were living paycheck to paycheck.

FREQUENTLY ASKED QUESTIONS

Q: Can I name anyone as beneficiary?

A: Yes. You can name a spouse, child, other family member, friend, trust, or even a funeral home directly. Make sure your

beneficiary designation is current—the designation on the policy overrides your will.

Q: What if I already have life insurance?

A: If your existing life insurance is sufficient to cover both income replacement and final expenses, a separate final expense policy may not be needed. But many people's group life insurance ends at retirement, and individual policies may have been surrendered. Evaluate your current coverage before deciding.

Q: Can I pre-pay my funeral instead?

A: Yes, through a pre-need funeral plan with a funeral home. This locks in today's prices. However, pre-need plans are tied to a specific funeral home. If you move or the funeral home closes, transferring can be complicated. A final expense policy is more flexible—your family can use the money at any funeral home.

CHAPTER 36 CHECKLIST

☐ I know the estimated cost of a funeral/burial in my area ☐
☐ I've considered a simplified issue or guaranteed issue final expense policy ☐ I've designated a beneficiary and communicated my wishes to my family ☐ I've considered pre-planning my funeral arrangements to reduce family burden ☐
 I've verified that my beneficiary designations are current on all policies **READY TO TAKE THE NEXT STEP?**

Final expense planning is one of the simplest and most affordable forms of insurance—and one of the most meaningful gifts you can give your family. A \$15,000 policy can cost less than your monthly cable bill. Let me help you find the right coverage so your family never has to scramble during the hardest week of their lives.

Darin Weidauer | 702-706-6564 | darinweidauer@ecos.care

CHAPTER 37

Chapter 37: Indemnity Insurance

Cancer, Critical Illness, and Hospital Cash Plans

We covered indemnity insurance briefly in Chapter 10 as part of the Medicare gap discussion. This chapter goes deeper—because for many retirees, indemnity plans are the difference between financial recovery and financial ruin after a major health event.

Let me be direct: Medicare covers your medical bills. Medigap or Medicare Advantage reduces your medical out-of-pocket costs. But neither Medicare nor any supplement covers the non-medical costs that accompany a health crisis—lost income, transportation, home modifications, caregiver expenses, household help, and the hundred small things that add up to thousands of dollars.

Indemnity insurance covers what health insurance can't.

Understanding How Indemnity Insurance Works

Indemnity insurance pays a fixed cash benefit directly to you when a qualifying event occurs. The benefit is not tied to medical bills. It's not paid to your doctor or hospital. It's paid to you, and you decide how to use it.

This is fundamentally different from health insurance. Health insurance pays for healthcare services. Indemnity insurance pays you for having a health event. The two work together but serve completely different purposes.

The Four Types of Indemnity Plans in Detail

Cancer Insurance

Cancer remains one of the most common and most expensive diagnoses in America. According to the American Cancer Society, approximately 40% of Americans will be diagnosed with cancer at some point in their lives. And while survival

rates have improved dramatically, the financial toll of cancer is staggering.

Cancer insurance pays benefits at multiple stages: initial diagnosis (lump sum, typically \$5,000–\$50,000), surgery (per procedure), chemotherapy (per treatment), radiation (per treatment), hospitalization (per day), transportation to treatment centers (mileage or flat rate), experimental treatments (some policies), and second opinion consultations.

Cancer Insurance Benefit	Typical Payment Range
Initial Diagnosis (lump sum)	\$5,000–\$50,000
Surgery	\$1,000–\$5,000 per procedure
Chemotherapy	\$200–\$1,000 per treatment
Radiation	\$200–\$1,000 per treatment
Hospital Confinement	\$100–\$500 per day
Transportation	\$0.50–\$1.00 per mile or flat fee
Skin Cancer Benefit	\$250–\$500 (reduced benefit for non-invasive)
Bone Marrow/Stem Cell Transplant	\$5,000–\$15,000
Second Medical Opinion	\$500–\$1,000

Cancer Insurance Benefit	Typical Payment Range
Annual Wellness Benefit	\$50–\$100 for cancer screening

Critical Illness Insurance

Critical illness insurance pays a single lump sum when you’re diagnosed with a qualifying condition. Unlike cancer insurance (which pays at each stage of treatment), critical illness pays one large check—and you decide how to use it.

Commonly covered conditions: heart attack, stroke, cancer (invasive), kidney failure, major organ transplant, coronary artery bypass, paralysis, blindness, deafness, coma, and some policies cover Alzheimer’s, Parkinson’s, and ALS.

Typical lump-sum benefits range from \$10,000 to \$100,000. Some policies allow you to receive a partial benefit for less severe events (e.g., 25% of the benefit for an angioplasty) and the full benefit for more severe events (e.g., 100% for a heart attack).

Hospital Indemnity Insurance

Hospital indemnity insurance pays a cash benefit when you’re admitted to the hospital, regardless of the reason. Benefits are structured as either a daily rate (\$100–\$500 per day of hospitalization), a per-admission lump sum (\$1,000–\$3,000 per hospital stay), or both.

Some policies also pay benefits for ICU confinement (often double the daily rate), emergency room visits, and outpatient surgery. The benefits are paid in addition to whatever Medicare or your supplemental insurance pays for the medical treatment itself.

Accident Insurance

Accident insurance pays benefits for injuries resulting from accidents. Given that falls are the leading cause of injury for adults over 65, and that one in four seniors falls each year, accident insurance is particularly relevant for retirees.

Benefits typically cover: fractures (scheduled amounts based on which bone), dislocations, emergency room visits, ambulance transportation, follow-up doctor visits, physical therapy, and prosthetic devices. Some plans also provide lump-sum benefits for accidental death or dismemberment.

How to Choose the Right Indemnity Plan

Not everyone needs every type of indemnity plan. The right mix depends on your health history, family history, lifestyle, and financial situation. Here’s a quick guide:

Your Situation	Recommended Plan(s)	Why
Family history of cancer	Cancer insurance	High probability; costs can exceed \$100K even with good Medicare coverage
History of heart disease or Critical illness stroke		Lump sum provides immediate financial cushion
common; cash covers non-medical costs		
Your Situation	Best Fit	Why
Chronic conditions (diabetes, COPD)	Hospital indemnity	Higher likelihood of

Your Situation	Best Fit	Why
General protection on a budget	Hospital indemnity + accident	hospitalization; cash covers daily costs during stays Covers the two most common events at a combined \$50-\$90/month
Maximum protection	Cancer + critical illness + hospital indemnity	Comprehensive coverage for the most financially devastating scenarios

devastating scenarios

Tax Treatment of Indemnity Benefits

Benefits received from personally-owned indemnity insurance policies are generally received income-tax-free under IRC Section 104(a)(3). The payments are considered reimbursement for personal injury or sickness, not income. This means indemnity benefits don’t increase your MAGI, don’t trigger IRMAA, and don’t affect your Social Security taxation. They’re invisible to the tax code.

REAL-WORLD EXAMPLE: The Triple Benefit

Harold, age 71, carried three indemnity policies: cancer (\$45/month), hospital indemnity (\$35/month), and accident (\$25/month). Total monthly cost: \$105. At age 74, Harold was diagnosed with colon cancer. Over the next 18 months, his cancer policy paid: \$25,000 initial diagnosis lump sum, \$4,800 in chemotherapy benefits, \$2,400 in radiation benefits, \$3,000 in surgery benefits, and \$1,200 in transportation benefits. Total cancer insurance payout: \$36,400.

During treatment, Harold was hospitalized three times (total: 21 days). His hospital indemnity policy paid \$250/day: \$5,250. He also fell during recovery (treatment-related weakness) and broke his wrist. His accident policy paid \$2,500 for the fracture and \$500 for the ER visit: \$3,000.

Total indemnity benefits received across all three policies: \$44,650. Total premiums paid over 3 years before the event: \$3,780. Harold used the money for: mortgage payments during treatment (\$9,600), his wife’s travel and hotel costs near the

treatment center (\$4,200), a home hospital bed and ramp (\$3,500), groceries and meal delivery (\$2,400), and the balance went to general household expenses and savings.

Harold's Medicare and Medigap Plan G covered virtually all of his medical costs (his total medical out-of-pocket: \$283 for the Part B deductible). The indemnity plans covered everything else—the costs that Medicare was never designed to address.

FREQUENTLY ASKED QUESTIONS

Q: Can I have indemnity insurance and Medicare at the same time?

A: Absolutely. Indemnity plans supplement Medicare—they don't replace it or conflict with it. They pay you directly for having a health event, regardless of what Medicare pays your providers.

Q: Are there waiting periods for pre-existing conditions?

A: Some policies have a 12-month waiting period for pre-existing conditions (meaning if you're diagnosed with cancer within 12 months of purchasing a cancer policy, the benefit may not apply if you had a prior cancer history). Policies vary—read the terms carefully or ask your agent.

Q: Can I buy indemnity plans at any age?

A: Most indemnity plans are available well into your 70s and 80s. Some guaranteed issue products have no age limit. However, premiums increase with age, and benefits may decrease. Buying earlier locks in lower rates.

Q: How do indemnity plans work with Medicare Advantage?

A: Exactly the same way as with Original Medicare. The indemnity plan pays you based on the health event, regardless of what your Medicare Advantage plan pays the provider. The two are completely separate.

Q: Do I need indemnity insurance if I have a Medigap plan?

A: Medigap covers your medical cost-sharing (deductibles, coinsurance). Indemnity covers the non-medical costs of a

health event (lost income, transportation, household expenses). They protect against different risks. Many of my clients carry both.

CHAPTER 37 CHECKLIST

- ☐ I understand the four types of indemnity insurance: cancer, critical illness, hospital, and accident
- ☐ I've reviewed my family health history and identified my highest-risk conditions ☐ I know that indemnity benefits are generally tax-free and don't affect IRMAA ☐ I've evaluated which indemnity plans match my risk profile and budget ☐
- ☐ I've considered how my household would handle non-medical costs during a health crisis ☐ I've spoken with a licensed agent about specific plan options and pricing **READY TO TAKE THE NEXT STEP?**

Indemnity insurance is the safety net under the safety net. It covers what Medicare can't—and for most retirees, it costs less than a daily cup of coffee. I can review your health history,

identify your risk factors, and recommend the right combination of plans to protect your household. No pressure. Just protection.

Darin Weidauer | 702-706-6564 | darinweidauer@ecos.care

PART SIX

PART VI — SPECIAL SITUATIONS

Veterans, federal employees, workers past 65, Medicaid eligibles,

and anyone who's been targeted by a Medicare scam. This section

is for you.

CHAPTER 38

Chapter 38: Veterans, TRICARE, and VA Benefits

How They Work With Medicare

This chapter is personal. I spent 22 years in the United States Air Force, starting as a Heavy Construction Equipment Operator and finishing as a Cyber Warfare Officer and University Professor. I know the military healthcare system from the inside. I've used it. I've navigated it. And now I help fellow veterans make the most of it alongside Medicare.

If you're a veteran, you're in a unique position. You may have access to VA healthcare, TRICARE, military retirement benefits, a Thrift Savings Plan, and Social Security—all at the same time. The question is how to coordinate these benefits with Medicare so you get the best coverage, the lowest costs, and the maximum financial protection.

Let's break it down.

VA Healthcare and Medicare: Two Separate Systems

The VA healthcare system and Medicare are completely independent programs. They don't talk to each other. They don't coordinate benefits automatically. And critically, VA healthcare does NOT count as creditable coverage for Medicare Part B purposes.

Let me repeat that because it's the most common mistake veterans make: VA coverage is not creditable coverage for Medicare. If you rely solely on VA healthcare and don't enroll in Medicare Part B at age 65, you will face a permanent Part B late enrollment penalty when you eventually do enroll.



**** CRITICAL FOR ALL VETERANS ****

Enroll in Medicare Part A at age 65. It's free (assuming 40+ quarters of work), and there's no reason not to. Enroll in Medicare Part B at age 65 unless you have creditable employer coverage from a current employer with 20+ employees. VA coverage alone does NOT protect you from the Part B penalty.

If you delay Part B without creditable employer coverage, you will pay a 10% penalty for every year you were late—for the rest of your life.

TRICARE and Medicare: The Three Paths

TRICARE is the healthcare program for active-duty service members, retirees, and their dependents. There are three main TRICARE programs that interact with Medicare:

TRICARE for Life (TFL)

TRICARE for Life is the gold standard for military retirees with 20+ years of service (or medical retirement). TFL acts as a wraparound supplement to Medicare—essentially functioning like a Medigap plan, but at no additional premium cost.

How it works: Medicare pays first, and TRICARE for Life picks up most or all of the remaining costs. For Medicare-covered services, your out-of-pocket cost is typically zero. TFL also covers many services Medicare doesn't, including prescription drugs through the TRICARE pharmacy benefit, overseas coverage, and certain dental and vision benefits.



**** TRICARE FOR LIFE REQUIRES PART B****

To remain eligible for TRICARE for Life, you **MUST** have both Medicare Part A and Part B. If you drop Part B or fail to enroll, TRICARE for Life coverage is suspended. This is non-negotiable.

The cost of Part B (\$202.90/month in 2026) is the “premium” for keeping TFL active—and it’s one of the best deals in healthcare.

TRICARE Prime

TRICARE Prime is an HMO-style managed care option available to military retirees under 65. At age 65 (when Medicare eligibility begins), most retirees transition from TRICARE Prime to TRICARE for Life. If you’re approaching 65 on TRICARE Prime, the transition to TFL should be seamless as long as you’re enrolled in both Medicare Part A and Part B.

TRICARE Select

TRICARE Select is a PPO-style option with annual enrollment fees, deductibles, and cost-shares. Like TRICARE Prime, at age 65 you transition to TRICARE for Life once enrolled in Medicare Parts A and B. TRICARE Select can act as secondary coverage for Medicare-covered services.

TRICARE for Life vs. Medigap vs. Medicare Advantage

Feature	TRICARE for Life	Medigap Plan G	Medicare Advantage
Monthly Premium (beyond Part B)	\$0	\$150-\$300	\$0-\$200+
Part A Deductible Covered	Yes	Yes	Varies
Part B Coinsurance Covered	Yes	Yes (after \$283 deductible)	Copays/coinsurance apply
Part B Excess Charges	Yes	Yes	N/A (network-based)
Prescription Drugs	TRICARE pharmacy	No (need separate	Usually included
Dental Coverage	(low cost) Limited through	Part D) No	Often included
Provider Network	TRICARE Dental Any Medicare provider + military facilities	Any Medicare provider	(basic) In-network only
Overseas Coverage	Yes (worldwide)	Limited (80% for emergencies)	Limited to service area
Prior Authorization	Rarely	No	Common
Out-of-Pocket	Catastrophic cap applies	None needed	\$9,250 in-network
Maximum		(comprehensive)	(2026)
Best For	Military retirees	Non-military	Cost-conscious enrollees wanting
	(20+ years)	Medicare enrollees	

extras

If you’re a military retiree eligible for TRICARE for Life, you have what is arguably the best healthcare coverage available to any American. Your only cost is the Part B premium. There is almost never a reason for a TFL-eligible retiree to purchase a

Medigap plan or enroll in a Medicare Advantage plan—TFL is superior to both in almost every way.

VA Healthcare for Non-Retirees

Not all veterans are TRICARE-eligible. TRICARE is for military retirees (20+ years or medical retirement) and their dependents. Veterans who served but didn’t retire from the military may be eligible for VA healthcare based on service-connected disability, income, or other factors.

VA healthcare is excellent, but it has limitations: you must use VA facilities (unless authorized for community care), wait times can be long for non-urgent care, and not all services are available at every VA facility. Many veterans benefit from having both VA healthcare AND Medicare—using the VA for specialty services, mental health, and service-connected conditions, and Medicare for everything else.

VA Disability Compensation and Income Planning

VA disability compensation is tax-free. It is not included in your Adjusted Gross Income, your MAGI, or your provisional income for Social Security taxation. This makes VA disability one of the most tax-efficient income sources in retirement.

However, VA disability compensation can affect other benefits. For military retirees, there are two programs that allow you to receive both military retirement pay and VA disability simultaneously: Concurrent Retirement and Disability Pay (CRDP) for those with 50%+ VA disability, and Combat-Related Special Compensation (CRSC) for combat-related disabilities. Without these programs, VA disability would offset your military retirement dollar-for-dollar.

VA Benefit	Taxable?	Counts in MAGI?	IRMAA Impact	SS Taxation Impact
VA Disability Compensation	No	No	None	None
VA Pension (non-disability)	Depends	Depends	Possible	Possible
Military Retirement Pay	Yes	Yes	Yes	Yes
CRDP	Retirement	Partial	Partial	Partial

VA Benefit	Taxable?	Counts in MAGI?	IRMAA Impact	SS Taxation Impact
	portion:			
(concurrent pay)	Yes; VA portion: No			
CRSC (combat-related)	No	No	None	None

VA Long-Term Care Benefits

The VA offers several long-term care programs for eligible veterans, including: Community Living Centers (VA nursing homes) for veterans with 70%+ service-connected disability or who meet clinical need. Aid and Attendance benefit (a pension supplement for veterans who need help with ADLs)—up to \$2,431/month for a single veteran in 2026 or \$2,874/month for a veteran with a dependent spouse. Housebound benefit for veterans confined to their home. Home-Based Primary Care and Homemaker/Home Health Aide services.

These VA benefits can complement private long-term care insurance and Medicare coverage. However, VA LTC programs have eligibility requirements, wait lists, and geographic limitations. They should be part of your long-term care plan, not your entire plan.

REAL-WORLD EXAMPLE: The Retired Colonel Who Almost Lost TFL

Colonel (Ret.) James, age 65, had relied on TRICARE Prime his entire post-military career. When he turned 65, he assumed TRICARE would “just keep going.” He didn’t enroll in Medicare Part B. Six months later, he discovered that TRICARE for Life had suspended his coverage because he lacked Part B. He enrolled during the General Enrollment Period, but coverage didn’t start until July 1—leaving him with a 6-month gap and a permanent 5% Part B penalty. His annual extra cost: \$111/year for life. A single phone call at age 64 would have prevented this.

FREQUENTLY ASKED QUESTIONS

Q: I have a 100% VA disability rating. Do I still need Medicare?

A: Even with 100% VA disability, Medicare Part A is free and provides coverage outside the VA system. Part B is strongly recommended to maintain TRICARE for Life eligibility (if applicable) and to give you the flexibility to see any Medicare provider. Many veterans with high disability ratings still benefit from having Medicare as a backup system.

Q: Does my military service count toward Social Security?

A: Yes. Active-duty military service is covered by Social Security. You paid FICA taxes during service, and those years count toward your 35-year earnings calculation. Special credits may apply for earlier service periods. Check your Social Security Statement to verify all military years are correctly credited.

Q: Can I use the VA and Medicare at the same time?

A: You can be enrolled in both, but for any single service, only one program pays. You can't bill both Medicare and the VA for the same treatment. However, you can use the VA for some services (like service-connected care or prescriptions) and Medicare for others (like seeing a civilian specialist). Many veterans use both systems strategically.

Q: My spouse was never in the military. Are they covered by TRICARE?

A: If you're a military retiree, your spouse is eligible for TRICARE (and TRICARE for Life after 65 with Medicare Parts A and B). If you're a veteran but not a military retiree, your spouse is generally not eligible for TRICARE but may qualify for CHAMPVA if you have a permanent and total service-connected disability.

Q: What is CHAMPVA?

A: CHAMPVA (Civilian Health and Medical Program of the Department of Veterans Affairs) provides healthcare coverage for the spouse and dependents of a veteran with a permanent and total service-connected disability, or for the surviving spouse and dependents of a veteran who died from a service-connected condition. At 65, CHAMPVA becomes secondary to Medicare.

CHAPTER 38 CHECKLIST

- ☐ I understand that VA coverage is NOT creditable coverage for Medicare Part B ☐ I've enrolled in (or plan to enroll in) Medicare Part A at age 65 ☐ If TRICARE-eligible, I've confirmed enrollment in Part B to maintain TRICARE for Life ☐ I know my VA disability rating and which VA benefits I'm eligible for ☐ I've verified my military years are correctly reflected on my Social Security Statement ☐ ☐ I've evaluated how VA disability compensation fits into my IRMAA-free income strategy ☐ I've updated all beneficiary designations on my TSP, SGLI/VGLI, and other policies READY TO TAKE THE NEXT STEP?

Fellow veteran: I served 22 years so I could help people like you navigate the transition from military to civilian retirement. TRICARE, VA benefits, TSP, Social Security, Medicare—I've lived all of it. Let me put my experience to work for you. Call a brother-in-arms. I've got your six.

Darin Weidauer | 702-706-6564 | darinweidauer@ecos.care

CHAPTER 39

Chapter 39: Federal Employees and FEHB

Your Medicare Strategy Is Different

If you're a federal civilian employee or retiree, your healthcare picture is different from everyone else's. You have the Federal Employees Health Benefits (FEHB) program—one of the largest and best employer-sponsored health plans in the country. And the way FEHB interacts with Medicare creates both opportunities and potential traps that most people don't understand.

FEHB Basics for Retirees

If you're a federal employee who retires with at least 5 years of FEHB enrollment (and you were enrolled at the time of retirement), you can continue FEHB coverage into retirement. The government continues to pay its share of the premium—typically 72–75% of the total—making FEHB one of the most affordable retiree health plans available.

Here's what makes FEHB unique: FEHB is creditable coverage for Medicare purposes. You can delay Medicare Part B without penalty as long as you have FEHB. However, there are significant advantages to enrolling in Part B even if you have FEHB.

Should You Enroll in Part B If You Have FEHB?

This is the most important question for federal retirees, and the answer for most people is yes. Here's why:

FEHB + Medicare = Virtually Zero Out-of-Pocket Costs

When you have both FEHB and Medicare, they coordinate benefits. Medicare becomes your primary insurance and FEHB becomes secondary. Together, they cover virtually all of your healthcare costs—most federal retirees with both FEHB and Medicare pay little to nothing out of pocket for covered services.

Additionally, many FEHB plans offer premium reductions or waive deductibles and copays when you’re enrolled in Medicare Parts A and B. Some plans even offer special “Medicare wraparound” options that are significantly cheaper than the standard FEHB premium.

The Math: FEHB Alone vs. FEHB + Medicare

Scenario	Monthly Cost	Out-of-Pocket Risk	Coverage Gaps
FEHB Only (Self Only)	\$200–\$500 (your share)	Deductibles + copays can reach \$5,000–\$8,000/year	Limited to FEHB network and benefits
FEHB + Medicare Part A Only	\$200–\$500 (FEHB) + \$0 (Part A)	Reduced hospital costs; Part A covers inpatient first	Part B services still subject to FEHB cost-sharing
FEHB + Medicare Parts A and B	\$200–\$500 (FEHB) + \$202.90 (Part B)	Near-zero for most services	Virtually none for Medicare-covered services
Reduced FEHB + Medicare A and B	\$50–\$200 (reduced FEHB) + \$202.90 (Part B)	Near-zero	Virtually none; best value

For most federal retirees, the sweet spot is enrolling in both Medicare Parts A and B, then switching to a lower-cost FEHB plan that’s designed to work with Medicare. The combined cost may be similar to or less than a full FEHB plan alone, with dramatically better coverage.

The Suspension Strategy

Federal retirees have a unique advantage: you can suspend your FEHB enrollment, enroll in Medicare (with or without a Medigap plan), and later re-enroll in FEHB during Open Season if your needs change. This gives you flexibility that private-sector retirees don’t have.

However, if you suspend FEHB for more than one Open Season period, you may need to wait for a qualifying life event or the next Open Season to re-enroll. Don’t suspend FEHB without understanding the re-enrollment rules.

FEHB and Part D: Do You Need It?

Most FEHB plans provide prescription drug coverage that's at least as good as Medicare Part D. As a result, FEHB is considered creditable drug coverage for Part D purposes. You do NOT need a separate Part D plan if you have FEHB, and you will NOT face a Part D late enrollment penalty as long as you maintain FEHB.

In fact, enrolling in a standalone Part D plan while keeping FEHB can create coordination-of-benefits confusion. In most cases, stick with your FEHB drug coverage unless a specific Part D plan offers significantly better coverage for your particular medications.

FERS Pension, TSP, and Income Planning

Federal employees under the Federal Employees Retirement System (FERS) have a three-legged retirement stool: FERS pension (a defined benefit based on years of service and high-3 average salary), TSP (the federal 401(k)—see Chapter 23), and Social Security.

For IRMAA planning, your FERS pension is fully taxable and counts in your MAGI. TSP withdrawals from the traditional side are fully taxable and count in your MAGI. TSP withdrawals from the Roth side are tax-free and do NOT count in your MAGI. Social Security is partially taxable.

The Roth TSP and Roth IRA are your most powerful IRMAA-avoidance tools as a federal retiree. Every dollar in a Roth is income that Medicare can't see.

REAL-WORLD EXAMPLE: The GS-14 Who Saved \$2,400/Year

Patricia, a retired GS-14 with 30 years of service, had been paying \$420/month for FEHB Blue Cross Blue Shield Standard (Self Only) in retirement. At 65, she enrolled in Medicare Parts A and B. She then switched to the FEHB Blue Cross Basic plan—which offers a Medicare wraparound option for only \$95/month. Her new total cost: \$95 (reduced FEHB) + \$202.90 (Part B) = about \$298/month, down from \$420. That's roughly \$1,465/year in savings. Her coverage? Better than before—

because Medicare and the wraparound FEHB plan together cover virtually everything, with near-zero copays and deductibles.

FREQUENTLY ASKED QUESTIONS

Q: Do I need Medigap if I have FEHB?

A: Generally no. FEHB with Medicare provides coverage comparable to or better than most Medigap plans. Adding Medigap on top of FEHB would be redundant and wasteful. However, if you plan to drop FEHB entirely, you'd want Medigap during your Medigap Open Enrollment Period.

Q: Can I keep FEHB in retirement if I only worked 5 years?

A: You must have been enrolled in FEHB for at least 5 consecutive years immediately before retirement (with some exceptions). If you meet this requirement and retire under FERS or CSRS, you can continue FEHB for life.

Q: I'm a CSRS retiree. Do I qualify for Medicare?

A: CSRS employees hired before 1984 did not pay into Medicare through payroll taxes. If you don't have 40 quarters of Medicare-covered employment (from a second job, spouse's record, or other covered work), you may not qualify for premium-free Part A. You can still purchase Part A (\$565/month in 2026) or qualify through a spouse's work record.

Q: Does my FERS pension count toward IRMAA?

A: Yes. Your FERS annuity is fully taxable ordinary income and is included in your MAGI for IRMAA purposes. If your FERS pension plus other income approaches IRMAA thresholds, consider Roth TSP withdrawals and other strategies to manage your MAGI.

CHAPTER 39 CHECKLIST

☐ I understand how FEHB coordinates with Medicare as secondary coverage ☐ I've evaluated whether enrolling in Part B reduces my total costs through FEHB premium reduction

- ☐ I know that FEHB is creditable coverage for both Part B and Part D
- ☐ I've compared my current FEHB plan to Medicare wraparound options
- ☐ I've considered the IRMAA impact of my FERS pension + TSP withdrawals
- ☐ I've explored Roth TSP contributions or conversions to manage MAGI READY TO TAKE THE NEXT STEP?

Federal retirement is a three-legged stool—FERS, TSP, and Social Security—with FEHB and Medicare layered on top. Getting the coordination right can save you thousands per year. I've worked with hundreds of federal employees and retirees to optimize this exact puzzle. Let me help you find the right combination.

Darin Weidauer | 702-706-6564 | darinweidauer@ecos.care

C H A P T E R 4 0

Chapter 40: Medicaid and Dual Eligibility

When You Qualify for Both

Medicaid is the other government healthcare program—the one designed for low-income Americans. While Medicare is based on age or disability, Medicaid is based on income and assets. And for some people, the two programs overlap.

If you qualify for both Medicare and Medicaid, you’re what’s called a “dual eligible.” And dual eligibles have access to a powerful combination of benefits that can dramatically reduce healthcare costs—if they know how to navigate the system.

How Medicaid Works

Medicaid is jointly funded by the federal government and individual states. Each state administers its own Medicaid program with its own eligibility rules, income limits, and covered benefits. This means Medicaid varies significantly from state to state.

In general, Medicaid eligibility for seniors (age 65+) is based on income below a certain threshold (typically 100–138% of the Federal Poverty Level, depending on the state and program) and assets below a certain limit (typically \$2,000 for an individual, with some exclusions for your home, one vehicle, and personal belongings).

What Dual Eligibles Get

If you qualify for both Medicare and Medicaid, you get the combined benefits of both programs:

Benefit	Medicare Covers	Medicaid Adds
Hospital care	Part A (with deductibles/coinsurance)	Pays Medicare cost-sharing
Doctor visits	Part B (with 20% coinsurance)	(deductibles, copays) Pays the 20% coinsurance
Prescription drugs	Part D	Extra Help/LIS

Benefit	Medicare Covers	Medicaid Adds
	(with copays)	reduces Part
Long-term care	Limited (100 days skilled nursing)	D costs to near \$0 Full custodial/nursing home coverage
Dental, vision, hearing	Not covered by Original	Often covered by Medicaid
Part B premium	Medicare \$202.90/month	(varies by state) Medicaid may pay Part B

reduces premium

The most significant benefit for dual eligibles: Medicaid covers long-term custodial care, including nursing home stays that Medicare doesn’t cover. For someone who needs extended nursing home care, Medicaid eligibility can mean the difference between financial devastation and comprehensive coverage.

Medicare Savings Programs (MSPs)

Even if you don’t qualify for full Medicaid, you may qualify for a Medicare Savings Program that helps pay your Medicare costs. There are four levels:

Program	Income Limit (2026, approximate)	What It Covers
Qualified Medicare Beneficiary (QMB)	~100% FPL (\$1,255/mo single)	Part A premium, Part B premium, deductibles, copays, coinsurance
Specified Low-Income Beneficiary (SLMB)	~120% FPL (\$1,506/mo single)	Part B premium only
Qualifying Individual (QI)	~135% FPL (\$1,694/mo single)	Part B premium only (limited funding)
Qualified Disabled Working Individual (QDWI)	~200% FPL (\$2,510/mo single)	Part A premium only

The QMB program is the most comprehensive—it pays virtually all of your Medicare cost-sharing, meaning you pay almost nothing out of pocket for Medicare-covered services. And here’s a critical protection: under federal law, providers cannot bill QMB beneficiaries for Medicare deductibles, coinsurance, or copays. If a provider tries to bill you for these costs and you’re QMB-enrolled, they’re violating the law.

Extra Help (Low-Income Subsidy) for Part D

Extra Help—also called the Low-Income Subsidy (LIS)—is a federal program that helps people with limited income and resources pay for Part D prescription drug coverage. If you qualify, Extra Help can pay your Part D premium (in whole or part), eliminate your Part D deductible, and reduce your drug copays to \$0–\$11.20 per prescription.

Income limits for Extra Help in 2026 are approximately \$22,590 for an individual and \$30,660 for a married couple, with asset limits of \$17,220 (individual) and \$34,360 (couple)—not counting your home, one car, or burial funds.

Many people who qualify for Extra Help don't know it exists or haven't applied. If your income and assets are modest, it's worth checking your eligibility at ssa.gov or your local State Health Insurance Assistance Program (SHIP).

Dual Eligible Special Needs Plans (D-SNPs)

D-SNPs are a type of Medicare Advantage plan specifically designed for people who have both Medicare and Medicaid. These plans coordinate benefits between the two programs and often provide additional services like care coordination, transportation, home-delivered meals, and personal care attendants.

If you're dual eligible, a D-SNP may simplify your healthcare by combining Medicare and Medicaid benefits into a single plan with one ID card and one care coordinator. However, D-

SNPs are HMO or PPO plans with network restrictions, so evaluate whether the network includes your preferred providers.

Medicaid Spend-Down and Asset Protection

For retirees who don't currently qualify for Medicaid but may need it for long-term care in the future, the "spend-down" process is a serious concern. To qualify for Medicaid's long-term care coverage, you must reduce your countable assets to approximately \$2,000 (individual) or meet the community

spouse resource allowance (\$157,920 in 2026 for the non-institutionalized spouse).

Medicaid also has a “lookback period”—typically 60 months (5 years)—during which any asset transfers (gifts, trust transfers, property transfers) are scrutinized. If you gave away assets within the lookback period to reduce your countable assets, Medicaid may impose a penalty period during which it won’t pay for your care.

This is why planning for long-term care should happen years before you need it—not during a crisis. Asset-based LTC insurance, irrevocable trusts, and other strategies can protect your savings while preserving Medicaid eligibility as a last resort. An elder law attorney and a retirement planner should both be part of this conversation.

REAL-WORLD EXAMPLE: The Widow Who Didn’t Know She Qualified

Evelyn, age 74, was a widow living on \$1,350/month in Social Security. She was paying \$202.90/month for Part B and \$42/month for Part D, with copays totaling another \$80/month. Her total healthcare cost: \$307/month—nearly 23% of her income.

When I reviewed her situation, she qualified for the QMB program and Extra Help. After enrollment: Part B premium: paid by Medicaid (\$0 out of pocket). Part D premium: eliminated by Extra Help (\$0). Drug copays: reduced to \$1.55–\$4.50 per prescription. Total healthcare cost: approximately \$15/month.

Evelyn’s savings: approximately \$3,500/year. She had been eligible for two years but didn’t know these programs existed. Total money she’d overpaid: approximately \$7,000.

FREQUENTLY ASKED QUESTIONS

Q: If I qualify for Medicaid, do I still need Medicare?

A: Yes. Medicaid and Medicare are separate programs. Medicare is your primary insurance; Medicaid supplements it.

Having both gives you the most comprehensive coverage available.

Q: Will Medicaid take my house?

A: Your primary home is generally exempt from Medicaid's asset count while you're alive. However, after your death, many states pursue "estate recovery"—seeking repayment from your estate for Medicaid benefits paid during your lifetime. The rules vary by state. An elder law attorney can help protect your home through proper planning.

Q: How do I apply for Medicare Savings Programs?

A: Contact your state Medicaid office, your local Area Agency on Aging, or your State Health Insurance Assistance Program (SHIP). You can also apply through ssa.gov for Extra Help.

Q: I own a home and have some savings. Can I still qualify?

A: Your primary home (up to a certain equity limit—typically \$713,000 in 2026), one vehicle, personal belongings, and irrevocable burial funds are generally excluded from Medicaid's asset count. Other assets (bank accounts, investments, secondary properties) are counted.

CHAPTER 40 CHECKLIST

- ☐ I understand the difference between Medicare (age/disability) and Medicaid (income/assets)
- ☐ I've evaluated whether I qualify for a Medicare Savings Program (QMB, SLMB, QI) ☐
- ☐ If dual eligible, I've explored Dual Eligible Special Needs Plans (D-SNPs) ☐ I understand the Medicaid spend-down process and 60-month lookback period ☐ I've considered consulting an elder law attorney for long-term Medicaid planning **READY TO TAKE THE NEXT STEP?**

Many people leave money on the table by not applying for programs they qualify for. Medicare Savings Programs, Extra Help, and Medicaid can save you thousands of dollars per year. I can help you evaluate your eligibility and connect you with the right resources. If you're struggling to afford your

healthcare costs, please call me. This is exactly why I do this work.

Darin Weidauer | 702-706-6564 | darinweidauer@ecos.care

C H A P T E R 4 1

Chapter 41: Working Past 65

HSAs, Creditable Coverage, and Timing Your Medicare

More Americans are working past 65 than at any time in the last 50 years. Some work because they want to. Some work because they need to. Either way, working past 65 creates a unique set of Medicare coordination challenges—and opportunities—that can save or cost you thousands of dollars depending on the decisions you make.

The Big Question: When Does Medicare Become Mandatory?

Medicare is never technically “mandatory.” But if you don’t enroll when you’re supposed to, penalties follow you for life. The key variable is whether your employer has 20 or more employees.

Your Situation at Age 65	Enroll in Part A?	Enroll in Part B?	Part D?	Key Rule
Working, employer 20+ employees, employer coverage	Yes (it’s free)	Can delay without penalty	Can delay if drug coverage is creditable	8-month SEP when you leave job or lose coverage
Working, employer <20 employees	Yes	Yes — MUST enroll	Yes — MUST enroll	Medicare is primary; employer plan is secondary
Self-employed (no group plan)	Yes	Yes — MUST enroll	Yes — MUST enroll	ACA/ marketplace plans are NOT creditable
Spouse’s employer (20+ employees)	Yes	Can delay if covered as dependent	Can delay if drug coverage is creditable	8-month SEP when spouse leaves job or coverage ends
COBRA	Yes	Yes — MUST enroll	Yes — MUST enroll	COBRA is NOT creditable coverage
Retired, no employer	Yes	Yes — MUST	Yes — MUST	Enroll

Your Situation at Age 65	Enroll in Part A?	Enroll in Part B?	Part D?	Key Rule
coverage		enroll	enroll	during IEP (7-month window around 65th birthday)
enroll	enroll	creditable coverage		
Retired, no Yes employer coverage	Yes — MUST enroll	Yes — MUST enroll		Enroll during IEP (7-month window around

65th birthday)

The HSA Advantage—And the HSA Trap

Health Savings Accounts (HSAs) are one of the most powerful tax tools in America. If you’re still working and have a High-Deductible Health Plan (HDHP) from your employer, you can contribute to an HSA with triple tax benefits: contributions are tax-deductible, growth is tax-free, and withdrawals for qualified medical expenses are tax-free.

The 2026 HSA contribution limits are \$4,400 for individual coverage and \$8,750 for family coverage, with an additional \$1,000 catch-up contribution for those age 55+.

 ** THE HSA-MEDICARE TRAP**

Once you enroll in any part of Medicare—including Part A—you can no longer contribute to an HSA. This creates a trap for workers over 65:

If you file for Social Security at or after 65, you are automatically enrolled in Part A (it’s retroactive up to 6 months). Once Part A starts, HSA contributions must stop. If you continue contributing after Part A enrollment, you’ll face a 6% excise tax on the excess contributions.

Strategy: If you want to keep contributing to your HSA past 65, do NOT file for Social Security and do NOT enroll in Medicare Part A. You can delay Part A and Part B as long as you have creditable employer coverage from an employer with 20+ employees. Once you’re ready to stop HSA contributions, enroll in Medicare.

Important timing detail: stop HSA contributions at least 6 months before you enroll in Part A, because Part A enrollment can be retroactive up to 6 months. If you fail to stop contributions in time, you may owe the excise tax.

2026 HSA Contribution Limits

Coverage Type	Under 55	Age 55+ (Catch-Up)	Total Maximum (55+)
Individual HDHP	\$4,400	\$4,400 + \$1,000	\$5,400
Family HDHP	\$8,750	\$8,750 + \$1,000	\$9,750

HSA as a Retirement Super-Account

Even if you stop contributing at 65, your existing HSA balance continues to grow tax-free. After age 65, you can withdraw HSA funds for any purpose (not just medical expenses) without penalty—you’ll just pay ordinary income tax, like a traditional IRA withdrawal. For medical expenses, withdrawals remain completely tax-free.

This makes the HSA a dual-purpose retirement account: tax-free for medical expenses and tax-deferred for non-medical expenses. If you’ve been maximizing your HSA for years, you may have a substantial balance that can help fund Medicare premiums, Medigap premiums, long-term care premiums, dental and vision costs, and prescription drug copays—all tax-free.

Coordinating Employer Coverage With Medicare

If your employer has 20+ employees and you’re covered by their group plan, your employer plan is primary (pays first) and Medicare is secondary. You can delay Part B without penalty. When you retire or lose that employer coverage, you have an 8-month Special Enrollment Period to enroll in Part B.

During that 8-month window, you should also enroll in Part D (if you don’t have creditable drug coverage elsewhere), select a Medigap plan or Medicare Advantage plan, and coordinate your transition timing so there’s no gap in coverage.

The Employer Coverage Verification Checklist

Before making any Medicare decisions while working past 65, verify these facts with your employer’s HR department:

QUESTIONS TO ASK YOUR EMPLOYER

1.	How many employees does the company have? (The 20-employee threshold determines everything.)
2.	Is the company’s group health plan “creditable coverage” for Medicare Part B purposes?
3.	Is the company’s prescription drug coverage “creditable” for Medicare Part D purposes?
4.	(Your employer is required to send you an annual notice about this.) What happens to my coverage when I retire—does the company offer retiree insurance?
5.	If retiree insurance is offered, does it become secondary to Medicare at 65?
6.	Can I get these answers in writing?

REAL-WORLD EXAMPLE: The HSA Maximizer

Kevin, age 64, earned \$130,000 as a senior engineer at a company with 500 employees. He had a family HDHP and had been contributing the maximum to his HSA for 12 years. His HSA balance: \$142,000. Kevin’s plan: delay Social Security and Medicare until 67, contributing an additional \$9,550/year to his HSA for three more years (\$28,650 total). At 67, he enrolls in Medicare, stops HSA contributions, and begins using his \$170,000+ HSA balance to pay for Medicare premiums (\$202.90/month Part B), Medigap premiums (\$200/month Plan G), and dental/vision costs—all tax-free. At \$6,000/year in medical expenses, his HSA will last 28+ years, providing tax-free healthcare funding through his entire retirement.

FREQUENTLY ASKED QUESTIONS

Q: I’m 67 and still working. Do I need Part B?

A: If your employer has 20+ employees and you have their group plan, you can continue to delay Part B. When you leave or lose the coverage, you'll have an 8-month SEP. If your employer has fewer than 20 employees, you should have enrolled at 65.

Q: Can I keep my ACA marketplace plan instead of Medicare?

A: You can, but marketplace plans are NOT creditable coverage for Medicare. If you delay Part B while on a marketplace plan, you'll face a permanent penalty. Also, once you're eligible for Medicare, you lose eligibility for marketplace premium subsidies. Switch to Medicare at 65.

Q: My husband is 67 and works. I'm 65 and on his plan. Do I need Medicare?

A: If his employer has 20+ employees and you're covered as a dependent on his group plan, you can delay Part B. When his employment or coverage ends, you get an 8-month SEP. If his employer has fewer than 20 employees, you should enroll in Part B at 65.

Q: I have an HSA. Can I use it to pay Medicare premiums?

A: Yes. You can use HSA funds to pay Medicare Part B, Part D, and Medicare Advantage premiums tax-free. You cannot use HSA funds for Medigap premiums tax-free (this is an IRS rule). All other qualified medical expenses can be paid from your HSA tax-free.

CHAPTER 41 CHECKLIST

☐

☐ I've verified my employer coverage is creditable for Part B and Part D ☐ If I have an HSA, I know to stop contributions before enrolling in Part A ☐ I understand the 8-month SEP for Part B when employer coverage ends ☐ I've asked my employer the six verification questions and gotten answers in writing ☐ If on COBRA or a marketplace plan, I know these are NOT creditable coverage ☐ I've planned my HSA as a long-term retirement healthcare fund **READY TO TAKE THE NEXT STEP?**

Working past 65 creates Medicare coordination questions that trip up even the smartest people. The rules around creditable coverage, HSAs, and enrollment timing are strict and unforgiving. Let me review your specific situation and make sure you're making the right decisions at the right time. A quick call now can prevent a permanent penalty later.

Darin Weidauer | 702-706-6564 | darinweidauer@ecos.care

C H A P T E R 4 2

Chapter 42: The First 90 Days

A Financial Checklist for Widows and Widowers

This is the chapter I hope you file away and never need. But half of all married readers will one day live it, usually with no warning, and the financial decisions of those first three months echo for decades. So let me walk beside you through them — slowly, in order, the way I do with clients sitting across from me in the hardest season of their lives.

First, the most important rule: grief and big decisions don’t mix. Almost nothing on the financial side is truly urgent. The house does not need to be sold this month. The advisor calling about “repositioning the portfolio” can wait. Anyone pressuring you to move quickly — on anything — has just identified themselves as someone to avoid. Here is what actually needs doing, and when.

Days 1-14: Stabilize

• • • •

Order 10-15 certified death certificates through the funeral home. Nearly every institution wants an original; running out mid-process stalls everything. Social Security notification. The funeral home usually reports the death. Know two rules that surprise everyone: Social Security is not paid for the month of death (a payment received for that month must be returned), and there is a one-time \$255 lump-sum death payment to an eligible surviving spouse — a figure unchanged since

1954. Call 800-772-1213 to claim it and to schedule your survivor benefits appointment. Locate the documents: will or trust, life insurance policies, deeds, titles, recent tax return, account statements, and the passwords list (this is why the digital-estate item in our estate checklist matters). Secure the

Order 10–15 certified death certificates through the funeral home. Nearly every institution wants an original; running out mid-process stalls everything. Social Security notification. The funeral home usually reports the death. Know two rules that surprise everyone: Social Security is not paid for the month of death (a payment received for that month must be returned), and there is a one-time \$255 lump-sum death payment to an eligible surviving spouse — a figure unchanged since

• • • •

practical: keep paying the mortgage, utilities, and insurance from the joint account — joint accounts remain accessible to the surviving owner. Don't rush to close or retitle anything yet.

Days 15–45: Claim What's Yours

Days 45–90: Reorganize — and Watch for the Widow's

Penalty

A hard warning that belongs here: predators read obituaries. Fake debts, “final expenses owed,” phony IRS calls, sudden “old friends” with investment ideas — the recently widowed are their favorite targets. Everything in the next chapter applies double during this year. When in doubt, hang up and call someone you trust.

REAL-WORLD EXAMPLE: Margaret's Three Saves

When Margaret, 66, lost Tom, a persuasive advisor urged her to immediately claim her own Social Security, roll Tom's IRA into hers, and annuitize the life insurance “for safety.” We slowed everything down. Instead she claimed the survivor benefit at her full retirement age (100% of Tom's larger check) and let her own benefit grow 8% a year until 70 — worth over \$60,000 across her plan. The IRA rollover waited two weeks until we'd confirmed she was past 59½ and wouldn't need

beneficiary access. And the insurance check went into insured savings for a full year before any decisions. Three pauses. Three saves.

FREQUENTLY ASKED QUESTIONS

Q: Do I have to pay my spouse's debts?

A: Generally the estate pays debts; you're personally responsible mainly for joint debts, co-signed loans, and community-property-state obligations. Never let a debt collector pressure you into paying from your own pocket before an attorney reviews it.

Q: What if I remarry?

A: Remarriage after age 60 (50 if disabled) does not affect survivor benefits — a rule from Chapter 17 that surprises almost everyone.

Q: When should I update my Medicare or Medigap coverage?

A: Your own Medicare continues unchanged. But your income situation just changed — if the new single-filer IRMAA brackets or a drop in income apply, file form SSA-44 to request a reduction based on the life-changing event. And use the next Annual Enrollment Period for a full plan review (Chapter 44).

CHAPTER 42 CHECKLIST

☐ 10-15 certified death certificates ordered ☐ SSA notified; month-of-death payment handled; \$255 lump sum and survivor appointment scheduled

☐ Life insurance, pension, and retirement account claims filed — spousal IRA choice made deliberately

☐ Health coverage secured (COBRA / Medicare SEP deadlines calendared) ☐ Step-up in basis documented; accounts retitled; my own beneficiaries and documents updated

☐ Widow's Penalty reviewed: filing status, brackets, and single IRMAA thresholds — SSA-44 filed if income dropped

☐ Guard up: no big irreversible decisions in year one, and no pressure from anyone — period > 📞 READY TO TAKE THE

NEXT STEP? If you've recently lost your spouse: I'm sorry, truly. You don't have to carry the paperwork alone. Call or text me — no fee, no pitch, no rush — and we'll walk the checklist together at whatever pace you need. Darin Weidauer | ECOS | 702-706-6564 | darinweidauer@ecos.care

CHAPTER 43

Chapter 43: Protecting Yourself from Medicare Scams, Fraud, and TV Commercial Lies

I'm going to be blunt: billions of dollars are stolen from seniors every year through Medicare fraud, scams, and misleading marketing. And the perpetrators range from sophisticated criminal organizations to the familiar faces on your television screen promising you "free" Medicare benefits.

As someone who has worked in this industry for years and taught at more than 54 colleges across Southern California, I've seen every scam and every misleading tactic. This chapter will arm you with the knowledge to protect yourself.

The TV Commercial Problem

Every fall during Medicare's Annual Enrollment Period, your TV becomes a battleground of Medicare Advantage commercials. Celebrity spokespeople promise "\$0 premiums," "free dental and vision," "money back in your Social Security check," and "benefits you're not getting."

Let me be clear: these commercials are not lying, exactly. But they are designed to mislead. Here's what they don't tell you:

"\$0 premium" plans still cost you your Part B premium (\$202.90/month) and have copays, deductibles, and coinsurance when you use care. The \$0 premium means \$0 additional premium—not \$0 total cost.

"Money back in your Social Security check" refers to Part B giveback programs where certain Advantage plans reduce your Part B premium by a small amount (\$30–\$100/month). This sounds great until you realize you may be giving up much more in network restrictions, prior authorization delays, and limited provider choices.

"Free dental, vision, and hearing" benefits in Advantage plans are often very basic. A \$1,000 annual dental allowance sounds good until you need a \$4,000 crown. A "free" eye exam doesn't cover the \$800 progressive lenses you actually need.

The celebrities in these commercials are paid spokespeople. They are not insurance agents. They are not Medicare experts. They are actors reading a script designed to generate phone calls to a sales center.

The Most Common Medicare Scams

Scam #1: The “New Medicare Card” Call

You get a phone call (or text) saying Medicare is issuing new cards and needs to verify your Medicare number and personal information. This is a scam. Medicare will never call you to ask for your Medicare number. They already have it. Anyone asking for your Medicare number by phone is trying to steal your identity.

Scam #2: The “Free” Genetic Testing Kit

Someone offers you a “free” genetic testing or cancer screening kit—at a health fair, at your door, or by mail. They ask for your Medicare number to “bill Medicare” for the kit. What actually happens: your Medicare number is used to submit fraudulent claims, often totaling

thousands of dollars. The test results (if any) are useless or never delivered. And you may receive bills for services you never authorized.

Scam #3: The Door-to-Door or Cold-Call Enrollment

Someone comes to your door or calls you unsolicited, offering to enroll you in a “better” Medicare plan. Federal law prohibits door-to-door Medicare sales without a prior appointment, and it prohibits cold-calling for Medicare plan enrollment. If someone shows up at your door or calls you without your permission to discuss Medicare plans, they are breaking the law.

Scam #4: The “Provider” Billing Scam

You receive a bill for a medical service, equipment, or test you never received. Or you receive medical equipment you never ordered (common with back braces, knee braces, and diabetic supplies). These are billed to Medicare in your name, and the scammer pockets the Medicare payment. Always review your

Medicare Summary Notices (MSNs) for services you didn’t receive.

Scam #5: The “Free Health Screening” at a Hotel or Community

Center

You’re invited to a “free lunch” or “free health screening” event at a hotel or community center. The event is actually a high-pressure sales pitch for a specific Medicare Advantage plan, annuity, or other financial product. The “screening” is designed to collect your personal information and Medicare number.

How to Protect Yourself

Rule	Why It Matters
Never give your Medicare number to anyone who contacts you first	Medicare will never call, text, or email asking for your number
Review your Medicare Summary Notice (MSN) every quarter	Catch fraudulent charges early; report anything unfamiliar
Don’t open the door to unsolicited Medicare salespeople	Door-to-door sales without a prior appointment are illegal
Never sign up for a plan at a “free lunch” event on the spot	Take materials home, compare options, and consult a trusted agent
If it sounds too good to be true, it is	No plan gives you everything for free with no trade-offs
Use only Medicare.gov or a licensed, trusted agent for plan comparisons	Official sources and independent agents work for you, not a sales quota
Report suspected fraud to 1-800-MEDICARE (1-800-633-4227)	Or online at Medicare.gov or the OIG hotline at 1-800-HHS-TIPS
Guard your Medicare card like a credit card	Your Medicare number is linked to your identity and your benefits
Be skeptical of celebrity endorsements	Paid actors are not insurance experts; their

recommendations are scripted sales pitches

Rule	Why It Matters
Work with an independent agent, not a captive agent	Independent agents compare plans from many companies; captive agents sell only

their company’s products

How to Spot a Legitimate Medicare Agent vs. a Scammer

Legitimate Agent	Scammer or Unethical Salesperson
Licensed and appointed in your state	Cannot produce a license number when asked
Sets appointments before visiting	Shows up unannounced at your door
Compares multiple plans from multiple carriers	Pushes only one plan or one company
Explains trade-offs honestly	Says a plan is “perfect” or “the best” with no downsides
Never asks for your Medicare number by phone first	Calls you and immediately asks for your Medicare number
Follows Medicare’s marketing rules	Uses high-pressure tactics, urgency, or fear
Gives you time to decide	Pressures you to sign up “right now”
Discloses that they earn a commission	Denies earning any compensation
Provides a Scope of Appointment form before the meeting	Skips the Scope of Appointment
Has verifiable credentials (AHIP certification, state license)	Cannot verify credentials

The Annual Enrollment Period: A Feeding Frenzy

The Annual Enrollment Period (October 15 – December 7) is the busiest—and most dangerous —time for Medicare scams and aggressive marketing. During this window, plan changes take effect for the following year, and the volume of calls, mail, and advertising explodes.

During AEP, protect yourself by working only with a licensed agent you trust or using Medicare.gov’s official Plan Finder. Don’t respond to unsolicited calls, texts, or emails. If you receive a robocall about Medicare, hang up. Take your time reviewing options—you have the full window from October 15 to December 7 to make a decision.

And remember: the right plan for you depends on your doctors, your medications, your budget, and your health needs. No TV commercial can assess those factors. Only a person who sits

down with you and reviews your specific situation can give you a trustworthy recommendation.

REAL-WORLD EXAMPLE: The \$6,500 Genetic Testing Scam

Howard, age 72, attended a health fair at a community center. A friendly representative offered him a “free cancer screening genetic test” and asked for his Medicare number to “cover the cost.” Howard provided his number and swabbed his cheek. Three months later, he received a Medicare Summary Notice showing a \$6,500 charge for a genetic test he didn’t understand or

need. The test results were never delivered. Howard reported the fraud to 1-800-MEDICARE, and after investigation, the charge was reversed. But the scam company had already been paid by Medicare—and taxpayers footed the bill. Howard was lucky; many people never check their MSNs and never know they’ve been victimized.

FREQUENTLY ASKED QUESTIONS

Q: Can I trust the insurance agents at my doctor’s office?

A: Be cautious. Some agents have relationships with medical offices to gain access to patients. Evaluate any agent the same way you would one who came to your door: are they licensed, independent, comparing multiple plans, and giving you time to decide?

Q: I gave my Medicare number to a caller. What should I do?

A: Contact Medicare immediately at 1-800-MEDICARE. Monitor your Medicare Summary Notices for fraudulent claims. Consider placing a fraud alert on your credit with the major credit bureaus. Report the incident to the Federal Trade Commission (FTC) at reportfraud.ftc.gov.

Q: Are those “Medicare enrollment centers” legitimate?

A: Some are; some aren’t. Many “enrollment centers” advertised online or on TV are call centers that sell specific plans for commissions. They may not compare all available options. An independent licensed agent in your community is typically a more trustworthy resource.

Q: How do I report Medicare fraud?

A: Call 1-800-MEDICARE (1-800-633-4227), visit Medicare.gov, or contact the HHS Office of Inspector General at 1-800-HHS-TIPS (1-800-447-8477). You can also contact your State Senior Medicare Patrol (SMP) program.

CHAPTER 43 CHECKLIST

- ☐ I know the most common Medicare scams (fake cards, genetic testing, door-to-door, billing fraud)
- ☐ I will never give my Medicare number to anyone who contacts me unsolicited ☐
- ☐ I understand that TV commercial promises require careful evaluation ☐ I know the difference between a legitimate agent and a scammer ☐
- ☐ During AEP, I will work only with a trusted, licensed, independent agent **READY TO TAKE THE NEXT STEP?**

In a world full of misleading commercials and outright scams, working with a trusted, independent, licensed agent is your best defense. I don't work for any insurance company—I

work for you. I compare plans across dozens of carriers, I explain the trade-offs honestly, and I never pressure you to make a decision. That's the difference between a salesperson and an advisor. Let me be your advisor.

Darin Weidauer | 702-706-6564 | darinweidauer@ecos.care

PART SEVEN

PART VII — FUTURE-PROOFING YOUR RETIREMENT

A plan that's right today may be wrong tomorrow. These final chapters give you the tools to stay ahead—every year, for the rest

of your life.

C H A P T E R 4 4

Chapter 44: Annual Medicare Review

Why Your Perfect Plan Today May Be Wrong Tomorrow

Let me tell you about a woman I'll call Angela. Angela spent three hours with me in 2024 finding the perfect Medicare Advantage plan. It covered her doctors. It covered her medications. It had a gym benefit and dental coverage she loved. She was thrilled.

One year later, that same plan dropped two of her three medications from its formulary, removed her cardiologist from the network, and increased her specialist copay from \$30 to \$50. The "perfect" plan had changed. Angela hadn't reviewed it. And by the time she noticed, the Annual Enrollment Period had closed.

Angela spent the next 12 months paying out of pocket for medications that used to be covered and driving 45 minutes to an out-of-network cardiologist because the closest in-network one wasn't accepting new patients.

Don't be Angela.

Why Plans Change Every Year

Medicare Advantage plans and Part D plans are allowed to change their benefits, networks, formularies, premiums, and cost-sharing structures every year. They submit these changes to CMS (the Centers for Medicare & Medicaid Services) for approval, and new plan details are published each fall before the Annual Enrollment Period.

Common changes include: drugs being removed from the formulary or moved to a higher (more expensive) tier, doctors and hospitals leaving the plan's network, copay and coinsurance amounts increasing, prior authorization requirements being added for services that were previously covered without them, premium increases, maximum out-of-pocket limits changing, and supplemental benefits (dental, vision, hearing, OTC allowances) being reduced or eliminated.

The Annual Enrollment Period: Your Window to Act

The Annual Enrollment Period runs from October 15 through December 7 every year. During this window, you can switch from one Medicare Advantage plan to another, switch from Medicare Advantage back to Original Medicare (and add a Medigap plan, subject to underwriting), change your Part D prescription drug plan, and join a Medicare Advantage plan from Original Medicare.

Changes made during AEP take effect on January 1 of the following year.

The Annual Review Process: Your Step-by-Step Guide

Every September, before the AEP begins, follow this process:

Step 1: Review Your Annual Notice of Change (ANOC)

Your plan is required to send you an Annual Notice of Change by September 30 each year. This document details every change to your plan for the coming year. Read it carefully. Look for changes to your premium, deductible, copays, drug formulary, provider network, and any new restrictions or prior authorization requirements.

Step 2: Check Your Medications

Go to Medicare.gov's Plan Finder and enter all your current medications, dosages, and your preferred pharmacy. Compare your current plan to other available plans. Even if your plan hasn't changed its formulary, a competing plan might now cover your drugs at a lower cost.

Step 3: Verify Your Doctors

Confirm that your primary care physician, specialists, and preferred hospital are still in your plan's network. Network changes are common and can leave you scrambling for new providers if you don't check.

Step 4: Reassess Your Health Needs

Your health may have changed since you last enrolled. If you’ve been diagnosed with a new condition, started new medications, or had a change in your mobility or care needs, your current plan may no longer be the best fit.

Step 5: Compare Plans

Use Medicare.gov’s Plan Finder to compare all available plans in your area. Look at total estimated annual cost (not just the monthly premium), formulary coverage for your specific drugs, network access for your providers, and the maximum out-of-pocket limit.

Step 6: Consider Whether Original Medicare + Medigap Is Better

If your Medicare Advantage plan has been causing frustrations—network restrictions, prior authorization denials, formulary changes—consider whether Original Medicare with a Medigap plan and standalone Part D might be a better fit. Remember: Medigap underwriting may apply if you’re past your initial open enrollment period, so evaluate this option carefully.

Step 7: Call a Licensed Agent

A good independent agent does this work every day. They know which plans are gaining or losing providers, which formularies have changed, and which plans offer the best value in your specific zip code. A 30-minute conversation with an agent who knows the local market can save you hours of research and thousands of dollars.

Month	Action	Why It Matters
September	Receive and review your	Know what’s changing before
October 1-14	ANOC Research alternatives on	AEP opens Be ready to act when AEP opens
October 15	Medicare.gov Plan Finder AEP opens — you can now make changes	October 15 Don’t rush; you have until
October–November	Meet with a licensed agent to compare options	December 7 Get personalized advice for your specific situation

Month	Action	Why It Matters
By December 7	Make your final plan selection	AEP closes at midnight; changes take effect January 1
Month	Action	Why It Matters
January 1–March 31	MA Open Enrollment — one more chance to switch	If your new plan isn't working, you can make one

change

REAL-WORLD EXAMPLE: The \$3,400 Formulary Surprise

Walter, age 74, had been on the same Medicare Advantage plan for four years. In year five, his plan moved his blood thinner (brand-name, \$380/month retail) from Tier 3 (preferred brand, \$45 copay) to Tier 4 (non-preferred brand, 40% coinsurance). His new monthly cost for that single drug: \$152. Annual increase: \$1,284 just for one medication. A competing plan in his zip code covered the same drug on Tier 3 with a \$40 copay. By switching during AEP, Walter saved \$1,344 in the first year alone. Over three years, the savings exceeded \$3,400—all because he reviewed his plan instead of auto-renewing.

FREQUENTLY ASKED QUESTIONS

Q: What happens if I don't make any changes during AEP?

A: Your current plan auto-renews with whatever new terms it's published for the coming year. You'll keep the plan, but you'll also keep any unfavorable changes (higher costs, narrower network, reduced formulary). Auto-renewal is not the same as active confirmation that the plan still fits your needs.

Q: Can I change plans outside of AEP?

A: In most cases, no. Outside of AEP, changes require a qualifying Special Enrollment Period (moving, losing coverage, qualifying for Medicaid, etc.) or the Medicare Advantage Open Enrollment Period (January 1–March 31, one change allowed).

Q: How often should I review my Medigap plan?

A: Medigap benefits are standardized and don't change. But premiums can increase. It's worth comparing premiums from

other carriers periodically—especially if your current carrier has had large rate increases. Switching Medigap plans may involve underwriting in most states.

Q: Is there a fee to work with a Medicare agent?

A: No. Licensed Medicare agents are paid by the insurance companies, not by you. There is no fee to use an agent, and the plans cost the same whether you enroll through an agent or on your own. You get professional guidance at no additional cost.

CHAPTER 44 CHECKLIST

☐

☐ I know how to access Medicare.gov's Plan Finder to compare plans ☐ I've created a list of all my current medications (name, dosage, frequency) ☐ I've confirmed my doctors are still in my plan's network for next year ☐

☐ I've scheduled a meeting with a licensed agent before December 7 **READY TO TAKE THE NEXT STEP?**

I review plans for my clients every single year. Not because I have to—because it's the right thing to do. Plans change. Needs change. And what was perfect last year may cost you thousands this year. Let me review your coverage before the next AEP. It's free, and it takes less time than you think.

Darin Weidauer | 702-706-6564 | darinweidauer@ecos.care

CHAPTER 45

Chapter 45: Helping the Grandkids

529s, Gifting Rules, and Leaving a Legacy Without Hurting Yourself

Your annual review (last chapter) keeps your own plan healthy. This chapter is about the question that follows once it is: “How much can I give the kids and grandkids — and how do I do it right?” It might be the happiest topic in this book. It’s also where I’ve watched generous people accidentally create tax bills, financial aid problems, and — worst of all — Medicaid penalties for themselves. Generosity deserves good engineering. Here it is.

Rule One: Secure Your Own Oxygen Mask First

Every gifting conversation starts with the same test: can you fund your own longevity — including the long-term care scenario from Chapter 32 — without this money? A gift that later forces you onto your children’s payroll wasn’t a gift. And remember Medicaid’s five-year look-back (Chapter 40): money given away within five years of applying for Medicaid long-term care coverage triggers a penalty period. Give from surplus, never from safety net.

The 2026 Gifting Rulebook — Simpler Than You Think

...	Annual exclusion: \$19,000 per recipient in 2026 — \$38,000 for a married couple giving together. To each child, grandchild, or anyone else, every year, with no gift tax return and no tax. Unlimited tuition and medical payments — if paid directly to the school or provider. Write the check to the university or the hospital, never to the grandchild, and it doesn’t even count against the
	\$19,000. The lifetime exemption: \$15 million per person in 2026 (\$30 million per couple) under the One Big Beautiful Bill Act, indexed for inflation starting in 2027. Gifts

...	Annual exclusion: \$19,000 per recipient in 2026 — \$38,000 for a married couple giving together. To each child, grandchild, or anyone else, every year, with no gift tax return and no tax. Unlimited tuition and medical payments — if paid directly to the school or provider. Write the check to the university or the hospital, never to the grandchild, and it doesn't even count against the
	above the annual exclusion simply require filing Form 709 and nibble at this enormous number — for the vast majority of families, federal gift tax will never actually be owed.

Source: IRS, 2026 estate and gift tax figures; Rev. Proc. 2025-32; Public Law 119-21 (OBBA).

The 529 Plan: A Grandparent's Best Friend — Now With Superpowers

A 529 is the flagship vehicle for education gifts: your money grows tax-free and comes out tax-free for qualified education costs, many states add a deduction or credit for contributions, and you stay in control — you own the account, you can change the beneficiary to another grandchild, and the grandkids can't spend it on a motorcycle.

Four features every grandparent should know:

1.	Superfunding. You may front-load five years of exclusions at once : up to \$95,000 per grandchild in 2026 — \$190,000 per couple — moved out of your estate in a single afternoon
2.	(file Form 709 to elect the five-year spread). The FAFSA fix. Under the simplified FAFSA, distributions from a grandparent-owned 529 are no longer counted as the student's income — eliminating the old aid penalty that made advisors

1.	Superfunding. You may front-load five years of exclusions at once : up to \$95,000 per grandchild in 2026 — \$190,000 per couple — moved out of your estate in a single afternoon
3.	steer grandparents away. Grandparent 529s went from “handle with care” to arguably the best way to help. New for 2026 — bigger K-12 power. The One Big Beautiful Bill Act doubles the annual K-12 withdrawal limit from \$10,000 to \$20,000 per student starting in tax year 2026 , and expands qualified expenses beyond tuition to include curriculum

materials, tutoring, testing fees, and educational therapies — plus, at any age, postsecondary credentialing programs (trade licenses, apprenticeships, certifications). Note: not every state conforms; check yours before withdrawing.

Give Smart: Cash vs. Appreciated Assets

...	Appreciated stock given during life carries your cost basis — the grandkids inherit your future capital-gains bill (and a working child in a higher bracket may pay more tax on the gain than you would). Appreciated assets left at death get a step-up in basis — the gains vanish for income tax purposes.
	The classic play: give cash or high-basis assets now; bequeath the low-basis winners later. And for the charitably minded 70½+, remember the QCD from your IRA (Chapter 24) satisfies RMDs without touching MAGI — protecting the IRMAA thresholds from Chapter 11, because ordinary gifting, generous as it is, does not reduce your MAGI.

One more caution: UTMA/UGMA custodial accounts hand full control to the child at the age of majority and weigh more heavily in financial aid formulas. There's a reason the 529 wins for most families.

REAL-WORLD EXAMPLE: Robert and Susan's Legacy Machine

Robert and Susan, 71 and 69, wanted to help four grandchildren without touching their care fund. The plan: \$19,000 each into grandparent-owned 529s for the two youngest (\$76,000/year total, gift-tax-free, state deduction captured); direct-to-college tuition payments for the eldest — unlimited and excluded; and for the grandson choosing an electrician apprenticeship, 529 dollars now cover his credentialing costs under the 2026 rules. Their own long-term care plan stayed fully funded, nothing crossed the Medicaid look-back tripwire, and every dollar moved with zero gift tax.

FREQUENTLY ASKED QUESTIONS

Q: Can I pay off my grandchild's student loans from a 529?

A: Yes — up to a \$10,000 lifetime limit per beneficiary (siblings get their own \$10,000 each).

Q: Do gifts reduce my taxable income or my IRMAA?

A: No — gifts are not deductible and don't lower MAGI. Charitable gifts can (especially QCDs); family gifts don't. Plan your Medicare brackets accordingly.

Q: What if my grandchild gets a scholarship?

A: You can withdraw up to the scholarship amount from the 529 penalty-free (earnings are taxed but the 10% penalty is waived), change the beneficiary, or leave it growing for graduate school — or that Roth rollover.

CHAPTER 45 CHECKLIST

- ☐ My own retirement and long-term care plan is fully funded before any major gifting ☐
- ☐ I'm using the \$19,000/\$38,000 annual exclusions — and direct tuition/medical payments — before touching the lifetime exemption

☐ Education gifts are flowing through grandparent-owned 529s, with the 2026 K-12 and credentialing rules in mind

☐

☐ My gifting plan is written into my estate documents so it continues the way I intend > 📞 READY TO TAKE THE NEXT STEP? Want to help the grandkids without ever endangering your own security? Bring your family tree to a free Retirement Blueprint Session and we'll design a gifting plan that's generous, tax-smart, and completely safe for you. Darin Weidauer | ECOS | 702-706-6564 | darinweidauer@ecos.care

C H A P T E R 4 6

Chapter 46: The Retirement Decision Timeline

A Year-by-Year Roadmap from Age 59½ to 75+

If this book is your retirement encyclopedia, this chapter is your cheat sheet. It’s a condensed, year-by-year timeline of every major retirement decision you’ll face from age 59½ through 75 and beyond.

Print this chapter. Pin it to your refrigerator. Refer to it every year. And when the time comes for each milestone, come back to the relevant chapter for the full details.

The Complete Retirement Decision Timeline

Age	Key Milestones	Action Items	Book Reference
59½	Penalty-free access to retirement accounts; Roth conversion window opens	Begin Roth conversion strategy; map your retirement income sources	Ch. 22, 25
feasibility; review all retirement accounts			
60	Survivor benefits eligibility begins;	Maximize 401(k) contributions	Ch. 17, 22
SECURE 2.0 Super Catch-Up starts (ages 60–63)		(\$34,750 with Super Catch-Up); if widowed, evaluate	
survivor benefit timing			
62	Earliest Social Security filing age; earliest age for (HECM)	Run Social Security (file or delay?); avoid filing (Earnings Test);	Ch. 16, 18, 28 analysis
evaluate reverse mortgage if needed			
63	IRMAA lookback begins to affect Medicare at 65; critical	Complete large Roth conversions before	Ch. 11, 14, 25
Roth conversion year		IRMAA at 65; manage income	
64	One year before Medicare; plan enrollment strategy	Determine whether to enroll in Part B at 65 or delay; verify	Ch. 3, 6, 7, 37

creditable coverage; research Medigap and Part D plans; if still working, confirm employer size (20+ employees?)

Age	Key Milestones	Action Items	Book Reference
65	Medicare eligibility begins;	Enroll in Part A	Ch. 1-10, 37
	Medigap	(free); enroll in Part	
	Open Enrollment Period starts	B (unless creditable employer coverage	

from 20+ employer); start 6-month Medigap OEP; choose Original Medicare + Medigap + Part D OR Medicare Advantage; stop HSA contributions if enrolling in Part A

65½ Medigap OEP midpoint	If you haven't Ch. 7 selected a Medigap
--------------------------	---

plan, do it NOW; this window is use-it-or-lose-it

66 One year before FRA	Finalize Social Ch. 15-20
(for those born 1960+); approaching	Security filing strategy; coordinate spousal filing; review income stack

Social Security decision point

67 Full Retirement Age;	File for Social Ch. 16, 18
Earnings Test disappears;	Security at FRA if that's your strategy;
PIA available	Earnings Test no longer applies;

evaluate income sources and tax brackets

70 Maximum Social	File for Social Ch. 16
Security benefit (124% of PIA);	Security by 70 — no benefit to further delay; lock in maximum benefit;

Delayed Retirement Credits end

adjust withdrawal strategy

70½ Qualified Charitable	If charitably inclined, Ch. 14, 24 use
Distributions (QCDs) become available	QCDs from traditional IRA to

satisfy RMDs tax-free and reduce MAGI

Age Key Milestones	Action Items Book Reference
1951-1959); required from traditional IRA, 401(k), TSP	December 31; avoid the April 1 double- RMD trap; use QCDs
if possible; review IRMAA impact of RMDs	
75 RMDs begin (born 1960+); long-term care risk accelerates	Take first RMD; Ch. 24, 29-31 intensify
already covered; review estate documents; annual Medicare review becomes critical	
75+ Ongoing annual reviews; increasing likely	Review Medicare Ch. 39, 41 plan annually; LTC coverage; update beneficiary
designations; consider simplified financial structure	

The Three Most Dangerous Birthdays

Not all birthdays are created equal. These three ages carry the highest risk of costly mistakes:

Age 65: Medicare Enrollment

Miss your Initial Enrollment Period and you face permanent Part B penalties. Choose the wrong plan type and you could be locked out of Medigap coverage later. This is the single most consequential healthcare decision of your life.

Age 62-70: Social Security Filing

The difference between filing at 62 and 70 can be \$150,000+ over your lifetime. For married couples with survivor benefit implications, the stakes are even higher. Once you file, the decision is largely irreversible.

Age 73/75: RMD Commencement

Miss your first RMD and the penalty is 25% of the shortfall. Take the double-RMD in year two and you could spike your taxes and IRMAA. The decisions you made (or didn't make) about Roth conversions in your 60s determine how painful RMDs will be.

REAL-WORLD EXAMPLE: The Couple Who Used the Timeline

Ray and Connie, both 59, came to me with \$1.1 million in traditional IRAs, no Roth accounts, a paid-off home worth \$550,000, and combined Social Security projections of \$5,200/month at age 67.

Using this timeline, we built a five-year action plan: Ages 59–63: Convert \$65,000/year to Roth (\$325,000 total), filling the 22% bracket and staying below IRMAA thresholds. Total conversion tax: ~\$71,500. Age 64: Research Medicare plans, verify doctors' networks, compare Medigap quotes. Age 65: Enroll in Medicare Parts A and B; select Medigap Plan G and a standalone Part D plan; begin Medigap OEP. Age 67: File for Social Security at FRA; Connie files for her own benefit; Ray delays to 70 for maximum benefit and survivor protection. Age 70: Ray files for SS at \$3,900/month (vs. \$3,100 at FRA). Age 73: RMDs begin on a traditional IRA balance of ~\$500,000 instead of \$1.1M. First RMD: ~\$18,900 (vs. ~\$41,500 without conversions). IRMAA: Standard tier (no surcharges). Estimated lifetime savings from following the timeline: \$120,000+ in taxes, IRMAA, and Social Security optimization.

FREQUENTLY ASKED QUESTIONS

Q: What if I'm already past some of these milestones?

A: Start where you are. If you're 68 and haven't done Roth conversions, you still have time before RMDs at 73 or 75. If you're 72 and haven't reviewed your Medicare plan, start now. Every optimization helps, regardless of when you begin.

Q: Do I need to do everything on this timeline?

A: Not everything applies to everyone. Use it as a checklist and focus on the items relevant to your situation. The timeline is comprehensive so that nothing falls through the cracks.

Q: Can I do this all myself?

A: You can. But retirement planning involves Medicare rules, Social Security strategy, tax optimization, insurance decisions, and estate coordination. Most people benefit enormously from working with a professional who sees the full picture.

CHAPTER 46 CHECKLIST

- ☐ I've identified which timeline milestones are coming up for me in the next 1–3 years ☐
- ☐ I've begun building a year-by-year action plan based on this timeline ☐
- ☐ I've identified which chapters to re-read for my upcoming milestones ☐

READY TO TAKE THE NEXT STEP?

This timeline is your retirement GPS. But even the best GPS works better with a co-pilot who knows the road. I've guided hundreds of people through every milestone on this list. Let me be your co-pilot. One conversation. Your complete roadmap. No cost.

Darin Weidauer | 702-706-6564 | darinweidauer@ecos.care

C H A P T E R 4 7

Chapter 47: Your Retirement Blueprint

Putting It All Together

You've made it. Forty-seven chapters of Medicare rules, Social Security strategies, tax optimization, income planning, insurance protection, and real-world examples. If you've read this far, you know more about retirement planning than 95% of Americans.

But knowledge without action is just information. This final chapter is about turning everything you've learned into a personalized, actionable retirement blueprint.

The Five Pillars of a Bulletproof Retirement

Every strong retirement plan rests on five pillars. If any one of them is weak, the whole structure is vulnerable.

Your Personal Blueprint Worksheet

Use this worksheet to capture your current status and action items for each pillar. Be honest about where you are and where you need to be.

Pillar 1: Healthcare

- ☐ Current Medicare coverage: Original Medicare / Medicare Advantage / TRICARE for Life (circle one)
- ☐ Medigap plan in place? (If Original Medicare): Yes / No / Plan letter: ☐ **Part D plan in place? Yes / No / Plan name:** ☐
- ☐ Last annual plan review date: ____ ☐ Indemnity/supplemental coverage: Cancer / Hospital / Accident / Critical Illness (circle applicable)

Pillar 2: Income

- ☐ Social Security filing age planned: 62 / FRA / 70 / Already filing at age ____ ☐ Spouse's SS filing strategy coordinated? Yes / No / N/A ☐

☐ Income sources identified: SS / Pension / IRA / Roth / 401(k) / TSP / Taxable / Annuity / Rental / Other

☐ IRMAA threshold monitored? Yes / No / Current MAGI estimate: \$___

Pillar 3: Protection

☐ Long-term care insurance: Traditional / Asset-Based / Short-Term / None / Self-insuring ☐ Final expense coverage: Yes (\$___) / No ☐
☐ Indemnity plans: Cancer / Hospital / Accident / Critical Illness / None

Pillar 4: Tax Optimization

☐ Roth conversion strategy: In progress / Planned / Not started / Completed ☐ Estimated traditional IRA/401(k) balance at RMD age: \$ ☐ **Estimated first RMD:** \$ ☐ QCD strategy for charitable giving? Yes / No / N/A ☐ Current marginal tax bracket: 10% / 12% / 22% / 24% / 32% / 35% / 37%

Pillar 5: Legacy

☐
☐ Power of attorney (financial and healthcare) designated? Yes / No ☐ Beneficiary designations current on all accounts and policies? Yes / No / Need to review ☐

The One-Page Retirement Snapshot

Fill this out and bring it to your consultation. It gives me—or any professional you work with— a complete picture in 60 seconds.

Item Name Age / Date of Birth Spouse’s Name and Age	Your Answer
Current Employment Status	Working / Retired / Part-time
Monthly Income Target	\$
Social Security (estimated or actual)	\$ /month at age ___
Pension	\$ /month
Traditional IRA / 401(k) Balance	\$
Roth IRA Balance	\$

Item Name Age / Date of Birth Spouse's Name and Age	Your Answer
Other Savings / Investments	\$
Current Medicare Coverage	Original / Advantage / TRICARE /
Medigap Plan	Employer / Not yet enrolled None / Plan ____
Part D Plan	None / Plan name: ____
LTC Insurance	None / Traditional / Asset-Based / Short-

The Final Word

You’ve earned your retirement. Every year of work, every sacrifice, every contribution to your 401(k), every tax dollar paid into Social Security and Medicare—it was all building toward this moment. The decisions you make now will determine whether those decades of effort pay off in comfort, security, and freedom—or whether they’re eroded by avoidable mistakes.

The retirement system is complex. That’s by design. It favors people who take the time to understand it—or who work with someone who does.

You don’t have to do this alone. You shouldn’t do this alone. And with this book in your hands, you’re already miles ahead of most Americans.

Now take the next step.

READY TO TAKE THE NEXT STEP?

You’ve read the book. You have the knowledge. Now let’s turn it into a plan that’s built for YOUR life. Call or text me for a free, no-obligation retirement blueprint session. We’ll review your five pillars, identify your gaps, and build an action plan that protects everything you’ve worked for. This is what I was made to do. Let’s do it together.

Darin Weidauer | 702-706-6564 | darinweidauer@ecos.care

Bonus: 10 Questions to Ask Before You Trust Anyone With Your Retirement

Not all advice is equal — and not all advisors are working for you. Some are captive agents who can only sell one company's products. Some earn nothing unless you buy today. Before you let anyone touch the plan you'll live on for thirty years, ask these ten questions and listen carefully to the answers.

1. **Are you independent, or do you represent one company?** An independent agent can shop dozens of carriers. A captive agent has one shelf.
2. **How are you paid — and will you tell me in writing?** Honest professionals answer this without flinching.
3. **Are you licensed for both insurance and Medicare products in my state?** Verify at your state's Department of Insurance website.
4. **Will you review my Social Security claiming strategy, or only sell me a policy?** The claiming decision is often worth more than any single product.
5. **What happens if my situation changes — will you still take my call in three years?** Ask for client references who have been with them longer than five years.
6. **Do you review my Medicare plan every year during Open Enrollment, at no charge?** Plans change every year. Your review should, too.
7. **Can you explain IRMAA and show me where my income lands?** If they've never heard of it, the interview is over.
8. **Will you tell me when the right answer is to do nothing?** The best advisors regularly recommend against a sale.
9. **What credentials do you hold, and who audits them?** Look for verifiable designations — for Social Security planning, an RSSA (Registered Social Security Analyst) has completed accredited training in claiming strategy.

10. **Will you put your recommendations in writing?** A plan that can't survive paper isn't a plan.

Bonus: The Five-Document Estate Checklist Every Retiree Needs

This is not a book about estate planning — but no retirement plan is complete without these five documents. Missing even one can cost your family months in court and thousands in fees. Review this list with a qualified estate attorney in your state.

Document	What It Does	What Happens Without It
Will	Directs who receives your property and who settles	State intestacy law decides — not you
Durable Power of Attorney	Names someone to handle finances if you're	Your family may need a court-ordered guardianship
Healthcare Power of Attorney	Names someone to make medical decisions for you	Doctors may follow default rules, not your wishes
Living Will / Advance Directive	States your wishes for end-of-life care	Your family guesses — under the worst possible stress
Beneficiary Designations	Controls IRAs, 401(k)s, life insurance	An ex-spouse or deceased relative may still be listed
(reviewed!)	— overriding your will	

The beneficiary trap: Retirement accounts and life insurance pass by beneficiary designation, not by will. A will that says one thing and a 401(k) form that says another? The form wins. Review every beneficiary designation after any marriage, divorce, birth, or death — and at least every three years.

Bonus item — the digital estate: Make a secure list of your online accounts and passwords (or use a password manager with an emergency contact). Your executor cannot settle accounts they cannot find.

Source: General estate-planning principles; consult a licensed estate attorney in your state for documents valid in your jurisdiction.

READY TO TAKE THE NEXT STEP? I don't draft legal documents — but I make sure your insurance, annuity, and retirement-account beneficiaries actually match your plan, and I can point you to qualified estate attorneys my clients trust. Call or text me for a free, no-obligation Retirement Blueprint Session . Darin Weidauer | ECOS | 702-706-6564 | darinweidauer@ecos.care

Conclusion: The Call That Could Change Everything

I started this book with a woman named Linda who missed her Medicare enrollment window and paid a penalty for the rest of her life. I end it with you—a person who now has the knowledge to make sure that never happens.

You know how Medicare works. You know when and how to enroll. You know the gaps in coverage and how to fill them. You know how IRMAA can silently drain your retirement income and how to prevent it. You know how to maximize your Social Security benefits, coordinate with a spouse, and avoid the Earnings Test trap. You know how to build a retirement income stack that's tax-efficient, IRMAA-aware, and diversified across multiple buckets. You know how to protect yourself from long-term care costs, final expenses, and the non-medical financial devastation of a health crisis.

You know more than 95% of retirees in America. That's not flattery. That's fact.

But knowledge alone doesn't protect you. Action does.

The difference between someone who reads this book and someone who reads this book and then makes one phone call is potentially hundreds of thousands of dollars over a lifetime. That's the difference between a retirement that feels like a reward and one that feels like a struggle.

I've spent 22 years serving this country in uniform. I've earned five Master's degrees. I've taught at universities across Southern California. I've sat across the table from thousands of people facing the exact decisions you're facing now. And I've committed the rest of my career to making sure no one walks into retirement without a plan.

You've done the hard part—you've educated yourself. Now let me do my part.

YOUR FREE RETIREMENT BLUEPRINT SESSION

Call or text: 702-706-6564

Email: darinweidauer@ecos.care

No pressure. No sales pitch. Just answers.

Because you've earned a retirement that honors every sacrifice you've made.

— Darin Weidauer

Glossary of Retirement and Medicare Terms

This glossary defines 60+ key terms referenced throughout this book. Use it as a quick reference whenever you encounter unfamiliar terminology.

Activities of Daily Living (ADLs)

Six basic self-care activities (bathing, dressing, eating, toileting, transferring, continence) used to determine eligibility for long-term care benefits.

Adjusted Gross Income (AGI)

Your total gross income minus specific deductions (IRA contributions, student loan interest, etc.). The starting point for calculating MAGI.

Annual Enrollment Period (AEP)

October 15 – December 7 each year. The window to change Medicare Advantage plans, Part D plans, or switch between Original Medicare and Advantage.

Annuity

An insurance contract that provides guaranteed periodic payments, often for life, in exchange for a premium payment.

Asset-Based Long-Term Care

A hybrid insurance product combining life insurance or an annuity with long-term care benefits. If unused for care, provides a death benefit or return of premium.

Average Indexed Monthly Earnings (AIME)

The average of your 35 highest years of indexed earnings, divided by 420 months. Used to calculate your Social Security PIA.

Bend Points

Dollar thresholds in the Social Security benefit formula where the replacement rate changes (90%, 32%, 15%). Adjusted annually for wage growth.

Benefit Period

In Medicare Part A, begins when you're admitted to a hospital and ends when you haven't received inpatient care for 60 consecutive days.

CHAMPVA

Civilian Health and Medical Program of the Department of Veterans Affairs. Provides coverage for dependents of veterans with permanent total service-connected disability.

COBRA

Continuation of employer health coverage after leaving a job. Lasts up to 18 months but is NOT creditable coverage for Medicare purposes.

Coinsurance

Your share of costs for a covered service, calculated as a percentage (e.g., 20% of the Medicare-approved amount for Part B services).

Community Spouse Resource Allowance (CSRA)

The maximum amount of assets the non-institutionalized spouse can retain when the other spouse qualifies for Medicaid long-term care.

Copay (Copayment)

A fixed dollar amount you pay for a covered service (e.g., \$30 for a doctor visit).

Cost-of-Living Adjustment (COLA)

Annual increase to Social Security benefits based on inflation, as measured by the Consumer Price Index. The 2026 COLA is 2.8%.

Creditable Coverage

Health insurance that is at least as comprehensive as Medicare Part B (or Part D for drug coverage). Determines whether late enrollment penalties apply.

CRDP

Concurrent Retirement and Disability Pay. Allows military retirees with 50%+ VA disability to receive both military retirement and VA disability without offset.

Custodial Care

Non-medical assistance with Activities of Daily Living (bathing, dressing, eating). NOT covered by Medicare.

Deductible

The amount you pay before insurance begins covering costs (e.g., the Part A inpatient deductible of \$1,736 in 2026).

Delayed Retirement Credits

The 8% per year increase in Social Security benefits for each year you delay filing beyond your Full Retirement Age, up to age 70.

Dual Eligible

A person who qualifies for both Medicare and Medicaid simultaneously.

D-SNP

Dual Eligible Special Needs Plan. A type of Medicare Advantage plan designed specifically for people with both Medicare and Medicaid.

Earnings Test

Social Security's rule that reduces benefits for people who file before FRA and continue to earn above a certain threshold (\$24,480 in 2026).

Elimination Period

A waiting period (typically 0–180 days) before long-term care insurance benefits begin paying.

Extra Help (Low-Income Subsidy)

A federal program that helps people with limited income and assets pay Part D premiums, deductibles, and copays.

FEHB

Federal Employees Health Benefits. The health insurance program for federal civilian employees and retirees.

FERS

Federal Employees Retirement System. The three-part retirement program (pension, TSP, Social Security) for federal employees hired after 1983.

Formulary

A list of prescription drugs covered by a Part D or Medicare Advantage plan, organized into cost tiers.

Full Retirement Age (FRA)

The age at which you're entitled to 100% of your Social Security PIA. Age 67 for those born in 1960 or later.

General Enrollment Period (GEP)

January 1 – March 31 each year. A backup enrollment period for Part B with coverage starting July 1 and potential penalties.

Government Pension Offset (GPO)

Reduces Social Security spousal or survivor benefits by two-thirds of a government pension from non-Social-Security-covered work.

Guaranteed Issue

An insurance policy that accepts all applicants regardless of health. Common in final expense and some short-term care policies.

HECM

Home Equity Conversion Mortgage. The FHA-insured reverse mortgage program for homeowners aged 62+.

HSA

Health Savings Account. A tax-advantaged account for people with High-Deductible Health Plans. Triple tax benefit: deductible contributions, tax-free growth, tax-free withdrawals for medical expenses.

Indemnity Insurance

Supplemental insurance that pays a fixed cash benefit directly to you when a qualifying health event occurs (e.g., cancer diagnosis, hospitalization, accident).

Initial Enrollment Period (IEP)

The 7-month window around your 65th birthday during which you can enroll in Medicare Parts A and B without penalty.

IRMAA

Income-Related Monthly Adjustment Amount. A surcharge on Medicare Part B and Part D premiums for people with higher incomes. Uses a two-year lookback.

Lifetime Reserve Days

60 days of additional Medicare Part A hospital coverage available once you've exceeded 90 days in a benefit period. Non-renewable.

MAGI

Modified Adjusted Gross Income. Your AGI plus tax-exempt interest income. Used to determine IRMAA and ACA premium subsidies.

Medicaid

A joint federal-state program providing healthcare coverage to low-income individuals, including long-term care for those who qualify.

Medicare Advantage (Part C)

A private plan that delivers Part A and Part B benefits (and usually Part D) through an insurance company instead of Original Medicare.

Medicare Savings Program (MSP)

State programs that help low-income Medicare beneficiaries pay for premiums, deductibles, and copays (QMB, SLMB, QI, QDWI).

Medigap (Medicare Supplement)

Standardized private insurance plans that cover cost-sharing gaps in Original Medicare (deductibles, coinsurance, copays).

Modified Adjusted Gross Income

See MAGI.

MYGA

Multi-Year Guaranteed Annuity. A fixed annuity with a guaranteed interest rate for a specified period, similar to a CD.

Original Medicare

Traditional Medicare consisting of Part A (hospital) and Part B (medical) administered directly by the federal government.

Part A

Medicare hospital insurance covering inpatient care, skilled nursing, home health, and hospice.

Part B

Medicare medical insurance covering outpatient care, doctor visits, preventive services, and durable medical equipment.

Part C

See Medicare Advantage.

Part D

Medicare prescription drug coverage, delivered through private insurance plans.

PIA (Primary Insurance Amount)

Your monthly Social Security benefit amount at Full Retirement Age, calculated from your AIME using the bend point formula.

Prior Authorization

A requirement that your insurance plan approve a medical service before it's provided. Common in Medicare Advantage plans.

Provisional Income

AGI (without Social Security) + tax-exempt interest + 50% of Social Security benefits. Used to determine how much of your Social Security is taxable.

QCD (Qualified Charitable Distribution)

A direct transfer of up to \$111,000 /year (2026) from a traditional IRA to a qualified charity. Counts toward RMD but not from taxable income.

QMB (Qualified Medicare Beneficiary)

A Medicare Savings Program that pays Part A and Part B premiums, deductibles, copays, and coinsurance for low-income beneficiaries.

Required Minimum Distribution (RMD)

Mandatory annual withdrawals from traditional retirement accounts starting at age 73 (or 75 for those born 1960+).

Roth Conversion

Moving money from a traditional retirement account to a Roth IRA, paying income tax on the converted amount but gaining tax-free growth and withdrawals.

RSSA

Registered Social Security Analyst. A professional certified in Social Security optimization strategies.

Section 1035 Exchange

A tax-free exchange of one life insurance policy or annuity for another, commonly used to reposition assets into an asset-based LTC policy.

SECURE 2.0 Act

Legislation signed in 2022 that changed RMD ages, introduced Super Catch-Up contributions, and made other retirement savings reforms.

Skilled Nursing Facility (SNF)

A facility providing skilled nursing care and rehabilitation after a qualifying hospital stay. Medicare covers up to 100 days per benefit period.

Special Enrollment Period (SEP)

A period outside standard enrollment windows when you can sign up for Medicare due to special circumstances (e.g., losing employer coverage).

SPIA

Single Premium Immediate Annuity. A lump-sum purchase that begins monthly income payments immediately, often for life.

SSA-44

The form used to request IRMAA reconsideration due to a life-changing event that reduced your income.

Thrift Savings Plan (TSP)

The federal government's retirement savings plan for federal employees and uniformed services members. Similar to a 401(k).

TRICARE for Life

Healthcare coverage for military retirees (20+ years) that wraps around Medicare. Requires enrollment in Medicare Parts A and B.

Windfall Elimination Provision (WEP)

A formula that reduces Social Security benefits for people who receive pensions from work not covered by Social Security.

2026 Quick Reference Card

All key numbers on one page. Print this card and keep it with your important documents.

Medicare Costs — 2026

Item	Amount
Part A Premium (40+ quarters)	\$0
Part A Inpatient Deductible	\$1,736
Part A Days 61-90 Coinsurance	\$434/day
Part A Lifetime Reserve (Days 91- 150)	\$868/day
Part A SNF Days 21-100	\$217.00/day
Part B Standard Premium	\$202.90/month
Part B Annual Deductible	\$283
Part B Coinsurance	20% (no OOP max)
Part D Max Deductible	\$615
Part D OOP Cap	\$2,100/year
Part D National Base Premium	\$38.99/month

IRMAA Thresholds — 2026 (Based on 2024 Tax Return)

Single	Married Filing Jointly	Part B Premium	Part D Surcharge
≤\$109,000	≤\$218,000	\$202.90	\$0
\$109,001-\$137,000	\$218,001-\$274,000	\$284.10	+\$14.50
\$137,001-\$171,000	\$274,001-\$342,000	\$405.80	+\$37.50
\$171,001-\$205,000	\$342,001-\$410,000	\$527.50	+\$60.40
\$205,001-\$500,000	\$410,001-\$750,000	\$649.20	+\$83.30
>\$500,000	>\$750,000	\$689.90	+\$91.00

Source: CMS/SSA, 2026 income-related monthly adjustment amounts. Based on 2024 MAGI.

Social Security — 2026

Item	Amount
COLA	2.8%
Maximum Taxable Earnings	\$184,500

Item	Amount
Average Monthly Benefit	~\$2,071
Maximum at FRA (67)	\$4,152/month
Maximum at 70	\$5,181/month

Figures accurate as of the official 2026 releases. Current figures and corrections: MyECOS360.com/book-updates

Item	Amount
Earnings Test (under FRA)	\$24,480 (\$1 withheld per \$2 over)
Earnings Test (FRA year)	\$65,160 (\$1 withheld per \$3 over)
FRA (born 1960+)	Age 67

Retirement Account Limits — 2026

Account	Under 50	50+ Catch-Up	60-63 Super Catch- Up
401(k) / 403(b) / TSP	\$24,500	\$32,500	\$35,750
Traditional / Roth IRA	\$7,500	\$8,600	\$8,600
HSA (Individual / Family)	\$4,400 / \$8,750	+\$1,000	—

Key Tax Numbers — 2026

Item	Amount
Standard Deduction (Single)	\$16,100
Standard Deduction (MFJ)	\$32,200
Standard Deduction (65+ additional)	\$2,050 (single) / \$1,650 (married, each)
RMD Starting Age (born 1951-1959)	73
RMD Starting Age (born 1960+)	75
QCD Annual Limit	\$111,000
Estate Tax Exemption	\$15 million (individual)

Contact

Darin Weidauer | 702-706-6564 | darinweidauer@ecos.care

Free Retirement Blueprint Session — Call or Text Anytime

Resources and Official Links

These are the official government and industry sources referenced throughout this book. Bookmark them for ongoing reference.

Medicare

Medicare.gov — <https://www.medicare.gov> (Plan Finder, enrollment, benefits)

1-800-MEDICARE — 1-800-633-4227 (24/7 assistance)

CMS.gov — <https://www.cms.gov> (Centers for Medicare & Medicaid Services)

State Health Insurance Assistance Program (SHIP) — <https://www.shiphelp.org> (free local Medicare counseling)

Medicare Rights Center — <https://www.medicarerights.org>

Medicare Fraud Reporting — 1-800-HHS-TIPS (1-800-447-8477)

Social Security

SSA.gov — <https://www.ssa.gov> (benefits, statements, enrollment)

My Social Security Account — <https://www.ssa.gov/myaccount>

Social Security Office Locator — <https://secure.ssa.gov/ICON/main.jsp>

1-800-772-1213 — Social Security Administration phone line

Form SSA-44 (IRMAA Appeal) — <https://www.ssa.gov/forms/ssa-44.pdf>

National Association of Registered Social Security Analysts — <https://www.narssa.org>

Internal Revenue Service (IRS)

IRS.gov — <https://www.irs.gov>

IRS Publication 590-B (IRA Distributions) —
<https://www.irs.gov/pub/irs-pdf/p590b.pdf>

IRS Publication 575 (Pension and Annuity Income) —
<https://www.irs.gov/pub/irs-pdf/p575.pdf>

RMD Worksheets —
<https://www.irs.gov/retirement-plans/required-minimum-distributions>

Form W-4V (Voluntary Withholding from SS) —
<https://www.irs.gov/forms-pubs/about-form-w-4v>

Veterans and Military

VA.gov — <https://www.va.gov> (benefits, healthcare, disability)

TRICARE — <https://www.tricare.mil>

TRICARE for Life — <https://www.tricare.mil/tfl>

VA Aid & Attendance — <https://www.va.gov/pension/aid-attendance-housebound>

Thrift Savings Plan — <https://www.tsp.gov>

CHAMPVA —
<https://www.va.gov/communitycare/programs/dependents/champva>

Federal Employees

OPM.gov (FEHB, FERS, CSRS) — <https://www.opm.gov>

FEHB Plan Comparison Tool —
<https://www.opm.gov/healthcare-insurance/healthcare>

Long-Term Care and Aging

LongTermCare.acl.gov — <https://longtermcare.acl.gov> (federal LTC planning resources)

Eldercare Locator — <https://eldercare.acl.gov> / 1-800-677-1116

Genworth Cost of Care Survey —
<https://www.genworth.com/aging-and-you/finances/cost-of-care.html>

National Academy of Elder Law Attorneys —
<https://www.naela.org>

Alzheimer's Association — <https://www.alz.org> / 1-800-272-3900

Consumer Protection

Federal Trade Commission (Report Fraud) —
<https://reportfraud.ftc.gov>

Senior Medicare Patrol — <https://www.smpresource.org>

National Do Not Call Registry — <https://www.donotcall.gov>

Consumer Financial Protection Bureau —
<https://www.consumerfinance.gov>

RETIRE WITH CONFIDENCE

“This book is the conversation I wish every American could have before they turn 65.”

— Darin Weidauer

Why You Need This Book

Medicare has four parts, dozens of plan types, and enrollment penalties that last a lifetime. Social Security has over 2,700 rules. The tax code punishes retirees who don't plan their withdrawals strategically. And long-term care can erase a lifetime of savings in a matter of years.

This book gives you everything you need to navigate the retirement gauntlet: every Medicare premium, deductible, and IRMAA bracket for 2026; the Social Security filing strategy that could

put \$100,000+ more in your pocket; Roth conversion tactics that slash your taxes and protect your Medicare premiums; and long-term care strategies that safeguard your family's financial future.

Written in plain English, packed with real numbers and real examples, and designed to be referenced year after year.

About the Author

Darin Weidauer is a Gerontologist, U.S. Air Force Veteran, Independent Insurance Agent, and Registered Social Security Analyst. He served 22 years in the United States Air Force and specializes in Medicare, retirement income, and long-term care planning. He holds five Master's degrees (including an MBA from Pepperdine, a Master's in Long-Term Care from USC, and a Master's from Pepperdine Law School) and has taught at Loyola Marymount University and more than 54 colleges across Southern California. He works for you—not for any insurance company.

FREE RETIREMENT BLUEPRINT SESSION

Call or Text: 702-706-6564

Email: darinweidauer@ecos.care

No pressure. No sales pitch. Just answers.

Page 1 | 702-706-6564 | darinweidauer@ecos.care

Sources & Data Credits

Every 2026 figure in this book was verified against the official government release listed below. Where private-sector research is used (healthcare cost projections, long-term care costs), the source is credited directly.

Medicare (all official government data):

Centers for Medicare & Medicaid Services (CMS), "2026 Medicare Parts A & B Premiums and Deductibles," fact sheet, November 14, 2025 — <https://www.cms.gov/newsroom/fact-sheets/2026-medicare-parts-b-premiums-deductibles>

CMS, “Final CY 2026 Part D Redesign Program Instructions” (Part D \$615 maximum deductible, \$2,100 out-of-pocket cap, benefit phases) — <https://www.cms.gov>

CMS, Medicare Drug Price Negotiation Program (negotiated prices effective 2026; second round effective 2027) — <https://www.cms.gov/inflation-reduction-act-and-medicare>

Medicare.gov, “How much does Medicare drug coverage cost?” (Part D late-enrollment penalty; \$38.99 national base beneficiary premium) — <https://www.medicare.gov>

Federal Register, “Medicare Program; Medicare Part B Monthly Actuarial Rates, Premium Rates, and Annual Deductible Beginning January 1, 2026” (2026 IRMAA amounts)

Social Security (all official government data):

Social Security Administration (SSA), “Social Security Announces 2.8 Percent Benefit Increase for 2026,” press release, October 24, 2025 — <https://www.ssa.gov/news>

SSA, “2026 Cost-of-Living Adjustment (COLA) Fact Sheet” (taxable maximum \$184,500; earnings test \$24,480/\$65,160; quarter of coverage \$1,890) — <https://www.ssa.gov/cola>

SSA, Frequently Asked Questions, “What is the maximum Social Security retirement benefit payable?” (\$2,969 at 62; \$4,152 at FRA; \$5,181 at 70 for 2026)

Taxes & Retirement Accounts:

Internal Revenue Service (IRS), Notice 2025-67 and news release “401(k) limit increases to \$24,500 for 2026; IRA limit increases to \$7,500” — <https://www.irs.gov>

IRS, Revenue Procedure 2025-32 (2026 standard deduction, estate tax exclusion of \$15 million)

IRS, “Enhanced Deduction for Seniors” (\$6,000 deduction for taxpayers 65+, tax years 2025–2028) — <https://www.irs.gov>

Private research (credited where used):

Fidelity Investments, Retiree Health Care Cost Estimate (healthcare spending in retirement)

CareScout Cost of Care Survey 2024, a Genworth company (long-term care cost medians) — <https://www.carescout.com/> / <https://www.genworth.com>

U.S. Department of Health and Human Services (HHS), long-term care usage statistics — <https://acl.gov>

All figures are for calendar year 2026 unless labeled otherwise. Items labeled “Coming in 2027” are announced but not yet in effect; they take effect January 1, 2027, unless noted. Figures change annually — verify current amounts at Medicare.gov, SSA.gov, and IRS.gov, or ask us to check for you.

Added for the Expanded Edition (Chapters 29-31, 35, 42, 45):

Internal Revenue Service —
Forms W-4V, W-4P, W-4R;
Publication 505; quarterly
underpayment interest rates
(Rev. Rul. 2025-22, 2026-5,
and IRB 2026-22: 7% Q1, 6%
Q2, 7% Q3 2026); 2026 gift tax
figures (annual exclusion
\$19,000; lifetime exemption
\$15,000,000 under Public Law
119-21). U.S. Department of
Housing and Urban
Development / FHA —
Mortgagee Letter 2025- 22,
2026 HECM maximum claim
amount of \$1,249,125.

• •

State departments of revenue
(incl. West Virginia Tax
Division, HB 4880 Social
Security phase-out completed
Jan. 1, 2026) and 2026 state
retirement-tax surveys by
Kiplinger and Money. AARP
and National Alliance for
Caregiving — Caregiving in the
US 2025 (July 24, 2025):

• • • •

63 million family caregivers,
average 27 hours/week,
≈\$7,200/yr out-of-pocket.
Centers for Medicare &
Medicaid Services — GUIDE
Model (dementia care; respite
benefit up to \$2,500/yr). One
Big Beautiful Bill Act (Pub. L.
119-21) — 529 K-12
withdrawal limit of \$20,000

• • • •	State departments of revenue (incl. West Virginia Tax Division, HB 4880 Social Security phase-out completed Jan. 1, 2026) and 2026 state retirement-tax surveys by Kiplinger and Money. AARP and National Alliance for Caregiving — Caregiving in the US 2025 (July 24, 2025):
• 	beginning tax year 2026; expanded qualified expenses and postsecondary credentialing; SECURE 2.0 529-to-Roth rollover (\$35,000 lifetime). CareScout Cost of Care Survey 2024 (long-term care medians, as cited in Chapter 32). FREE RETIREMENT BLUEPRINT SESSION — ECOS conversation could save you thousands.
One	Call or text Darin Weidauer at 702-706-or email darinweidauer@ecos.care .
6564	No pressure. No sales pitch. Just answers.



Your Retirement Deserves a Plan. Not a Guess.

One wrong Medicare box, one mistimed Social Security claim, one Roth conversion that crosses an invisible income line — and the cost follows you for the rest of your life. This book walks you through every decision, in order, in plain English, using the actual 2026 numbers from the agencies that set them.

INSIDE THIS BOOK

- ✓ Every 2026 Medicare premium, deductible, and copay — verified against CMS
- ✓ The IRMAA “cliff” that adds up to \$13,872 a year — and how to stay under it
- ✓ Claiming Social Security at 62 vs. 67 vs. 70: what each choice really pays
- ✓ The \$2,100 drug cap, \$35 insulin, and the 2027 changes already in law
- ✓ Roth conversions, RMDs, and the tax mistakes hiding in your 401(k)
- ✓ Long-term care, life insurance, and veteran benefits

ABOUT THE AUTHOR

Darin Weidauer is an independent licensed insurance agent, gerontologist, and Registered Social Security Analyst who served 22 years in the U.S. Air Force. Through ECOS he has helped thousands of Americans enroll in Medicare, time their Social Security, and build retirement income plans — with straight answers instead of sales pitches.

YOUR FREE RETIREMENT BLUEPRINT SESSION

No cost. No obligation. No sales pitch. Just straight answers from someone who's been there.

CALL OR TEXT: 702-706-6564

darinweidauer@ecos.care



Scan to save my contact

ECOS